

NOTICE OF DISPUTE — STAGE 2

Clause 1.11.2(b), Queensland Public Health Sector Certified Agreement (No. 12) 2025

To	Ms Noelle Cridland, Health Service Chief Executive, Metro South Health — as the appropriate management representative under clause 1.11.2(b), for the reasons at paragraph 2
Copied to	Ms Jacqui Roberts, Principal Consultant, Human Resources, Logan and Beaudesert Health Service · Ms Emily Petering, Together Queensland · Metro South Correspondence
Employee	Cory Shepherd, employee number 388372 — AO3, Switchboard Services, Logan Hospital. Permanent full-time, 76 hours per fortnight. Together Queensland member and endorsed workplace delegate.
Date	7 September 2026
Matter	Attendance, roster, leave and pay from 26 June 2026 (MSH-INJ-5795)

1. THE REFERRAL

I refer this dispute to Stage 2 under clause 1.11.2(b) of the Agreement, and ask that a conference of the parties be arranged. I ask that Together Queensland attend with me.

A Stage 1 notice was given on 3 August 2026. On 4 August 2026 Ms Taylor acknowledged that the 24-hour requirement in clause 1.11.2(a) "will not be achieved", confirmed the dispute was under review, and undertook that I would be contacted on completion of the review. The seven days allowed by clause 1.11.2(a) expired on 10 August 2026. No outcome of Stage 1 has ever been communicated to me.

I earlier referred the matter to Stage 2 and withdrew that referral on 28 August 2026, in order to work through the issues directly with Ms Roberts. In doing so I withdrew the referral only, and recorded that my concerns regarding pay, the leave debited since 3 July 2026 and the adjustments in the Employee Capability Checklist of 3 July 2026 remained unresolved and in dispute. They remain unresolved, and the position has worsened since.

The matter can be put shortly. I have been absent from the workplace since 3 July 2026 because I was directed not to attend. On each day of that absence a leave type has been selected and entered on my record on my behalf. Of the leave types available, three produce a payment and one does not. The one that does not has been selected on every day since 3 July 2026 — after I instructed in writing on that day that annual leave was to be applied; after every objection I have made since; and again on 4 August 2026, on the very day the Health Service itself identified the paid alternative and undertook to apply it. That is why I have not been paid.

2. WHY THIS REFERRAL IS MADE TO YOU

Clause 1.11.2(b) requires the matter to be referred to "the appropriate management representative". Stage 1 was conducted with my immediate supervisor, as clause 1.11.2(a) provides. I refer Stage 2 to you because two of the matters in dispute cannot be decided below your office.

Special leave is a chief executive's power. Directive 12/24, which applies to every employee covered by the Agreement through clause 9.12, provides at clause 6.1 that "a chief executive may approve paid leave for employees for any purpose", and at clause 6.5 that "in determining an application for leave under clause 6.1 or clause 6.2, a chief executive must consider" the reason for the leave, its duration, and the impact on the employee if it is not approved. Section 6 of HR Policy C13 records that the delegate for a decision of this kind is the officer listed in the HR Delegations Manual. I applied for special leave on full pay on 28 July and again on 5 August 2026. It has never been determined. No officer below your office, or your delegate under the HR Delegations Manual, is able to determine it.

A decision on special leave would resolve the immediate position on its own. It is paid at full pay, and under Directive 12/24 it is not debited from any leave account. Granting it would therefore restore my income and leave my accrued entitlements intact. It would not require the Health Service to determine the leave coding, the hours at which leave has been applied, the long service leave eligibility date, or the basis of the direction that I not attend — each of which would remain in dispute and can be dealt with at the conference. It is the one decision in this matter that can be made now, by one office, and it requires no view to be taken on any of the others.

No instrument or officer has ever been identified for the direction that I not attend. I asked for both in the Stage 1 process on 4 August 2026 and have asked since. The identification of the delegate and the instrument, or the making of a decision, sits at executive level.

I do not ask that you convene the conference personally. If you nominate a representative to hold it and to decide the matters within their delegation, I will attend with them, and I ask only that the matters reserved to the chief executive be determined by you or by your delegate under the HR Delegations Manual.

3. THE MATTERS IN DISPUTE

1. **Pay.** I have received no wages since 13 July 2026 — fifty-six days, and three pay days at nil: 29 July, 12 August and 26 August 2026. I have been ready, willing and available throughout, and certified fit with restrictions on the Health Service's own form since 3 July 2026.
2. **The direction not to attend.** I have been held out of the workplace since 3 July 2026. I asked in the Stage 1 process on 4 August 2026 for the instrument authorising that direction, and have asked repeatedly since. It has never been identified, and no written decision returning me to work or declining to has ever been issued. The reason given for my continued absence has been the state of the medical evidence. My treating psychiatrist wrote to the Health Service with four questions between 13 and 19 August 2026 and, as at 5 September 2026, had received no response.
3. **The leave coding, and how the payment became nil.** Each "Sick Leave – No Pay" entry is a positive act — a leave type selected from those available and submitted on my behalf. The alternatives were available, and they were known to the officer entering them: on 4 August 2026 Ms Taylor identified my recreation leave balance at 41.21 hours and undertook that, absent a response, she would apply it "to ensure you receive the available paid leave entitlement for this period". The pay advice for that fortnight records "NP Sick Leave" 76.00 hours and gross pay of \$0.00. Section 6 of HR Policy C13 permits leave to be submitted on an employee's behalf only where the employee's request for leave "is documented in writing" and the supporting documents "are sourced and retained". My written requests are for annual leave (3 July 2026) and special leave on full pay (5 August 2026). There is no written request for sick leave without pay, and on 31 August 2026 I confirmed in terms that I was not applying for personal leave. My leave record shows these not as a single standing code but as **discrete daily requests, each separately entered and separately approved** — among them "SICK LEAVE - NO PAY (03.08.2026 - 03.08.2026)" and "SICK LEAVE - NO PAY (04.08.2026 - 04.08.2026)", each marked APPROVED. The second of those was entered for 4 August 2026 — the same day on which Ms Taylor wrote that she was required to process leave on my behalf, identified my recreation leave balance at 41.21 hours, and undertook to apply it. On that day four courses were open: annual leave, which I had requested on 3 July; recreation leave, which she had identified and undertaken to apply; special leave on full pay, which I requested the following morning; and, within the very category selected, paid sick leave, of which I held a balance. The course entered and approved was the unpaid variant — the only one of the four which produces no payment. I have at no point declined to be paid: on 5 August 2026 I declined only that the absence be funded out of my own accrued recreation leave, and asked in the same sentence that special leave on full pay be applied instead.
4. **The entry of 30 August 2026.** The leave takings report provided to me on 4 September 2026 records, for 30 August 2026, a leave type of "Absent Without Leave" (AWOL_NP), 7.60 hours, approved. It is the only entry of that type in the report, which covers 25 March 2019 to 4 September 2026. On 31 August 2026 Ms Taylor wrote that she had applied "S/L and A/L in lieu of S/L for your absence from the workplace through an AVAC as requested, for the past fortnight 17th August 2026 - 30th August 2026 ... PRN 249 863 66". 30 August 2026 is the last day of that fortnight. I have at no time been absent without authority.
5. **Special leave.** My application for special leave on full pay under Directive 12/24, made on 28 July and 5 August 2026, has been neither granted nor refused.
6. **Hours.** Leave was applied on 31 August 2026 at "0.5 FTE". The last reduced-hours arrangement expired on 28 June 2026 and nothing replaced it. The Health Service's own Request for Medical Information of 31 July 2026 describes my employment as "Permanent Full-time (76 hours per fortnight)".
7. **Long service leave.** On 4 September 2026 Payroll confirmed that I commenced on 25 March 2019 and that my long service leave eligibility would ordinarily arise on 25 March 2026, and advised a corrected date of 29 September 2026 after excluding three periods of leave without pay: 18 May to 10 June 2022; 21 June to 20 September 2024; and 13 December 2024 to 23 February 2025. My accrued balance is 342.5518 hours.

Three instruments give access to long service leave at seven years. Clause 22(c) of the Hospital and

Health Service General Employees (Queensland Health) Award – State 2015 provides that "Employees who have completed 7 years' continuous service are entitled to take long service leave on full pay or half pay". Clause 8.1 of Directive 10/24 provides that "employees are entitled to take pro rata long service leave after 7 years continuous service". Section 1 of HR Policy C38, to which clause 9.10.1 of the Agreement refers, provides that employees "may apply for pro rata long service leave on full pay, or half pay, after completing seven years continuous service". Seven years of continuous service fell on 25 March 2026.

I have read that chain for a provision excluding approved leave without pay from continuous service and have not found one. Section 94 of the *Industrial Relations Act 2016* applies Part 4 of the Act to long service leave, and section 134(3) provides that "An employee's continuity of service with an employer is not broken by an absence, including through illness or injury — (a) on paid leave approved by the employer; or (b) on unpaid leave approved by the employer". The leave takings report the Health Service produced on 4 September 2026 records 103 entries of leave without pay across the whole of my service, and every one of them is marked approved. Sixty-seven of those entries fall in the period 21 June to 20 September 2024, each recorded as leave category "Special", leave type "Leave Without Pay", status "APPROVED". The Health Service's own record therefore places every excluded day inside section 134(3)(b), not outside it. Where the Act intends a period not to count towards service it says so expressly, as it does for parental leave at section 90(2). If a provision exists which authorises the exclusion, I ask that it be identified. If there is none, my eligibility arose on 25 March 2026.

Separately, the agreement by which I was reinstated deals with one period of leave without pay, being 13 December 2024 to 23 February 2025, which the calculation applies. It does not deal with 21 June to 20 September 2024 in that way. It reserved those absences, and it reserved my position in relation to them. The Health Service then recorded them as approved leave without pay, which is the very absence section 134(3)(b) says does not break continuity of service. Each of the three excluded periods is recorded on that report as approved leave without pay: a single entry of 112 hours for 18 May to 10 June 2022; sixty-seven daily entries for 21 June to 20 September 2024; and a single entry of 395.2 hours for 13 December 2024 to 23 February 2025. I accept that under clause 7.1 of Directive 10/24 the taking of long service leave is subject to entity convenience, that requests are not to be unreasonably refused, and that clause 7.4 requires a response "in a timely manner indicating whether the leave applied for has been approved or not". I have had neither.

- 8. Consultation and risk.** The workplace psychosocial risk assessment sought under clause 7.2 on 28 July 2026 has not been conducted, and the task-by-task match of the role description against the Employee Capability Checklist, sought the same day, has not been produced.

4. THE AGREEMENT'S REQUIREMENTS

Clause 1.11.4 provides that "the status quo existing before the emergence of a dispute is to continue whilst the procedure is being followed". The status quo before 3 August 2026 was that I was employed, at 76 hours per fortnight, and paid. That has not been maintained: since the dispute was notified, my absence has been coded to a leave type that pays nothing, leave has been applied at half my hours, and my long service leave eligibility has been deferred.

Clause 1.11.5 provides that "no party shall act in a manner unreasonably or intentionally delay the timely resolution of a dispute".

Clause 1.11.2(b) provides that the Stage 2 process "should not extend beyond seven days". I ask that the conference be arranged and held on or before **Monday 14 September 2026**.

5. WHAT I SEEK

1. A determination of my application for special leave on full pay from 3 July 2026, by you or by your delegate under the HR Delegations Manual, in writing, with reasons addressing the considerations in clause 6.5 of Directive 12/24.
2. Payment of the wages withheld since 13 July 2026; alternatively, the leave I have applied for entered without debit to my accrued credits, and the ad hoc payment pending under enquiry 4471091 processed against it.

3. Correction of the leave coding from the first date it was applied, and before long service leave eligibility is recalculated in the leave audit opened on 3 September 2026.
4. Correction of the entry of 30 August 2026 recorded as "Absent Without Leave", and identification of the basis on which it was made and the officer who made it.
5. Leave and pay applied at 76 hours per fortnight, or the instrument relied on for any lesser figure.
6. A written decision either returning me to work within the restrictions recorded on 3 July 2026 by a stated date, or declining to, identifying the instrument relied on and the officer who decided it.
7. A copy of the written request and supporting documents on which each "Sick Leave – No Pay" entry since 3 July 2026 was made, as required by section 6 of HR Policy C13.
8. Determination of my application for long service leave of 31 August 2026, as clause 7.4 of Directive 10/24 requires, and identification of the provision relied on to exclude any period from continuous service.
9. The workplace psychosocial risk assessment sought under clause 7.2 on 28 July 2026, and the task-by-task match of the role description against the Employee Capability Checklist sought the same day.

6. IF THE DISPUTE REMAINS UNRESOLVED

If the dispute is not resolved at Stage 2, I reserve my position under clause 1.11.2(c) to refer the matter to the EB12 Implementation Group, and under clause 1.11.2(d) to refer it to the Queensland Industrial Relations Commission.

Everything I have applied for remains on the basis set out at paragraph 4 of my application of 31 August 2026 — under protest and subject to re-credit. I remain certified fit with restrictions and available to work.

This notice concerns my employment only. My appeal in the Queensland Industrial Relations Commission is a separate proceeding and nothing in this notice forms part of it.

Attached, and forming part of this notice: "The employment record — analysis, complete chronology and original documents". It sets out what the record shows, a chronology of every communication event on this matter from February to 3 September 2026, and behind it, in fifty-one tabs, the original documents themselves — the movement forms, the Employee Capability Checklist, the Request for Medical Information and its attachments, the Stage 1 notice and its acknowledgement, the pay advice, the payroll and leave records, and the whole of the correspondence. Nothing in it has been retyped. A supplement follows the record containing the two emails of 4 September 2026 by which the ad hoc payment and the long service leave calculation were advised. The leave takings report of 25 March 2019 to 4 September 2026, referred to above, was produced by the Health Service and is held by it.

Cory Shepherd

Employee number 388372 · 0417 400 227 · coryshepherd1@hotmail.com

THE EMPLOYMENT RECORD — ANALYSIS, COMPLETE CHRONOLOGY AND ORIGINAL DOCUMENTS

Cory Shepherd, employee number 388372 · AO3, Switchboard Services, Logan Hospital, Metro South Health · February to 3 September 2026 · compiled 3 September 2026

PART A — WHAT THE RECORD SHOWS

- 1. No instrument has ever been identified.** From 3 July 2026 Mr Shepherd was directed away from the workplace. He asked for the source of that power in the Stage 1 process on 4 August 2026 and repeatedly afterwards. Six positions have been advanced in nine weeks — sick leave by default (2 July); the QSuper claim manager (15 July); further medical information (29 July); the delegate's approval of the RFMI (30 July); "reduced roster 0.5 FTE" (31 August); and the pay cycle (3 September). None identifies a power to direct a fit and available employee away from work, and none of them is a decision.
- 2. No decision has ever been made.** No written decision returning Mr Shepherd to work within the restrictions of 3 July 2026, or declining to, identifying the instrument and the officer, has ever been issued. The application for special leave on full pay made on 28 July and 5 August 2026 has been neither granted nor refused.
- 3. The employer's own medical document contradicts the exclusion.** The Employee Capability Checklist of 3 July 2026 was completed on Metro South Health's own form. It certifies fitness with restrictions and records that "usual switchboard operational duties remain suitable", excluding only complaint handling — and even then complaints are to be "logged and redirected, not actioned or resolved". It records the recommended pattern as one Mr Shepherd "has in fact worked and tolerated over the past twelve months". The task-by-task match of the role description against that document, demanded on 28 July 2026, has never been produced.
- 4. The employer's own delegate contradicts the hours.** Scott Hughes approved reduced part-time hours on 27 February, 17 April and 9 June 2026, and then described the employment as "Permanent Full-time (76 hours per fortnight)" in the RFMI of 31 July 2026. The last reduced-hours arrangement expired on 28 June 2026 by its own terms and nothing replaced it. The leave applied on 31 August 2026 was nevertheless applied at "0.5 FTE".
- 5. The leave type was chosen, not defaulted to.** Each "Sick Leave – No Pay" entry is a positive act — a leave type selected from those available and submitted on the employee's behalf. The alternatives were available and known to the person entering them: on 4 August 2026 the recreation leave balance was identified as 41.21 hours and an undertaking given to apply it "to ensure you receive the available paid leave entitlement for this period". Of the leave types in play, three produce a payment and one does not. The one that does not was selected, and selected again on each subsequent day, after the written instruction of 3 July 2026 and after every objection since.
- 6. The entries were unauthorised on the employer's own policy.** HR Policy C13 (QH-POL-188) section 6 permits a line manager to submit leave on an employee's behalf only where "the employee's request for leave is documented in writing" and supporting documents "are sourced and retained". The written requests on the record are annual leave (3 July) and special leave on full pay (5 August). There is no written request for sick leave without pay, and on 31 August 2026 Mr Shepherd confirmed in terms that he was not applying for personal leave.
- 7. Payment was never declined — only its source.** The email of 5 August 2026 declines recreation leave and in the same sentence nominates special leave on full pay instead, which under Directive 12/24 is not debited from any leave account. What was declined was the funding of an employer-directed absence out of the employee's own accrued credits.
- 8. The obligation to correct was mandatory and was not discharged.** HR Policy C13 section 2 provides that a line manager notified of an incorrect wage payment "must take all steps to rectify the error including ... preparing and submitting further forms ... liaising with Payroll to correct the error". Notice was given on 3 July 2026 and on fourteen occasions since. The corrective form was submitted once, for one fortnight, on 31 August 2026.
- 9. The dispute procedure has not been followed.** The Stage 1 notice was given on 3 August 2026. Clause 1.11.2(a) requires Stage 1 discussions within 24 hours and provides that the procedure "should not extend beyond seven days". The 24-hour requirement was conceded on 4 August 2026 as not achieved. The seven days expired on 10 August 2026. No outcome has ever been communicated. Clause 1.11.4 provides that "the status quo existing before the emergence of a dispute is to continue whilst the procedure is being followed"; the status quo before 3 August 2026 has not been maintained. Clause 1.11.5 provides that "no party shall act in a manner unreasonably or intentionally delay the timely resolution of a dispute".
- 10. The defect now supplies its own justification.** Periods recorded as leave without pay are excluded from continuous service. On 3 September 2026 Payroll advised that long service leave eligibility arises "only from 04/09/2026" — 163 days after the date on which seven years' continuous service fell due on the service dates

recorded in the agreement under which Mr Shepherd was reinstated. The coding objected to since 3 July has deferred the entitlement now relied on to refuse the leave, and it moves a day further out for each day it continues.

- 11. The loss.** Three pay days have passed with nil paid: 29 July, 12 August and 26 August 2026. At the substantive 76 hours per fortnight and the base rate of \$44.46 per hour, a fortnight is \$3,378.96 gross. Three full fortnights unpaid is \$10,136.88, and the shortfall on the fortnight 17–30 August, processed at half hours, is a further \$1,689.48 — approximately \$11,800 gross to 30 August 2026, exclusive of shift penalties, superannuation and leave accrual. The consequence, stated to the employer in writing on 31 August and 2 September 2026, is the loss of accommodation.

PART B — THE COMPLETE CHRONOLOGY

Every communication event on the employment matter, in date order. The Tab column gives the page range of the original document in Part C. Events without a tab are recorded from the correspondence itself or from contemporaneous note.

Date	Time	Between	What happened	Tab
27 Feb 2026		Hughes (delegate)	Change to Working Conditions approved — 56.00 hrs/fortnight, 1–15 March 2026, employment basis recorded as Part Time.	1 pp 8–11
17 Apr 2026		Hughes (delegate)	Change to Working Conditions approved — 56.00 hrs/fortnight, 16 March – 26 April 2026.	2 pp 12–15
9 Jun 2026		Hughes (delegate)	Change to Working Conditions approved — 40.00 hrs/fortnight, 25 May – 28 June 2026. Comment: "Temp request until 28.06.26 until further review and documentation to be provided to support request."	3 pp 16–19
28 Jun 2026		—	The last reduced-hours arrangement expires by its own terms. No instrument replaces it. The substantive appointment of 76 hours per fortnight resumes.	—
2 Jul 2026	2:38 pm	Taylor → Shepherd	"Unless you advise me otherwise, I will process your shifts that you have not worked as Sick Leave — please confirm by Friday 3rd July before 2:00pm."	4 pp 20–21
3 Jul 2026		Dr Ma	Employee Capability Checklist completed on Metro South Health's own form: fit with restrictions; "Usual switchboard operational duties remain suitable"; complaint handling excluded; six 8-hour shifts per fortnight, "the pattern Mr Shepherd has in fact worked and tolerated over the past twelve months".	5 pp 22–26
3 Jul 2026	2:49 pm	Taylor → Shepherd	"As you have not confirmed leave type ... I will continue to process as Sick leave / Sick leave no pay."	6 pp 27–30
3 Jul 2026	3:18 pm	Shepherd → Taylor	"Could you please process the balance as ANNUAL LEAVE rather than sick leave without pay." Twenty-nine minutes after the message saying the leave type had not been confirmed.	—
3 Jul 2026		—	Held out of the workplace from this date. "Sick Leave – No Pay" applied to the record every day thereafter.	—
7 Jul 2026	10:54 am	Forrest → Shepherd	ECC further information sought.	7 pp 31–36
10 Jul 2026	8:35 am	Shepherd → Forrest	Asks that the shifts "be treated as PAID TIME and ... not deducted from my leave"; asks that the return be facilitated. Unanswered.	8 pp 37–48
13 Jul 2026	3:53 pm	Injury Management	Responds by reference to the QSuper income protection claim.	—
13 Jul 2026	4:39 pm	Shepherd → Injury Mgmt	"I ask again" — paid time; the basis in writing if refused. Interim duties offered (finance; scanning and records).	—
13 Jul 2026		—	LAST WAGES RECEIVED. No payment on any pay day after this date.	—
14–15 Jul 2026		Moran (Together)	"VERY SERIOUS" — urgent referral to Together's industrial team.	9 pp 49–53
15 Jul 2026	1:10 pm	MSH → Shepherd, Zappia	QSuper income protection claim update.	10 pp 54–61
15 Jul 2026		Harrison → Shepherd	Pay refused — it "sits with your QSuper Claim Manager". Interim duties refused "in any capacity". Accommodation of restrictions "remains at the discretion of the Employer".	11 pp 62–69
17 Jul 2026		Moran (Together)	Brief delivered in full; the industrial team is engaged and still reviewing.	12 pp 70–79
20 Jul 2026		Taylor (in myHR)	Leave of the type "Income Protection – No Pay" requested on Mr Shepherd's behalf by C Donovan-Taylor (371009).	13 pp 80–80
22 Jul 2026		Shepherd → Payroll	Payroll Enquiry 4438861 lodged. Status "Pending Investigation".	14 pp 81–81
28 Jul 2026	5:39 pm	Shepherd → HR, cc Moran	Incorrect application of EB12, the Award and QH policies — eight numbered instruments. Special leave on full pay applied for (point 4). Workplace psychosocial risk assessment demanded under cl 7.2. Task-by-task match of the role description to the ECC demanded. Delegate and instrument demanded for each day's coding since 3 July.	15 pp 82–87
29 Jul 2026	4:10 pm	MSH → Shepherd, cc Moran	Reply: further medical information required before capacity can be determined. Special leave not addressed.	16 pp 88–96
29 Jul 2026		—	PAY DAY — nil. Fortnight 6–19 July.	—
30 Jul 2026	11:25 am	Shepherd → MSH, cc Moran	Roster, pay and leave "remain live in parallel". First request that the shifts from 3 July be processed by ad hoc payment and the leave re-credited.	17 pp 97–114

Date	Time	Between	What happened	Tab
30 Jul 2026	2:33 pm	MSH → Shepherd, cc Moran	Holding reply — the Request for Medical Information is with the delegate for approval. Special leave not addressed.	18 pp 115–135
31 Jul 2026	11:43 am	Harrison → Shepherd	RFMI covering email.	19 pp 136–138
31 Jul 2026		Hughes → Shepherd	RFMI letter to the employee, signed by Scott Hughes as delegate, describing the employment as "Permanent Full-time (76 hours per fortnight)" — seven weeks after the same delegate approved reduced part-time hours for the third time.	20 pp 139–140
31 Jul 2026		Hughes → Dr Ma	RFMI letter to the treating general practitioner — nine questions.	21 pp 141–145
31 Jul 2026		MSH	RFMI attachment — AO3 Switchboard role description.	22 pp 146–149
31 Jul 2026		MSH	RFMI attachment — authorisation form (option 2, unsigned).	23 pp 150–150
31 Jul 2026	3:15 pm	Harrison → Shepherd	Reimbursement of the 3 July consultation gated on confirmation from the practice.	24 pp 151–153
3 Aug 2026	11:38 am	Shepherd → IM, HR, cc Moran	RFMI response.	25 pp 154–157
3 Aug 2026	11:38 am	Shepherd → IM, HR, cc Moran	Request under clause 10.3.	26 pp 158–158
3 Aug 2026	11:38 am	Shepherd → Taylor, IM, HR, cc Moran	NOTICE OF DISPUTE under EB12 cl 1.11 — Stage 1. Attendance, roster, leave and pay from 26 June 2026.	27 pp 159–159
3 Aug 2026	11:38 am	Shepherd → Chief Executive	Letter to the Health Service Chief Executive.	28 pp 160–161
3 Aug 2026	12:45 pm	MSH Correspondence	Acknowledges the Chief Executive letter at Mr Cridland's direction.	29 pp 162–164
4 Aug 2026	1:35 pm	Taylor → Shepherd, IM, HR, cc Moran	STAGE 1 ACKNOWLEDGEMENT. Concedes the cl 1.11.2(a) 24-hour timeframe "will not be achieved". States "I am required to process leave on your behalf" for 20 July – 2 August. Records the recreation leave balance at 41.21 hours. Undertakes that absent a response she "will apply your remaining recreation leave balance to ensure you receive the available paid leave entitlement for this period".	30 pp 165–168
4 Aug 2026		Shepherd	Asks in the Stage 1 process for the instrument authorising the exclusion. Never answered.	—
5 Aug 2026	7:30 am	Shepherd → Taylor, IM, HR, cc union	"I do not consent to my recreation leave being applied, and I have asked that the period be coded as special leave on full pay instead."	31 pp 169–173
10 Aug 2026		—	The seven days allowed for Stage 1 by cl 1.11.2(a) expire. No outcome has ever been communicated.	—
11 Aug 2026	4:31 pm	Petering (Together)	Acknowledges the s 89 letter forwarded; telephone contact.	32 pp 174–175
12 Aug 2026		Payroll	PAY DAY — nil. Pay advice for the fortnight 20 July – 2 August 2026 records "NP Sick Leave" 76.00 hours, gross \$0.00, net \$0.00.	33 pp 176–176
12 Aug 2026	6:20 pm	Shepherd → LBH	RFMI reply as sent.	34 pp 177–179
13 Aug 2026	3:48 pm	Harrison → Shepherd	Reply — consent and the Employee Authority Form.	35 pp 180–185
13 Aug 2026	4:42 pm	Roberts → Shepherd	"Follow up on Enquiries" — subsequently RECALLED.	36 pp 186–188
13 Aug 2026	4:45 pm	Roberts	Recall notice.	37 pp 189–189
18 Aug 2026	10:47 am	Roberts → Shepherd	The recalled email re-sent to a personal address.	38 pp 190–193
18–28 Aug 2026		Roberts ↔ Shepherd, cc Petering	"Follow up on Enquiries" thread. 24 August 11:13 am: "the appropriate LEAVE BE APPLIED and ... an AVAC be submitted". 28 August 2:18 pm: the Stage 2 referral withdrawn — the referral only, with pay, the leave debited since 3 July and the ECC adjustments expressly "unresolved and in dispute"; AVAC update sought; Dr Krishnaiah reports Queensland Health has not responded to his request for information.	39 pp 194–205
21 Aug 2026		Roberts, McQuillan, Petering, Shepherd	Conference — two hours, five agenda items, the union present. No outcome. Cut short.	—
25 Aug 2026	3:08 pm	Roberts → Shepherd, McQuillan, Petering	Teams invitation to continue the conference.	40 pp 206–208
26 Aug 2026		—	PAY DAY — nil. Fortnight 3–16 August.	—

Date	Time	Between	What happened	Tab
28 Aug 2026		Taylor	Absence notifications record.	41 pp 209–221
31 Aug 2026	10:59 am	Shepherd → Taylor, cc Roberts, Petering	Formal application: long service leave on full pay from 13 July 2026; annual leave on its exhaustion; the AVAC to be processed today; "I confirm I am not applying for personal leave"; made under protest and subject to re-credit; financial hardship and the loss of accommodation stated.	42 pp 222–228
31 Aug 2026	3:34 pm	Taylor → Shepherd	AVAC processed for one fortnight only (17–30 August), "S/L and A/L in lieu of S/L", "as you are on reduced roster 0.5 FTE". Long service leave "the system is currently not allowing me to apply".	—
31 Aug 2026	4:06 pm	Shepherd → Taylor	Four-point correction: leave type; 76 hours not 0.5 FTE; the period from 13 July; the service dates.	—
2 Sep 2026	11:07 am	Shepherd → Taylor	Ad hoc payment requested to be processed today. Fifty-one days without wages; accommodation given up; unable to log into myHR.	43 pp 229–243
2 Sep 2026	12:42 pm	Taylor → Shepherd	PRN processed for the fortnight ending 30 August, to be paid 9 September; "S/L and A/L in lieu of S/L" applied; leave balances given.	—
2 Sep 2026	2:05 pm	Shepherd → Payroll, Taylor	myHR access regained; no record of the PRN visible; Payroll advised by telephone at 1:24 pm that no information was recorded against it; copies of the enquiries requested.	—
2 Sep 2026	4:16 pm	Taylor → Shepherd	Process number corrected to 24926366. Long service leave enquiry 4469554 lodged 31 August, "Pending Investigation".	—
2 Sep 2026	5:29 pm	Roberts → Shepherd (SMS)	In answer to a request for a call and for help being paid because of a need for food: "I know that Chloe has been working hard on this today for you. I am sure she will get back to you asap tomorrow about this."	44 pp 244–244
3 Sep 2026	9:05 am	Payroll (Layson)	"You aren't able to put in LSL leave as Cory's LSL eligibility date is only from 04/09/2026." A log has been created for a leave audit.	—
3 Sep 2026	10:27 am	Taylor → Shepherd, Payroll	Long service leave "cannot be backdated & paid prior to the date you become eligible". The ad hoc payment for 17–30 August "cannot be processed prior to Wednesday 9th September 2026".	—
3 Sep 2026	1:30 pm	Shepherd → Roberts, Taylor, Payroll, cc Petering	The two-part response — the coding since 3 July, and payment under HR Policy C13 §§2, 6, 8 and 9; long service leave from 13 July; special leave under Directive 12/24.	45 pp 245–264
3 Sep 2026	2:04 pm	Shepherd → Layson (Payroll)	Who within Payroll has authority to action an ad hoc payment today; or the name, position, telephone and email of the officer who can; and if refused, the reason in writing, the policy relied on and the decision-maker.	46 pp 265–266
3 Sep 2026	2:10 pm	Petering (Together)	"I am currently reviewing the 14 emails I've received on your matter this week ... reply to you with answers to your specific questions ... by the end of the week."	47 pp 267–273
3 Sep 2026	2:52 pm	Beetham (Together)	Referral to the Workers' Psychological Support Service, described as assisting with work-related injury "including a lack of support from management", and providing financial counselling.	—
3 Sep 2026		Harrison → Shepherd	The reimbursement paperwork for the 3 July consultation to be completed today; the clinic's invoice was received 2 September.	48 pp 274–277
2–3 Sep 2026		myHR records	Payroll enquiries: 4471091 Ad Hoc Payment, 2 September, Pending; 4438861 Leave, 22 July, shown Resolved though never communicated; 4045255 Ad Hoc Payment, 11 March 2025, Resolved. Leave balances and the daily "Sick Leave – No Pay" entries.	49 pp 278–278
2–3 Sep 2026		myHR records	Leave record. Balances as at 2 September 2026: Recreation Leave 51.31 hours; Sick Leave 14.66 hours. Request overview showing "SICK LEAVE - NO PAY" entered as DISCRETE DAILY LEAVE REQUESTS, each separately APPROVED — including 03.08.2026 and 04.08.2026.	50 pp 279–279
Jul–Sep 2026		Switchboard manager	Payment correspondence record.	51 pp 280–288

PART C — THE ORIGINAL DOCUMENTS

Each tab is the original document as sent or received. Nothing has been retyped.

Tab	Document	Pages
1	2026-02-27_Movement_56hrs_1Mar-15Mar_HughesApproved.pdf	8–11
2	2026-04-17_Movement_56hrs_16Mar-26Apr_HughesApproved.pdf	12–15
3	2026-06-09_Movement_40hrs_25May-28Jun_HughesApproved.pdf	16–19
4	2026-07-02_Taylor_ECC_LeaveType_ORIGIN_DOCUMENT.pdf	20–21
5	medical/Employee_Capability_Checklist_CShepherd_03-07-2026.pdf	22–26
6	correspondence-2026/2026-07-03_1518_Cory_to_Taylor_ECC_LeaveType_reply.pdf	27–30
7	2026-07-07_Forrest_ECC_further_information_FULL.pdf	31–36
8	correspondence-2026/2026-07_Cory_to_InjuryMgmt_ECC_further_information_FULL_THREAD_12pp.pdf	37–48
9	2026-07-15_Together_Heath-Moran_thread_urgent-industrial-referral.pdf	49–53
10	correspondence-2026/2026-07-15_1310_MSH_to_Cory_and_Zappia_QSuper_IP_claim_update.pdf	54–61
11	2026-07-15_Harrison_reply_pay_refused_discretion.pdf	62–69
12	2026-07-17_Heath_still-reviewing_industrial-team.pdf	70–79
13	2026-07-20_myHR_IncomeProtection_NoPay_requested_by_Taylor.pdf	80
14	2026-07-22_Payroll_Enquiry_4438861_PendingInvestigation.pdf	81
15	correspondence-2026/2026-07-28_1739_Cory_to_HR_incorrect_application_EB12_Award_policies.pdf	82–87
16	correspondence-2026/2026-07-29_1610_MSH_reply_to_28July_letter.pdf	88–96
17	correspondence-2026/2026-07-30_1125_Cory_reply_to_MSH_29July.pdf	97–114
18	correspondence-2026/2026-07-30_1433_MSH_holding_reply_delegate_approval.pdf	115–135
19	2026-07-31_Harrison_RFMI_covering_email_1143.pdf	136–138
20	2026-07-31_ScottHughes_RFMI_letter_to_EMPLOYEE.pdf	139–140
21	2026-07-31_ScottHughes_RFMI_letter_to_GP_DrMa_9questions.pdf	141–145
22	2026-07-31_RFMI_Attachment2_AO3_Switchboard_Role_Description.pdf	146–149
23	2026-07-31_RFMI_Authorisation_Form_OPTION2_NOT_SIGNED.pdf	150
24	correspondence-2026/2026-07-31_1515_Harrison_reimbursement_practice-confirmation_gate.pdf	151–153
25	sent-2026-08-03/2026-08-03_1138_SENT_Email1_RFMI_Response_to_IM_cc_HR_Moran.pdf	154–157
26	sent-2026-08-03/2026-08-03_1138_SENT_Email2_Request_cl10.3_to_IM_and_HR_cc_Moran.pdf	158
27	sent-2026-08-03/2026-08-03_1138_SENT_Email3_Dispute_cl1.11_to_Taylor_IM_HR_cc_Moran.pdf	159
28	sent-2026-08-03/2026-08-03_1138_SENT_Email4_CE_to_Cridland_cc_Corro.pdf	160–161
29	sent-2026-08-03/2026-08-03_1245_MSH_Corro_ACK_CE_letter_at_Cridlands_direction.pdf	162–164
30	correspondence-2026/2026-08-04_1335_Taylor_Stage1_ack_24h_not_achieved_and_leave_request.pdf	165–168
31	correspondence-2026/2026-08-05_0730_SENT_Stage1_reply_cover_email.pdf	169–173
32	2026-08-11_1631_Petering_ack_s89_letter_forwarded.pdf	174–175
33	2026-08-12_Payslip_ZERO_NPSickLeave_76hrs.pdf	176
34	sent-2026-08-12/2026-08-12_1820_Cory_to_LBH_RFMI_reply_AS_SENT.pdf	177–179
35	2026-08-13_1548_Harrison_reply_consent_and_EAF.pdf	180–185
36	2026-08-13_1642_Roberts_FollowUpOnEnquiries_RECALLED.pdf	186–188
37	2026-08-13_1645_Roberts_RECALL_NOTICE.pdf	189
38	2026-08-18_1047_Roberts_FollowUpOnEnquiries_RESENT_personal_email.pdf	190–193
39	correspondence-2026/2026-08-18_to_28_Roberts_FollowUpOnEnquiries_FULL_THREAD.pdf	194–205
40	correspondence-2026/2026-08-25_1508_Roberts_Teams_invite.pdf	206–208
41	2026-08-28_Taylor_absence_notifications_Att9.pdf	209–221
42	correspondence-2026/2026-08-31_FULL_THREAD_v2_Taylor_LSL_AVAC_incl_1606_correction.pdf	222–228
43	correspondence-2026/2026-09-03_1027_Taylor_Payroll_LSL_eligibility_04SEP_thread_2-3Sep.pdf	229–243
44	correspondence-2026/2026-09-02_1729_SMS_Roberts_to_Cory_Chloe_working_on_it.jpg	244
45	correspondence-2026/2026-09-03_1330_Cory_to_Roberts_Taylor_Payroll_TWO_PARTS_coding_and_payment_SENT.pdf	245–264
46	correspondence-2026/2026-09-03_1404_Cory_to_Layson_Payroll_urgent_ad_hoc_authority_SENT.pdf	265–266
47	correspondence-2026/2026-09-03_1452_Together_Beetham_WPSS_referral_and_Petering_holding_reply.pdf	267–273
48	correspondence-2026/2026-09-03_0738_Harrison_ECC_reimbursement_paperwork_to_be_completed_today.pdf	274–277
49	correspondence-2026/2026-09_My_payroll_enquiries.pdf	278

Tab	Document	Pages
50	correspondence-2026/2026-09_Leave.pdf	279
51	correspondence-2026/2026-09_Manager_Switchboard_Payment_Corro.pdf	280–288

Movement Details

Movement Task: Change to Working Conditions (Temporary)
Effective Period: 01.03.2026 - 15.03.2026
Movement Reason: Change in Hours - Other
Position ID: 32002276
Position Title: AO Switchboard
Person ID: 388372
Person (Preferred) Name: Cory Shepherd
Personnel Assignment Number: 388372
Organisational Unit ID: 72010875
Organisational Unit Name: LOGH CORP Switchboard Operations

Employment Conditions

Award/CBA/Contract: Hospital and Health Service General Employees (Queensland Health) Award – State 2015
Classification: A03
Level: 04
Employment Basis: Part Time
Number Of Fortnightly Hours: 56.00
Concurrent/Aggregate Current Status: Not Applicable
Concurrent/Aggregate Proposed Status: ☐ Indicate here if the employee will be working in multiple positions at the same time resulting in Concurrent or Aggregate employment.
Working Arrangement: Standard Hours
Shift Arrangements: Continuous Shift Worker
Reason For Additional Leave: Working Public Holidays, Continuous Shift Work
Recreation Leave Accrual (weeks): 6

Work Schedule

Roster Type: Non-Cyclic

Other

Comments: Cory Shepherd to extend reduce hours contract - 1.03.26 - 15.03.26 possible extension

Manager Certification:



I certify that I have:

*(Where the employee is seeking release or extension of a previously approved movement from another work unit) successfully negotiated the terms of the agreement with the line manager of the employee's substantive position

*Informed this employee of any changes to their FBT Concession Eligibility status as a consequence of this variation to their employment contract

*Discussed with this employee the consequences of this change to their position, employment status, terms of employment and/or roster

*Informed the employee where this change applies to a temporary employee moving between temporary assignments, of any impact (i.e. the ending or likelihood of extension of their previous contract) as a consequence of accepting appointment to this proposed position and

*Undertaken all required checks and cited all relevant documentation including, where required, verification against the AHPRA Registers of Practitioners.

*Completed all necessary pre-employment checks with evidence attached and have submitted registration and Working with children checks to Payroll Services for processing.

*Where the employee is required to work in a position which requires evidence of vaccination or proof that they are not susceptible to specified vaccine preventable diseases (VPDs); that the relevant vaccination status in accordance to their assigned position has been sighted.

Note: Vaccination records must be retained in a manner that conforms with both the *Information Privacy Act 2009 (QLD)* and the *Public Records Act 2002 (QLD)*

Chloe Donovan-Taylor

19.02.2026

Employee:

I agree to the above changes to my employment hours/position. I hereby claim for the extra remuneration for hours worked in a higher duties capacity (where applicable). I also certify that I have been informed by my line manager/supervisor of the consequences of this change to my:

* FBT Concession Eligibility status that may result from this variation to my employment contract and

* Position, employment status, terms of employment and/or roster. I also acknowledge that as this appointment is of a temporary nature, the contract may be ended by my line manager with the appropriate notice in accordance with award provisions.

I understand that my current existing superannuation scheme and/or contribution arrangements will continue to apply to my new health employer within Queensland Health (where applicable), with any future changes to superannuation arrangements to be applicable to current and past employer entities within Queensland Health, unless otherwise specifically advised.

Temporary employees only:

Your appointment is for a temporary period. Continuation or extension of employment in this position cannot be guaranteed beyond the end date stated above. Please note that your employment may be terminated prior to the end date of your fixed term appointment, by either party, in accordance with the termination clause in the above mentioned Award or other industrial instruments relevant to your employment.

Disclosure of personal information:

I understand that personal information in relation to my employment may be disclosed by my current health employer to another health or government agency, subject to applicable law, in connection with my movement/change to a role or employment with (or any application for or assessment of me having a role or employment with) that other health or government agency.

Cory Shepherd

26.02.2026

Reviewer Group:

I, the authorised Endorser for this appointment confirm that all impacts have been reviewed and confirmed.

Paul Ianna

27.02.2026

Delegate:



If applicable review the changes to the employee recurring allowances contained in this application.

I, the authorised delegate for appointments, approve:
*any increase above the position's established FTE as a consequence of this appointment, and
*the above appointment subject to the receipt of acceptable criminal history report (where necessary) in accordance with *Section 67 of the Public Service Act 2008 or Section 67 of the Hospital and Boards Act 2001* [whichever is applicable].

HES/SES higher duties only:
Will the employee be allocated a government owned motor vehicle for private use or home garaging during this period of relief?

Scott Hughes

27.02.2026

Movement Details

Movement Task: Change to Working Conditions (Temporary)
Effective Period: 16.03.2026 - 26.04.2026
Movement Reason: Change in Hours - Other
Position ID: 32002276
Position Title: AO Switchboard
Person ID: 388372
Person (Preferred) Name: Cory Shepherd
Personnel Assignment Number: 388372
Organisational Unit ID: 72010875
Organisational Unit Name: LOGH CORP Switchboard Operations

Employment Conditions

Award/CBA/Contract: Hospital and Health Service General Employees (Queensland Health) Award – State 2015
Classification: A03
Level: 04
Employment Basis: Part Time
Number Of Fortnightly Hours: 56.00
Concurrent/Aggregate Current Status: Not Applicable
Concurrent/Aggregate Proposed Status: ☐ Indicate here if the employee will be working in multiple positions at the same time resulting in Concurrent or Aggregate employment.
Working Arrangement: Standard Hours
Shift Arrangements: Continuous Shift Worker
Reason For Additional Leave: Working Public Holidays, Continuous Shift Work
Recreation Leave Accrual (weeks): 6

Work Schedule

Roster Type: Non-Cyclic

Other

Comments: Cory Shepherd reduced hours from 1FTE to 0.7fte per fortnight 16.03.2026 - 26.04.2026 with possible extension after review.

Manager Certification:



I certify that I have:

*(Where the employee is seeking release or extension of a previously approved movement from another work unit) successfully negotiated the terms of the agreement with the line manager of the employee's substantive position

*Informed this employee of any changes to their FBT Concession Eligibility status as a consequence of this variation to their employment contract

*Discussed with this employee the consequences of this change to their position, employment status, terms of employment and/or roster

*Informed the employee where this change applies to a temporary employee moving between temporary assignments, of any impact (i.e. the ending or likelihood of extension of their previous contract) as a consequence of accepting appointment to this proposed position and

*Undertaken all required checks and cited all relevant documentation including, where required, verification against the AHPRA Registers of Practitioners.

*Completed all necessary pre-employment checks with evidence attached and have submitted registration and Working with children checks to Payroll Services for processing.

*Where the employee is required to work in a position which requires evidence of vaccination or proof that they are not susceptible to specified vaccine preventable diseases (VPDs); that the relevant vaccination status in accordance to their assigned position has been sighted.

Note: Vaccination records must be retained in a manner that conforms with both the *Information Privacy Act 2009 (QLD)* and the *Public Records Act 2002 (QLD)*

Chloe Donovan-Taylor

09.04.2026

Employee:

I agree to the above changes to my employment hours/position. I hereby claim for the extra remuneration for hours worked in a higher duties capacity (where applicable). I also certify that I have been informed by my line manager/supervisor of the consequences of this change to my:

* FBT Concession Eligibility status that may result from this variation to my employment contract and

* Position, employment status, terms of employment and/or roster. I also acknowledge that as this appointment is of a temporary nature, the contract may be ended by my line manager with the appropriate notice in accordance with award provisions.

I understand that my current existing superannuation scheme and/or contribution arrangements will continue to apply to my new health employer within Queensland Health (where applicable), with any future changes to superannuation arrangements to be applicable to current and past employer entities within Queensland Health, unless otherwise specifically advised.

Temporary employees only:

Your appointment is for a temporary period. Continuation or extension of employment in this position cannot be guaranteed beyond the end date stated above. Please note that your employment may be terminated prior to the end date of your fixed term appointment, by either party, in accordance with the termination clause in the above mentioned Award or other industrial instruments relevant to your employment.

Disclosure of personal information:

I understand that personal information in relation to my employment may be disclosed by my current health employer to another health or government agency, subject to applicable law, in connection with my movement/change to a role or employment with (or any application for or assessment of me having a role or employment with) that other health or government agency.

Cory Shepherd**13.04.2026****Reviewer Group:**

I, the authorised Endorser for this appointment confirm that all impacts have been reviewed and confirmed.

Paul Ianna**15.04.2026**

Delegate:



If applicable review the changes to the employee recurring allowances contained in this application.

I, the authorised delegate for appointments, approve:
*any increase above the position's established FTE as a consequence of this appointment, and
*the above appointment subject to the receipt of acceptable criminal history report (where necessary) in accordance with *Section 67 of the Public Service Act 2008 or Section 67 of the Hospital and Boards Act 2001* [whichever is applicable].

HES/SES higher duties only:

Will the employee be allocated a government owned motor vehicle for private use or home garaging during this period of relief?

Scott Hughes

17.04.2026

Movement Details

Movement Task: Change to Working Conditions (Temporary)
Effective Period: 25.05.2026 - 28.06.2026
Movement Reason: Change in Hours - Other
Position ID: 32002276
Position Title: AO Switchboard
Person ID: 388372
Person (Preferred) Name: Cory Shepherd
Personnel Assignment Number: 388372
Organisational Unit ID: 72010875
Organisational Unit Name: LOGH CORP Switchboard Operations

Employment Conditions

Award/CBA/Contract: Hospital and Health Service General Employees (Queensland Health) Award – State 2015
Classification: AO3
Level: 04
Employment Basis: Part Time
Number Of Fortnightly Hours: 40.00
Concurrent/Aggregate Current Status: Not Applicable
Concurrent/Aggregate Proposed Status: ☐ Indicate here if the employee will be working in multiple positions at the same time resulting in Concurrent or Aggregate employment.
Working Arrangement: Standard Hours
Shift Arrangements: Continuous Shift Worker
Reason For Additional Leave: Working Public Holidays, Continuous Shift Work
Recreation Leave Accrual (weeks): 6

Work Schedule

Roster Type: Non-Cyclic

Other

Comments: Cory Shepherd request to reduce contracted hours from 1.0FTE - 0.5TE 40hrs per fortnight.
Temp request until 28.06.26 until further review and documentation to be provided to support request.

Manager Certification:



I certify that I have:

*(Where the employee is seeking release or extension of a previously approved movement from another work unit) successfully negotiated the terms of the agreement with the line manager of the employee's substantive position

*Informed this employee of any changes to their FBT Concession Eligibility status as a consequence of this variation to their employment contract

*Discussed with this employee the consequences of this change to their position, employment status, terms of employment and/or roster

*Informed the employee where this change applies to a temporary employee moving between temporary assignments, of any impact (i.e. the ending or likelihood of extension of their previous contract) as a consequence of accepting appointment to this proposed position and

*Undertaken all required checks and cited all relevant documentation including, where required, verification against the AHPRA Registers of Practitioners.

*Completed all necessary pre-employment checks with evidence attached and have submitted registration and Working with children checks to Payroll Services for processing.

*Where the employee is required to work in a position which requires evidence of vaccination or proof that they are not susceptible to specified vaccine preventable diseases (VPDs); that the relevant vaccination status in accordance to their assigned position has been sighted.

Note: Vaccination records must be retained in a manner that conforms with both the *Information Privacy Act 2009 (QLD)* and the *Public Records Act 2002 (QLD)*

Chloe Donovan-Taylor

27.05.2026

Employee:

I agree to the above changes to my employment hours/position. I hereby claim for the extra remuneration for hours worked in a higher duties capacity (where applicable). I also certify that I have been informed by my line manager/supervisor of the consequences of this change to my:

* FBT Concession Eligibility status that may result from this variation to my employment contract and

* Position, employment status, terms of employment and/or roster. I also acknowledge that as this appointment is of a temporary nature, the contract may be ended by my line manager with the appropriate notice in accordance with award provisions.

I understand that my current existing superannuation scheme and/or contribution arrangements will continue to apply to my new health employer within Queensland Health (where applicable), with any future changes to superannuation arrangements to be applicable to current and past employer entities within Queensland Health, unless otherwise specifically advised.

Temporary employees only:

Your appointment is for a temporary period. Continuation or extension of employment in this position cannot be guaranteed beyond the end date stated above. Please note that your employment may be terminated prior to the end date of your fixed term appointment, by either party, in accordance with the termination clause in the above mentioned Award or other industrial instruments relevant to your employment.

Disclosure of personal information:

I understand that personal information in relation to my employment may be disclosed by my current health employer to another health or government agency, subject to applicable law, in connection with my movement/change to a role or employment with (or any application for or assessment of me having a role or employment with) that other health or government agency.

Cory Shepherd**05.06.2026****Reviewer Group:**

I, the authorised Endorser for this appointment confirm that all impacts have been reviewed and confirmed.

Melissa Spink**08.06.2026**

Delegate:



If applicable review the changes to the employee recurring allowances contained in this application.

I, the authorised delegate for appointments, approve:
*any increase above the position's established FTE as a consequence of this appointment, and
*the above appointment subject to the receipt of acceptable criminal history report (where necessary) in accordance with *Section 67 of the Public Service Act 2008 or Section 67 of the Hospital and Boards Act 2001* [whichever is applicable].

HES/SES higher duties only:

Will the employee be allocated a government owned motor vehicle for private use or home garaging during this period of relief?

Scott Hughes

09.06.2026

Cory - ECC/Leave Type

From: Chloe Taylor

Chloe.Taylor3@health.qld.gov.au

To: Cory Shepherd

Cory.Shepherd@health.qld.gov.au

Cc: Cory Shepherd coryshepherd1@hotmail.com

Sent: Thursday 2 July at 14:38

Hi Cory,

I just wanted to check in to see how you are doing and how things have been progressing for you. I know you had a scheduled appointment with your doctor yesterday as per text message sent on Monday 29th July.

Did your doctor give you any indication when the required documentation will be completed? Until we receive appropriate medical clearance, including a completed Employee Capacity Certificate (ECC), we are unable to facilitate your return to work, please wait for my advice to when your next returning shift will be. Unless you advise me otherwise, I will process your shifts that you have not worked as Sick Leave – Please confirm by Friday 3rd July before 2:00pm.

Kind regards,

Chloe

Chloe Taylor

Switchboard Manager | Local Telecommunications Representative | Switchboard Services

Logan Hospital | Metro South | Queensland Health

P (07) 2891 9190

E chloe.taylor3@health.qld.gov.au

W www.health.qld.gov.au

A Level 2, Building 1, Cnr Loganlea and Armstrong Roads QLD 4131

T [Call me on Teams](#)



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Employee Capability Checklist – Cory Shepherd

*** Please note that a separate medical certificate is also required to support the completed Capability Checklist.*

Information provided in the Capability Checklist below will assist our Organisation to determine appropriate tasks in the event that the identified worker is cleared for suitable duties. Please ensure that all sections of the Capability Checklist are completed in full.

I have examined Cory Lea Shepherd (DOB 11 January 1991) and consider that they are diagnosed with the following condition:

Major Depressive Disorder with anxious distress (DSM-5 296.23) — work-related psychological injury, onset 18 June 2024.

- ☒ I have viewed the worker's Position Description (attached — Administration Officer, Switchboard Services (AO3), Logan Hospital) and/or
- ☒ I am aware of the requirements of their role

It is my opinion that they are fit to resume:

- ☐ Normal duties
- ☒ Modified duties
- ☒ Reduced hours (hours/days)
- ☐ Workplace Performance Discussions

Modified duties relate to:

- ☒ Tasks performed
- ☒ Duration

Hours to be worked

Week 1: 8 hours/day 3 days/week	Week 5: 8 hours/day 3 days/week
Week 2: 8 hours/day 3 days/week	Week 6: 8 hours/day 3 days/week
Week 3: 8 hours/day 3 days/week	Week 7: 8 hours/day 3 days/week
Week 4: 8 hours/day 3 days/week	Week 8: 8 hours/day 3 days/week
(6 shifts per 10-shift fortnight: max 2 weekday day/afternoon + 2 night + 2 weekend shifts — continuation of the reduced pattern Mr Shepherd has sustained since 2025)	

Fit to commence the above from: 3 July 2026 (continuation of existing arrangement)

Date of next review: 28 August 2026 (approximately 8 weeks)

Activity	No capacity	Restricted / As Tolerated	Normal	Details of any restrictions and reasons
COGNITIVE				
Learning new tasks	☐	☐	☒	
Ability to initiate tasks independently	☐	☐	☒	
Understanding instructions	☐	☐	☒	
Carrying out instructions accurately	☐	☐	☒	
Maintain focus/concentration	☐	☒	☐	Concentration reduced during sustained high-stress periods; managed by rostering to lower call-volume shifts.
Executing tasks within a reasonable timeframe	☐	☐	☒	
Working autonomously without supervision	☐	☐	☒	
Prioritising tasks	☐	☐	☒	
Making decisions	☐	☐	☒	
Multi-tasking	☐	☒	☐	Concurrent high-volume demand exacerbates symptoms; suitable on lower-volume shifts.
Memorising information	☐	☒	☐	Working memory affected under stress (treating psychiatrist report 13-Feb-2025).
Working in a high demand environment	☐	☒	☐	Limit weekday day/afternoon shifts — which carry the peak call volume and the majority of complaints — to no more than 2 per fortnight; remaining shifts rostered to the lower-volume weekend (2) and night (2) shifts.
RELATIONAL				
Communicating with internal/external customers	☐	☐	☒	
Working in a team environment	☐	☐	☒	
Handling conflict	☐	☒	☐	No complaint-handling duties: complaints received at the Switchboard are to be logged and redirected to management/the dedicated complaints area, not actioned, resolved or absorbed by Mr Shepherd.
Supervising others	☐	☐	☒	Not a requirement of the AO3 role.
Interacting appropriately with others	☐	☐	☒	
Do you foresee any existing stressors that could impact on a return to substantive role? If so, please provide additional comments:				Yes. (1) Complaint-handling exposure at the Switchboard; (2) being held accountable, and blamed, for failures that are the responsibility of others across the hospital, regardless of where the fault lies; (3) rostering and fatigue-management practices; (4) unresolved workplace matters involving line management. Recommend return-to-work communications and this checklist be managed through the Injury Management Team Human Resources

Medications

Is the worker currently taking any medication?

☒ Yes

☐ No

Please outline any side effects from medication currently being taken that may impact on a person's functional capacity or task performance (for example drowsiness, dizziness, confusion, impaired concentration, fatigue, slurred speech):

Current medications:

1. Fluoxetine 60 mg mane (3 × 20 mg capsules).
2. Lisdexamfetamine (Vyvanse) 30 mg mane — for cognitive symptoms and focus.

Quetiapine (Seroquel) 25 mg nocte, previously prescribed for rumination and sleep, has been ceased.

Relevant effects on task performance: maintaining a regular sleep routine remains central to symptom management following cessation of the night-time medication. This supports (a) limiting night shifts to no more than 2 per fortnight, (b) predictable rostering published in advance, and (c) a minimum 10-hour break between rostered shifts consistent with the applicable Award and fatigue risk management standards. Residual symptoms include reduced concentration and working memory under sustained high-demand load and fatigue sensitivity.

Additional Comments:

1. Please elaborate on the current medical condition Mr Shepherd is suffering from?

Major Depressive Disorder with anxious distress (DSM-5 296.23), a work-related psychological injury with onset 18 June 2024. Treatment is ongoing (fluoxetine, lisdexamfetamine) under psychiatric care. Residual symptoms most relevant to work capacity are reduced concentration and working memory under sustained high-demand load, fatigue sensitivity, and symptom exacerbation on exposure to the identified workplace stressors (complaint handling; being held accountable and blamed for the failures of others; unpredictable rostering).

2. In your opinion do you feel that Mr Shepherd's current condition is temporary, episodic or permanent in nature? What is the expected duration of the current restrictions?

Episodic, with symptom exacerbation linked to stressor exposure. The restrictions below are expected to remain necessary for at least the next 3–6 months, subject to treatment response and 8-weekly review.

3. Based on your current assessment, what work capacity (including number of shifts, hours, and type of duties) would best support Mr Shepherd's recovery while minimising risk of exacerbation?

Six 8-hour shifts per fortnight (of the ten rostered in a full-time fortnight), comprising:

(a) No more than two weekday day/afternoon shifts. These shifts carry the peak call volume and the majority of complaints — the identified stressors. Retaining two per fortnight maintains graded exposure to the full role and preserves work capacity; the cap prevents multiple high-load shifts accumulating within a fortnight, which is the pattern associated with symptom exacerbation.

(b) Two weekend day shifts. Weekend call volume and complaint exposure are materially lower while the duties are identical, allowing the same work to be performed at an intensity consistent with Mr Shepherd's current concentration and working-memory capacity, and preserving a normal sleep-wake cycle.

(c) No more than two night shifts at this stage. Night shifts are low in volume and complaints, but a stable sleep routine is central to symptom management following cessation of the night-time medication; capping nights protects that routine and limits cumulative fatigue. This cap is suitable for reassessment at review.

This reduced pattern of approximately six shifts per fortnight is the pattern Mr Shepherd has in fact worked and tolerated over the past twelve months without deterioration, and maintaining rather than increasing it best supports recovery while minimising risk of exacerbation. Usual switchboard operational duties remain suitable; complaint-handling duties are excluded as below.

4. Are there any specific workplace modifications or supports you would recommend (e.g. reduced workload, predictable rostering, supervision level, reduced exposure to stressors) to assist Mr Shepherd during this period?

(a) No complaint-handling duties — complaints received at the Switchboard to be logged and redirected, not actioned or resolved by Mr Shepherd; (b) predictable rostering published in advance, with a minimum 10-hour break between shifts consistent with the Award and fatigue risk management standards; (c) return-to-work coordination and correspondence through the Injury Management team, Human Resources; (d) access to EAP (TELUS Health) as an available support; (e) review in approximately 8 weeks.

Please complete this form and return to the Injury Management team, Logan & Beaudesert Health Service, Human Resources by email lbh_injurymanagement@health.qld.gov.au

Please include your practice details/stamp here

Signature

Date



3-7-26.

**MY DOCTORS CLINIC
OPEN 24 HOURS**

Dr Day Hong Ma

MBBS,FRACGP,FACRRM Provider No: 4696842K

16/3221 Surfers Paradise Blvd

Surfers Paradise, QLD 4217

Ph: 07-5592 2299 Fax: 07-5539 9188

www.mydoctorsclinicsurfers.com.au

Re: Cory - ECC/Leave Type

From: Cory Shepherd

coryshepherd1@hotmail.com

To: Chloe Taylor Chloe.Taylor3@health.qld.gov.au

Sent: Friday 3 July at 15:18

Hi Chloe,

Thanks for that. As the time off was due to my medical condition and my paid sick leave is exhausted, could you please process the balance as annual leave rather than sick leave without pay.

On returning the completed Employee Capability Checklist went to the Injury Management team today. It certifies me fit to resume from 3 July 2026. Could you please confirm I'm right to attend, or otherwise let me know my next returning shift.

Kind regards,

Cory

From: Chloe Taylor <Chloe.Taylor3@health.qld.gov.au>

Sent: Friday, July 3, 2026 2:49:32 pm

To: Cory Shepherd <coryshepherd1@hotmail.com>

Cc: Cory Shepherd

<Cory.Shepherd@health.qld.gov.au>

Subject: RE: Cory - ECC/Leave Type

Hi Cory

Thank you for letting me know. As you have not confirmed leave type for the past pay period – I will continue to process as Sick leave / Sick leave no pay

Kind regards,

Chloe

Chloe Taylor

Switchboard Manager | Local Telecommunications Representative | Switchboard Services

Logan Hospital | Metro South | Queensland Health

P (07) 2891 9190

E chloe.taylor3@health.qld.gov.au

W www.health.qld.gov.au

A Level 2, Building 1, Cnr Loganlea and Armstrong Roads QLD 4131

T [Call me on Teams](#)



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From: Cory Shepherd <coryshepherd1@hotmail.com>

Sent: Friday, 3 July 2026 2:35 PM

To: Chloe Taylor <Chloe.Taylor3@health.qld.gov.au>

Subject: Re: Cory - ECC/Leave Type

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safe.**

Hey Chloe,

I had a long consultation and the doctor filled out the form.

I sent it off to the Injury Management team today.

Kind regards,

Cory

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From: Chloe Taylor <Chloe.Taylor3@health.qld.gov.au>

Sent: Thursday, July 2, 2026 2:38:11 pm

To: Cory Shepherd <Cory.Shepherd@health.qld.gov.au>

Cc: Cory Shepherd <coryshepherd1@hotmail.com>

Subject: Cory - ECC/Leave Type

Hi Cory,

I just wanted to check in to see how you are doing and how things have been progressing for you. I know you had a scheduled appointment with your doctor yesterday as per text message sent on Monday 29th July.

Did your doctor give you any indication when the required documentation will be completed? Until we receive appropriate medical clearance, including a completed Employee Capacity Certificate (ECC), we are unable to facilitate your return to work, please wait for my advice to when your next returning shift will be. Unless you advise me otherwise, I will process your shifts that you have not worked as Sick Leave – Please confirm by Friday 3rd July before 2:00pm.

Kind regards,
Chloe

Chloe Taylor
[Switchboard Manager](#) | [Local Telecommunications Representative](#) | [Switchboard Services](#)
Logan Hospital | Metro South | Queensland Health
P (07) 2891 9190
E chloe.taylor3@health.qld.gov.au
W www.health.qld.gov.au
A Level 2, Building 1, Cnr Loganlea and Armstrong Roads QLD 4131
T [Call me on Teams](#)







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Cory Shepherd_ECC further information

From: Lyndelle Forrest

Lyndelle.Forrest@health.qld.gov.au

To: coryshepherd1@hotmail.com

coryshepherd1@hotmail.com

Cc: LBH Injury Management

LBH_InjuryManagement@health.qld.gov.au

Sent: Tuesday 7 July at 10:54

Good morning Cory

I refer to the Employee Capability Checklist (**ECC**), completed by Dr Day Hong Ma of My Doctor Clinic on 3 July 2026 and returned to the Injury Management Team at 2:30pm that day. It is further noted that due to a section of this document being missing, you were asked to resubmit your ECC for consideration, you sent this on Friday at 3:06pm.

While you have been certified fit to return to work effective from 3 July 2026, this certification is subject to reasonable adjustments being made to your duties and hours of work. As the completed ECC was received late on Friday, 3 July 2026, the Health Service was unable to review Dr Hong Ma's medical recommendations at that time to determine whether the recommended restrictions and adjustments could be reasonably accommodated and whether it was safe for you to return to duty.

I can advise that your ECC has now been reviewed and carefully considered. However, before a final decision can be made regarding your return to the workplace, further information is required. Specifically, it is noted that your treating medical practitioner has advised that there are unresolved workplace matters involving line management. At this stage, the Health Service does not have sufficient information to understand the nature of these matters or assess whether they present any psychosocial hazards or risks that may affect your proposed return to work.

The Health Service remains committed to supporting your safe and sustainable return to work, with your health, safety and wellbeing remaining central to that process. Accordingly, further information is required to enable the Health Service to assess any identified risks, determine whether reasonable adjustments can be supported, and ensure that any return to work is planned and managed in accordance with its obligations under work health and safety legislation.

To assist us in expediting consideration of your reasonable adjustments, please provide further clarification and/or reports referenced below, within seven (7) calendar days.

ECC Recommendation	Further information required
<i>Diagnosis of Major Depressive Disorder with anxious distress, a work-related psychological injury with onset 18 June 2024, with symptoms exacerbated on exposure to identified workplace stressors (complaint handling; being held accountable and blamed for failures of others; unpredictable rostering.</i>	The Health Service is seeking to better understand the work-related psychological injury for which you are currently receiving treatment from a psychologist. It is also noted that Dr Day Hong Ma has referenced a report from your treating psychiatrist dated 13 February 2025. To date, the Health Service has not been provided with, nor had the opportunity to review, this report. To enable the Health Service to appropriately consider and address these matters, further information and/or a copy of this report is requested regarding the workplace issues referred to in the ECC and their impact on your capacity for work.

Complaint handling No complaint-handling duties, complaints received at the Switchboard to be logged and redirected, not actioned or resolved by Mr Shepherd.	The Health Service understand that the established process for management of complaints is for all patient concerns to be redirection to the Consumer Liaison Services (CLS). There is no expectation for Switchboard operators to manage or deal with these matters. Where a consumer raises concerns in relation to a Switchboard employee, these concerns are referred to the Manager, Switchboard or in her absence, CLS. To understand this concern better and adjustments required, please clarify what features of complaint handling are being referenced and you are expected to resolve within this recommendation.
Predictable Rostering practices, minimum 10-hour break between shifts	Rosters are generally published with four (4) weeks' notice, on four-week rotation. Occasionally, in extenuating circumstances these might be delayed. However, all rosters are released with a minimum of two weeks' notice. The Health Service is not aware of breaching

	this provision and has not received any Workload Concerns to review, can you please clarify the concerns relating to not providing the minimum break between rostered shifts? Noting that rostering currently provides more than 10-hour breaks between shifts, however, acknowledge you also signed an 8 hour agreement on 17 June 2020, which allows you to work with only an 8 hour break, however this is only applied where staff initiated shift swaps have occurred. Please clarify the specific stressors which may exacerbate symptoms.
Being held accountable and blamed for failures of others	It is unclear what matters you are being held accountable and blamed for based on the failures of others or what these concerns relate to. To-date the Health Service is not aware of any concerns being raised for appropriate management. Further clarity on this would assist in understanding this matter.

Until this information is available and has been appropriately assessed, the Health Service is not in a position to safely facilitate your return to normal duties.

Should you have any further questions or concerns, please do not hesitate to contact me via the details below.

Kind regards

Lyndelle Forrest
Senior Consultant
Human Resources
Logan and Beaudesert Health Service
[Metro South Health](#) | 

e. Lyndelle.Forrest@health.qld.gov.au | [Chat on Teams](#)
p. [07 3542 2028](tel:0735422028)
a. Mon - Fri 7:30am - 4:00pm



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**Re: Cory Shepherd_ECC further
information (MSH-INJ-5795)**

From: Cory Shepherd

coryshepherd1@hotmail.com

To: LBH Injury Management

LBH_InjuryManagement@health.qld.gov.au

Cc: Notes - IPEC (notes@solv.com.au)

notes@solv.com.au

Sent: Monday 13 July at 16:39

Good afternoon Michelle,

Thank you for your email. Four matters in response.

1. QSuper. I must correct the position stated in your email. I no longer have an active open QSuper Income Protection claim; that claim is now closed. In any event, the supporting medical documentation could only support a partial payment at most: I am not totally incapacitated. I am certified fit for my existing arrangement. There is no income protection payment that could respond to the present situation, in which I am receiving no shifts at all.

My current loss of income arises from the Health Service's decision not to roster me — which your email itself confirms ("we are unable to support a return to work at this time") — and not from my capacity for work. There is accordingly no income support available to me from that source.

2. Pay. My request, made in each of my previous emails — that rostered shifts during this employer-directed period be treated as paid time and not deducted from my leave — has not yet been answered. Given the above, I ask again. If the Health Service's position is that these shifts will be unpaid or deducted from my leave, could you please confirm that in writing, together with the basis for it, so that I can consider my position.

3. Interim alternative duties. Separately, should the Health Service require further time to assess my return to the Switchboard, I note that I also bring prior skills and experience from within Metro South Health, including in the finance department and in scanning/records. I am ready and willing to perform suitable interim duties of that kind, within my certified capacity, as an alternative to remaining off work unpaid while the assessment is completed. This offer is made to my substantive Switchboard position and certified arrangement, to which I remain ready to return. I would welcome a discussion about how my experience can be used to the benefit of the Service in the interim.

4. Dr Ma. My consent for the Health Service to contact Dr Day Hong Ma for clarification, limited to my present work capacity and the recommended adjustments, stands. As I have now been held out of work since 3 July while certified fit, could you please send any questions to Dr Ma promptly, copy me for my records, and advise the timeframe within which my return will be decided once his response is received.

I remain certified fit, ready, and available to work my rostered shifts.

Kind regards,
Cory Shepherd

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From: LBH Injury Management

<LBH_InjuryManagement@health.qld.gov.au>

Sent: Monday, 13 July 2026 15:53:26

To: Cory Shepherd <coryshepherd1@hotmail.com>

Cc: Notes - IPEC (notes@solv.com.au)

<notes@solv.com.au>

Subject: RE: Cory Shepherd_ECC further information
(MSH-INJ-5795)

Good afternoon Cory,

Thank you for your email and for providing your response to the further information requested.

I acknowledge that you have advised you do not wish to provide further information regarding the WorkCover matter referenced in your Employee Capability Checklist (ECC) and that you have provided consent for the Health Service to contact your treating practitioner, Dr Day Hong Ma, for clarification regarding your current work capacity and the recommended workplace adjustments.

As mentioned below in the email sent to you from Ms Forrest on 7 July 2026, we are currently unable to safely support or accommodate a Graduated Return to Work Plan that meets your medical requirements. For us to consider your request to return to duties further we require further medical clarification.

Once this information has been received and appropriately reviewed, the Health Service will be in a position to assess whether a return to restricted duties can be facilitated, and we will advise you of the outcome as soon as practicable.

As you currently have an active open QSuper Income Protection claim, any payments that are applicable to you receiving, would be facilitated by them. You should contact your claim manager James Zappia at QSuper and inform them that we are unable to support a return to work at this time.

Should you have any questions in the meantime, please do not hesitate to contact me.

Kindest Regards

Michelle Harrison

Injury

Management Consultant | LBHS HR Team 1

Human Resources
Logan and Beaudesert Hospital and Health Service
4/3-5 University Drive, Meadowbrook Qld 4131

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lbh_injurymanagement@health.qld.gov.au | [Chat on Teams](#)
e. qld.gov.au
p. 07 3542 2744
a. Mon-Fri 8am - 4pm

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Please be advised that we are currently transitioning into Solv, a new Injury Management Case System. Our documents and communications may change appearance slightly and you may even receive communications from us via the @Solv.com.au email pathway, as this is a new email pathway, please ensure you are regularly checking your Junk folder in your email accounts and save the @solv to your frequent emails so you do not miss any communications. Please note these emails are legitimate and are from us here at lbh_injurymanagement@health.qld.gov.au. If you have any concerns, please reach out to us via the above contact details.

From: Cory Shepherd <coryshepherd1@hotmail.com>
Sent: Friday, 10 July 2026 8:35 AM
To: Lyndelle Forrest <Lyndelle.Forrest@health.qld.gov.au>
Cc: LBH Injury Management
<LBH_injurymanagement@health.qld.gov.au>
Subject: Re: Cory Shepherd_ECC further information

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Good morning Ms Forrest,

Thank you for your email of 7 July 2026, which I have only now had the opportunity to consider. Apologies for the delay in responding.

I am certified fit to return to work from 3 July 2026 and am ready and available to return. The ECC records the same reduced work arrangement I have performed for approximately the past twelve months. That arrangement is not new; it is now formally documented by my treating practitioner.

I note that the adjustments identified in the ECC appear to be accommodated by existing practice. Your email confirms that Switchboard operators are not expected to manage or resolve consumer complaints, and that current rostering provides more than 10-hour breaks between shifts and is published in advance. Together with my existing shift arrangement, this appears to address the recommendations in the ECC.

I ask that my return to rostered shifts be facilitated, with any remaining clinical clarification obtained in parallel.

Regarding the further information requested:

- **Workplace matters:** These matters are the subject of proceeding WC/2024/227 before the Queensland Industrial Relations Commission. The Health Service is legally represented through Metro South Legal Services, reference K-LM26/729. I will not provide further particulars through this return-to-work process. Any questions about those matters should be directed to Metro South Legal Services.

- **Clinical recommendations:** The recommendations are those of Dr Day Hong Ma. I am not able to interpret or expand on clinical recommendations. If clarification is required, I consent to the Health Service contacting Dr Hong Ma directly, limited only to my present work capacity and the recommended adjustments.
- **Report dated 13 February 2025:** I do not consent to providing this report through this process. It contains sensitive information beyond what is necessary to implement the ECC recommendations. My present fitness for duty is addressed in the ECC and may be clarified by Dr Hong Ma if required.

If the Health Service decides not to roster me pending its assessment, please confirm that these shifts will be treated as paid time and will not be deducted from my leave, as I am certified fit and available to work.

I remain available to commence at my next rostered shift.

Kind regards,
Cory Shepherd

From: Lyndelle Forrest

<Lyndelle.Forrest@health.qld.gov.au>

Sent: Tuesday, July 7, 2026 10:54:35 am

To: coryshepherd1@hotmail.com

<coryshepherd1@hotmail.com>

Cc: LBH Injury Management

<LBH_InjuryManagement@health.qld.gov.au>

Subject: Cory Shepherd_ECC further information

Good morning Cory

I refer to the Employee Capability Checklist (**ECC**), completed by Dr Day Hong Ma of My Doctor Clinic on 3

July 2026 and returned to the Injury Management Team at 2:30pm that day. It is further noted that due to a section of this document being missing, you were asked to resubmit your ECC for consideration, you sent this on Friday at 3:06pm.

While you have been certified fit to return to work effective from 3 July 2026, this certification is subject to reasonable adjustments being made to your duties and hours of work. As the completed ECC was received late on Friday, 3 July 2026, the Health Service was unable to review Dr Hong Ma's medical recommendations at that time to determine whether the recommended restrictions and adjustments could be reasonably accommodated and whether it was safe for you to return to duty.

I can advise that your ECC has now been reviewed and carefully considered. However, before a final decision can be made regarding your return to the workplace, further information is required. Specifically, it is noted that your treating medical practitioner has advised that there are unresolved workplace matters involving line management. At this stage, the Health Service does not have sufficient information to understand the nature of these matters or assess whether they present any psychosocial hazards or risks that may affect your proposed return to work.

The Health Service remains committed to supporting your safe and sustainable return to work, with your health, safety and wellbeing remaining central to that process. Accordingly, further information is required to enable the Health Service to assess any identified risks, determine whether reasonable adjustments can be supported, and ensure that any return to work is planned and managed in accordance with its obligations under work health and safety legislation.

To assist us in expediting consideration of your reasonable adjustments, please provide further clarification and/or reports referenced below, within seven (7) calendar days.

ECC Recommendation	Further information required
<i>Diagnosis of Major Depressive Disorder with anxious distress, a work-related psychological injury with onset 18 June 2024, with symptoms exacerbated on exposure to identified workplace stressors (complaint handling; being held accountable and blamed for failures of others; unpredictable rostering.</i>	The Health Service is seeking to better understand the work-related psychological injury for which you are currently receiving treatment from a psychologist. It is also noted that Dr Day Hong Ma has referenced a report from your treating psychiatrist dated 13 February 2025. To date, the Health Service has not been provided with, nor had the opportunity to review, this report. To enable the Health Service to appropriately consider and address these matters, further information and/or a copy of this report is requested regarding the workplace issues referred to in the ECC and their impact on your capacity for work.
Complaint handling No complaint-handling duties, complaints received at the Switchboard to be logged and redirected, not actioned or resolved by Mr Shepherd.	The Health Service understand that the established process for management of complaints is for all patient concerns to be redirection to the Consumer Liaison Services (CLS). There is no expectation for

	Switchboard operators to manage or deal with these matters. Where a consumer raises concerns in relation to a Switchboard employee, these concerns are referred to the Manager, Switchboard or in her absence, CLS. To understand this concern better and adjustments required, please clarify what features of complaint handling are being referenced and you are expected to resolve within this recommendation.
Predictable Rostering practices, minimum 10-hour break between shifts	Rosters are generally published with four (4) weeks' notice, on four-week rotation. Occasionally, in extenuating circumstances these might be delayed. However, all rosters are released with a minimum of two weeks' notice. The Health Service is not aware of breaching this provision and has not received any Workload Concerns to review, can you please clarify the concerns relating to not providing the minimum break

	between rostered shifts? Noting that rostering currently provides more than 10-hour breaks between shifts, however, acknowledge you also signed an 8 hour agreement on 17 June 2020, which allows you to work with only an 8 hour break, however this is only applied where staff initiated shift swaps have occurred. Please clarify the specific stressors which may exacerbate symptoms.
Being held accountable and blamed for failures of others	It is unclear what matters you are being held accountable and blamed for based on the failures of others or what these concerns relate to. To-date the Health Service is not aware of any concerns being raised for appropriate management. Further clarity on this would assist in understanding this matter.

Until this information is available and has been appropriately assessed, the Health Service is not in a position to safely facilitate your return to normal duties.

Should you have any further questions or concerns, please do not hesitate to contact me via the details

below.

Kind regards

Lyndelle Forrest
Senior Consultant
Human Resources
Logan and Beaudesert Health Service
[Metro South Health](#) | 

e. Lyndelle.Forrest@health.qld.gov.au | [Chat on Teams](#)
p. [07 3542 2028](tel:0735422028)
a. Mon - Fri 7:30am - 4:00pm



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RE: Request for contact re workplace issues

From: Heath Moran heath.moran@together.org.au

To: Cory Shepherd coryshepherd1@hotmail.com

Sent: Wednesday 15 July at 15:27

Thank you for providing this Cory,

I'll have time to go through these concerns on Friday as there is significant material here I don't want to make any comments until I've had a chance to review them. Once I've reviewed them I'll get back to you with my thoughts.

Yours in Union,

Heath Moran

Organiser – South Team



0414 225 851

[heath.moran@together](mailto:heath.moran@together.org.au)

[her.org.au](http://together.org.au)



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From: Cory Shepherd <coryshepherd1@hotmail.com>

Sent: Tuesday, 14 July 2026 5:03 PM

To: Heath Moran <heath.moran@together.org.au>

Subject: Re: Request for contact re workplace issues

Hi Heath,

Thanks for getting back to me.

I'm handing over my appeal submissions — not to address the appeal itself, but because they give the fullest picture of the workplace issues I raised.

The documents I attach are for you to read over and identify the risks associated in the role. They are not to be shared but the information I provide in the submissions are to assist in identifying the psychosocial risks I believe that are unassessed.

Furthermore, I have no grievances with Ms Taylor personally; I don't know if the same view is shared but I could be wrong.

Happy to talk any of it through once you've read the attachments.

Form 20 is my affidavit and have I described the role and the workplace. I will send a further email of the completed ECC my Dr has completed, and HR has since told me that I may not return.

Regards,
Cory

From: Heath Moran <heath.moran@together.org.au>

Sent: Tuesday, July 14, 2026 4:04:39 pm

To: Cory Shepherd <coryshepherd1@hotmail.com>

Subject: RE: Request for contact re workplace issues

Hey Cory,

Thank you for reaching out this is very serious.

For your matter I'll be sending an urgent referral to our industrial team for assistance, if you've been put on an ECC, completed it and are still being told you cannot return, I need to make sure an industrial officer from Together looks over this before anything is actioned as I need to make sure you get the best information.

I'd be very eager to discuss your ongoing concerns for the hazards in switchboard as this is a recurring theme raised by members.

I have time to talk on Thursday morning around 9-10:30am and time sporadically tomorrow to call in between site visits, are you available any time tomorrow or Thursday morning.

Looking forward to hearing from you.

Yours in Union,

Heath Moran

Organiser – South Team



0414 225 851

[heath.moran@together](mailto:heath.moran@together.org.au)

her.org.au



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From: Cory Shepherd <coryshepherd1@hotmail.com>

Sent: Tuesday, 14 July 2026 10:50 AM

To: Heath Moran <heath.moran@together.org.au>

Subject: Request for contact re workplace issues

Good morning Heath,

I would like to request a time to speak with you by phone regarding some workplace issues that are causing me serious concern.

Firstly, I understand that Chloe Taylor has informed others that a full-time position will be available soon. My understanding is that this role falls within my line of work, and I would appreciate clarification about the position and my status in relation to it.

Secondly, I have been required to undergo an ECC assessment in order to return to my duties, despite having substantially performed the same role over the past 12 months on reduced hours that I have been able to manage. This requirement was not raised when I previously returned to work following an unfair dismissal matter, which arose after I submitted a public interest disclosure in 2024 that was validated and related to concerns involving Chloe Taylor.

More recently, I was directed not to return to work until my doctor completed the ECC, which has now been done. I am nevertheless still being instructed to remain away from the workplace. I am concerned that this may be connected to my current WorkCover appeal, in which I have raised systemic risks to patient safety arising from how the switchboard is being run, and have explained in detail how those risks contributed to my injury and to subsequent reprisal.

I would welcome the opportunity to discuss these matters with you, including the full-time position, the ECC requirements, and my ongoing exclusion from work. I would also like to share the records I hold, the risks I have identified, and why I consider Switchboard to be an unassessed psychosocial

hazard, including the ways in which its current operation affects both patients and employees, together with the adverse action that I believe is presently being applied against me.

Kind regards

Cory

RE: INFORMATION UPDATE | QSuper
Income Protection Claim: Cory Shepherd
(MSH-INJ-5795) [CLM-317073]

From: LBH Injury Management

LBH.InjuryManagement@health.qld.gov.au

To: Cory Shepherd coryshepherd1@hotmail.com,

James Zappia james.zappia@art.com.au

Cc: notes@solv.com.au notes@solv.com.au

Sent: Wednesday 15 July at 13:10

Good afternoon Cory,

The only information supplied to QSuper/ART from injury management is the email I cc'd you in on Monday 13/07/2026, which confirmed if the Health Service could support your return or not. This is in line with our process where a QSuper Claimant requests to return to duties.

In relation to conversations, again, these have related to if the Health Service could support your return to duties and that you have a current QSuper Income Protection Claim.

Consent for QSuper (Australian Retirement Trust) to obtain information from Metro South Health was provided when you completed and signed the declaration and authorisation on page 7 of your Income Protection Application. That authorisation permits QSuper to request relevant information from your employer in relation to your claim.

I will endeavour to complete the Request for Medical Information expeditiously; however, I am unable to commit to a timeframe for this.

In relation to your pay request, as we have been provided information that you do hold a current QSuper Income Protection Claim, we are not responsible for your payments whilst on this claim,

that does sit with your QSuper Claim Manager to discuss with you. Where you have received advice from QSuper/ART that your claim has been closed, please provide a copy of this correspondence and/or email from QSuper/ART for our records.

In relation to your request to perform duties in an alternative work area while this matter is being assessed and having regard to the medical information provided by Dr Ma in the Employee Care Centre (ECC) assessment, the Health Service is unable to consider this arrangement at this stage. This is because further medical information is required to understand the nature of the matters identified by Dr Ma and to assess whether they present any psychosocial hazards or risks that may affect your proposed return to work in any capacity. Until this information is received and considered, the Health Service is unable to undertake an appropriate assessment of your request.

I will also clarify that the obligation to provide rehabilitation and suitable duties under the *Workers' Compensation and Rehabilitation Act 2003 (Qld)* applies to workers with accepted workers' compensation claims. It does not extend to employees being managed through Health Management processes or those receiving QSuper (Australian Retirement Trust) Income Protection benefits.

As such, any return to work under a Health Management or QSuper Income Protection arrangement is subject to the employer being able to safely accommodate the medical restrictions within the operational requirements of the workplace and remains at the discretion of the Employer if they are safely able to accommodate this.

Kindest Regards
Michelle Harrison

Injury Management Consultant | LBHS HR Team 1

Human Resources

Logan and Beaudesert Hospital and Health Service
4/3-5 University Drive, Meadowbrook Qld 4131

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From: Cory Shepherd <coryshepherd1@hotmail.com>

Sent: Monday, 13 July 2026 4:57 PM

To: LBH Injury Management

<LBH_injurymanagement@health.qld.gov.au>; James Zappia <james.zappia@art.com.au>

Cc: notes@solv.com.au

Subject: Re: INFORMATION UPDATE | QSuper Income Protection Claim: Cory Shepherd (MSH-INJ-5795) [CLM-317073]

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safe.**

Good afternoon Michelle,

I acknowledge receipt of your correspondence to Mr Zappia at QSuper. Four matters in response.

1. Disclosure to QSuper. For my records, could you please confirm what documents and information have been provided to QSuper/ART in relation to me, on what dates, and the authority relied upon for providing my health information to the fund — noting my advice that the income protection claim is closed. Could you also confirm the substance of what was discussed with Mr Zappia by telephone. I will be corresponding with Mr Zappia directly regarding the fund's position.

2. The request to Dr Ma. I note the Request for Medical Information will be forwarded "over the coming days." As I have now been held out of work since 3 July while certified fit, I ask that it be sent within three (3) business days, with a copy provided to me as previously requested, and that you advise the timeframe within which my return to work will be decided once Dr Ma's response is received.

I would also note that 28 August 2026 is Dr Ma's review date for the restrictions — it is not a start date for my capacity to work within them, which is certified now and has been since 3 July. A process paced to late August would leave me without work or income for some eight weeks while certified fit, which I trust is not the Service's intention.

3. Pay. My request — made in each of my previous emails — that rostered shifts during this employer-directed period be treated as paid time and not

deducted from my leave remains unanswered. I renew it. If the Health Service's position is that these shifts will be unpaid or deducted from my leave, please confirm that in writing, together with the basis for it, so that I can consider my position.

4. Interim alternative duties. My offer to perform suitable interim duties within my certified capacity — drawing on my prior Metro South Health experience in the finance department and in scanning/records and to my substantive Switchboard position — also remains open and unanswered. I would welcome a response.

I remain certified fit, ready, and available to work my rostered shifts.

Kind regards,
Cory Shepherd

From: LBH Injury Management

<LBH_InjuryManagement@health.qld.gov.au>

Sent: Monday, July 13, 2026 4:20:56 pm

To: James Zappia <james.zappia@art.com.au>

Cc: notes@solv.com.au <notes@solv.com.au>; Cory Shepherd <coryshepherd1@hotmail.com>

Subject: INFORMATION UPDATE | QSuper Income Protection Claim: Cory Shepherd (MSH-INJ-5795) [CLM-317073]

Good afternoon James,

Thank you for taking the time to speak with me this afternoon.

As discussed, based on the information currently available, we are unable to facilitate Cory's return to work in accordance with the restrictions outlined in the attached Employee Capabilities Checklist (ECC) completed by his treating practitioner.

To ensure we have sufficient information to make an informed decision, we will be seeking further medical clarification from Dr Day Hong Ma. This will assist us in better understanding the context of the recommended workplace adjustments and whether a restricted return to work can be safely and operationally supported. Once this information has been received, we will undertake a further review of Cory's capacity for work.

I note that Dr Day Hong Ma has nominated a review date of 28 August 2026. Over the coming days, we will forward a Request for Medical Information to Dr Day Hong Ma to obtain further clarification regarding the recommended restrictions, including the reduction in hours from Cory's substantive position, to assist us in determining whether a safe and sustainable return to work can be facilitated.

Please don't hesitate to contact me if you have any questions or require any further information.

Kindest Regards

Michelle Harrison

Injury

Management Consultant | LBHS HR Team 1

Human Resources

Logan and Beaudesert Hospital and Health Service

4/3-5 University Drive, Meadowbrook Qld 4131

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RE: INFORMATION UPDATE | QSuper
Income Protection Claim: Cory Shepherd
(MSH-INJ-5795) [CLM-317073]

From: LBH Injury Management

LBH.InjuryManagement@health.qld.gov.au

To: Cory Shepherd coryshepherd1@hotmail.com,

James Zappia james.zappia@art.com.au

Cc: notes@solv.com.au notes@solv.com.au

Sent: Wednesday 15 July at 13:10

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Kindest Regards
Michelle Harrison

Injury Management Consultant | LBHS HR Team 1

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Sent: Monday, 13 July 2026 4:57 PM

To: LBH Injury Management

<LBH_injurymanagement@health.qld.gov.au>; James

Zappia <james.zappia@art.com.au>

Cc: notes@solv.com.au

Subject: Re: INFORMATION UPDATE | QSuper Income Protection Claim: Cory Shepherd (MSH-INJ-5795) [CLM-317073]

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Good afternoon Michelle,

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I remain certified fit, ready, and available to work my rostered shifts.

Kind regards,
Cory Shepherd

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<LBH_InjuryManagement@health.qld.gov.au>

Sent: Monday, July 13, 2026 4:20:56 pm

To: James Zappia <james.zappia@art.com.au>

Cc: notes@solv.com.au <notes@solv.com.au>; Cory Shepherd <coryshepherd1@hotmail.com>

Subject: INFORMATION UPDATE | QSuper Income Protection Claim: Cory Shepherd (MSH-INJ-5795) [CLM-317073]

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To ensure we have sufficient information to make an informed decision, we will be seeking further medical clarification from Dr Day Hong Ma. This will assist us in better understanding the context of the recommended workplace adjustments and whether a restricted return to work can be safely and operationally supported. Once this information has been received, we will undertake a further review of Cory's capacity for work.

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Please don't hesitate to contact me if you have any questions or require any further information.

Kindest Regards

Michelle Harrison

Injury

Management Consultant | LBHS HR Team 1

Human Resources

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RE: Request for contact re workplace issues

From: Heath Moran heath.moran@together.org.au

To: Cory Shepherd coryshepherd1@hotmail.com

Sent: Friday 17 July at 15:27

Hey Cory,

To keep you updated I'm still reviewing this information and I'm speaking to our industrial team about best steps forward.

Once I have more I'll let you know.

Yours in Union,

Heath Moran

Organiser – South Team



0414 225 851

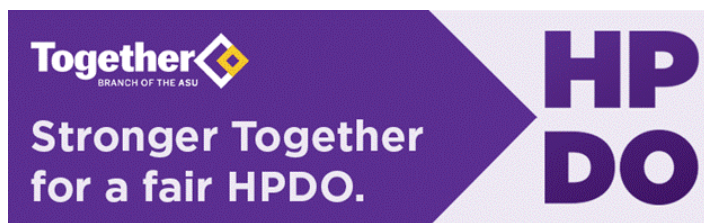
[heath.moran@together](mailto:heath.moran@together.org.au)

[her.org.au](http://together.org.au)



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From: Cory Shepherd <coryshepherd1@hotmail.com>

Sent: Thursday, 16 July 2026 4:42 PM

To: Heath Moran <heath.moran@together.org.au>

Subject: Re: Request for contact re workplace issues

Good afternoon Heath,

The final piece for your review — the full email chain ending with HR's letter of 15 July 2026 (attached), which sets out their position in writing. I've held off responding pending your review and the industrial team's advice. I will however include you in any further correspondence. To make Friday easier, here is the chain in short, and the issues as I understand them.

What the chain shows

13 July: HR (Michelle Harrison, Injury Management) phoned and then wrote to my QSuper case manager advising the Health Service is "unable to facilitate" my return — before the further medical information they say they need had been requested from my GP.

13 July (me): I asked four things — what was disclosed to the fund and on what authority; that the Request for Medical Information to my GP go out within 3 business days with a copy to me, and the decision timeframe once his response arrives; that my rostered shifts while held out be paid and not taken from my leave; and a response to my standing offer of interim duties.

15 July (their letter): the request to my GP has no timeframe ("unable to commit"); pay is refused on the basis that I "hold a current QSuper Income Protection Claim" (I don't — it is closed, and I've asked the fund for written confirmation); interim duties are refused "in any capacity" pending the un-timetabled GP request; and my situation "remains at the discretion of the Employer" because I don't have an accepted workers' compensation claim, with reference to employees "being managed through Health Management processes."

Issue 1 — the situation is being mischaracterised as a "return to work." I have been continuously employed and working my shifts for the past year, on hours I adjusted within my certified capacity. Throughout that period I kept the fund side correct myself: while the appeal was ongoing I asked QSuper to assess my payslips and adjust my benefit to partial payments reflecting the hours I was actually working, and ultimately my income protection payments stopped at my own request, to prevent overpayment, because I was working. On 3 July at about 2:32pm I provided the Employee Capability Checklist my GP completed certifying me fit to work with identified restrictions — as a serving employee updating my medical information as requested by my employer while finalising my super matters, not as an application for permission to work. I was rostered to work at that time. By about 3:30pm the same day — under an hour — I was held out of the workplace "pending review," with my rostered shifts "paid out of [my] available sick leave." I have been out since, thirteen days now, with no timeframe. Accurately described, I am a working employee directed not to attend by my employer while certified fit — not an absent employee requesting to return. Their letters (including to my super fund) describe me as "a QSuper Claimant request[ing] to return to duties," which is wrong on both counts.

Issue 2 — pay and leave. I am, and at all times since 3 July have been, ready, willing and able to work my rostered shifts; the decision that I not attend is the Health Service's, not mine. No stand-down, suspension or other power has been identified to me, yet the shifts are being debited against my own sick leave — leave that exists to cover incapacity, which on their own file I don't have, and which I have already substantially exhausted through my illness. Their stated basis for refusing pay — that the fund is responsible — rests

on a claim that is closed, and in any event an insurance benefit is not wages.

Issue 3 — the policies and frameworks I believe are being misapplied or ignored. Their letter treats the absence of an accepted workers' compensation claim as leaving everything to "the discretion of the Employer." As I understand it, that is not a complete statement of the position:

(a) Queensland Health's Workplace Rehabilitation Standard (QH-IMP-401-5) provides that rehabilitation, suitable duties and temporary adjustments may be provided to employees without an accepted workers' compensation claim where operationally reasonable, and that managers are responsible for identifying such duties;

(b) Queensland Health's Reasonable Adjustment policy (QH-POL-210) applies to my circumstances;

(c) the Anti-Discrimination Act 1991 (Qld), including the positive duty in force since 1 July 2025, and the Human Rights Act 2019 (Qld) s 58, apply to decisions the Health Service makes about my employment;

(d) the Work Health and Safety Act 2011 (Qld), ss 47–49, requires consultation with me about matters affecting my health and safety. There has been none: the 3 July decision was made and communicated within the hour, and I have not been consulted since — even though their letter now frames the assessment in psychosocial-risk terms.

Issue 4 — "Health Management processes." Their letter says the rehabilitation obligations don't extend to employees "being managed through Health Management processes" — but no one has told me whether I am in such a process, which one, under what policy or statutory instrument, when it commenced, or who the decision-maker is. If this is the Public Sector Act 2022 chapter 3, part 8 pathway, I don't understand how its preconditions

could be met — I was working until directed not to attend, and I hold a current fitness-for-duty certification.

Issue 5 — the request to my GP. There is no timeframe to send it and no timeframe to decide once it's answered, and their letter describes its subject as "the nature of the matters identified by Dr Ma" — which reads wider than the restrictions themselves. I have asked that any request to my GP be copied to me and confined to my capacity, my restrictions, and their operational application.

Issue 6 — the two-year reversal on "psychosocial risk." The psychosocial concerns in this workplace were raised by me, beginning more than two years ago: fatigue and rest-break breaches (including the March 2024 rostering that left me a 7-hour break between shifts), unresolved pay failures, and switchboard workload and communication-flow issues — including in a written grievance on 7 August 2023 that was dismissed the same day it was lodged, and later in the disclosure I mentioned in my first email.

To my knowledge, no psychosocial risk assessment of the switchboard was ever undertaken in response to any of it, and I managed my own exposure over the past year by adjusting my hours within my certified capacity.

The first time the Health Service has applied psychosocial-risk language to this workplace in writing is the 15 July letter — and its subject is not the hazards I raised, but me: whether the "matters identified by Dr Ma" (that is, my medical condition) "present any psychosocial hazards or risks." Two years on, the hazards remain unassessed, and the worker who raised them — and was injured — has become the subject of the assessment. Given you mentioned switchboard hazards are a recurring

theme from members, I'd particularly value the industrial team's view on that sequence.

I had prepared a full response to HR raising all of this, with the policy citations — I'm holding it and won't send anything until you and the industrial team have advised.

One further matter for awareness: while I have been held out, a colleague Caroline has contacted me asking whether I have been dismissed, and I understand the upcoming full-time position I mentioned in my first email has been communicated to casual staff. So, while I am excluded from the workplace with no timeframe, my absence is being noticed by colleagues, and a full-time role in my line of work is being discussed with casuals.

Thanks again for moving quickly on this.

Kind regards,
Cory Shepherd
[0417 400 227](tel:0417400227)

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From: Heath Moran <heath.moran@together.org.au>
Sent: Wednesday, July 15, 2026 3:27:25 pm
To: Cory Shepherd <coryshepherd1@hotmail.com>
Subject: RE: Request for contact re workplace issues

Thank you for providing this Cory,

I'll have time to go through these concerns on Friday as there is significant material here I don't want to make any comments until I've had a chance to review them. Once I've reviewed them I'll get back to you with my thoughts.

Yours in Union,

Heath Moran

Organiser – South Team



0414 225 851

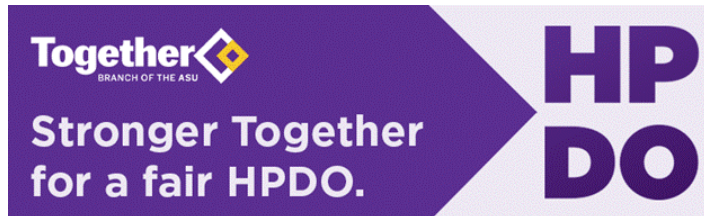
heath.moran@together.org.au

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From: Cory Shepherd <coryshepherd1@hotmail.com>

Sent: Tuesday, 14 July 2026 5:03 PM

To: Heath Moran <heath.moran@together.org.au>

Subject: Re: Request for contact re workplace issues

Hi Heath,

Thanks for getting back to me.

I'm handing over my appeal submissions — not to address the appeal itself, but because they give the fullest picture of the workplace issues I raised.

The documents I attach are for you to read over and identify the risks associated in the role. They are not to be shared but the information I provide in the submissions are to assist in identifying the psychosocial risks I believe that are unassessed.

Furthermore, I have no grievances with Ms Taylor personally; I don't know if the same view is shared but I could be wrong.

Happy to talk any of it through once you've read the attachments.

Form 20 is my affidavit and have I described the role and the workplace. I will send a further email of the completed ECC my Dr has completed, and HR has since told me that I may not return.

Regards,
Cory

From: Heath Moran <heath.moran@together.org.au>
Sent: Tuesday, July 14, 2026 4:04:39 pm
To: Cory Shepherd <coryshepherd1@hotmail.com>
Subject: RE: Request for contact re workplace issues

Hey Cory,

Thank you for reaching out this is very serious.

For your matter I'll be sending an urgent referral to our industrial team for assistance, if you've been put on an ECC, completed it and are still being told you cannot return, I need to make sure an industrial officer from Together looks over this before anything is actioned as I need to make sure you get the best information.

I'd be very eager to discuss your ongoing concerns for the hazards in switchboard as this is a recurring theme raised by members.

I have time to talk on Thursday morning around 9-10:30am and time sporadically tomorrow to call in between site visits, are you available any time tomorrow or Thursday morning.

Looking forward to hearing from you.

Yours in Union,

Heath Moran

Organiser – South Team



0414 225 851

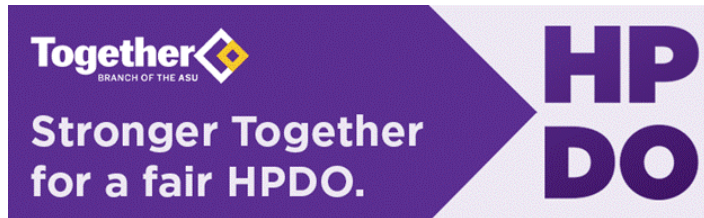
heath.moran@together.org.au

together.org.au



Looking for information? Check the [Member Portal!](#)

View our [Privacy Policy](#) here.



Together recognises and respects the Aboriginal and Torres Strait Islander peoples of the lands upon which we undertake our work to promote and defend the interests and values of members. We acknowledge the Traditional Owners and custodians who have cared for this country for thousands of years.

From: Cory Shepherd <coryshepherd1@hotmail.com>

Sent: Tuesday, 14 July 2026 10:50 AM

To: Heath Moran <heath.moran@together.org.au>

Subject: Request for contact re workplace issues

Good morning Heath,

I would like to request a time to speak with you by phone regarding some workplace issues that are causing me serious concern.

Firstly, I understand that Chloe Taylor has informed others that a full-time position will be available soon. My understanding is that this role falls within my line of work, and I would appreciate clarification about the position and my status in relation to it.

Secondly, I have been required to undergo an ECC assessment in order to return to my duties, despite having substantially performed the same role over

the past 12 months on reduced hours that I have been able to manage. This requirement was not raised when I previously returned to work following an unfair dismissal matter, which arose after I submitted a public interest disclosure in 2024 that was validated and related to concerns involving Chloe Taylor.

More recently, I was directed not to return to work until my doctor completed the ECC, which has now been done. I am nevertheless still being instructed to remain away from the workplace. I am concerned that this may be connected to my current WorkCover appeal, in which I have raised systemic risks to patient safety arising from how the switchboard is being run, and have explained in detail how those risks contributed to my time off work and to subsequent reprisal.

I would welcome the opportunity to discuss these matters with you, including the full-time position, the ECC requirements, and my ongoing exclusion from work. I would also like to share the records I hold, the risks I have identified, and why I consider Switchboard to be an unassessed psychosocial hazard, including the ways in which its current operation affects both patients and employees, together with the adverse action that I believe is presently being applied against me.

Kind regards

Cory

Process Notification - Employee

From myHR Workflow System <no-reply_cts@health.qld.gov.au>

Date Mon 2026-07-20 11:22

To Cory Shepherd <Cory.Shepherd@health.qld.gov.au>

The Smart Leave Request details below have been processed:

Personnel Assignment - 00388372

Employee Name - Cory Shepherd

Start Date : 19.07.2026

End Date : 19.07.2026

Leave details:

- Income Protection - No Pay

- No Pay

Requested by: Chloe Donovan-Taylor (371009)

Requested on: 20.07.2026

Technical Support Information:

If you encounter technical difficulties with this work item, please include the following information in your correspondence with support.

Work Item ID: 000978806568

Process Reference Number: 000024480353

New Payroll Enquiry

From noreply@health.qld.gov.au <noreply@health.qld.gov.au>

Date Wed 2026-07-22 16:23

To Cory Shepherd <Cory.Shepherd@health.qld.gov.au>; Chelsey Meighan
<Chelsey.Meighan@health.qld.gov.au>

Thank you for your payroll enquiry, the following information is supplied for your reference:

Enquiry Status

Log Number: 4438861

Current Status: Pending Investigation

Lodgement Date: 22-Jul-2026

Enquiry Employee Details

Employee PAN: 00388372

Employee Name: Cory Shepherd

Enquiry Details

Enquiry Type: Enquiry

Response to 2-15 July 2026

correspondence - incorrect application of EB12, Award and QH policies - ECC 3 July 2026 (MSH-INJ-5795)

From: Cory Shepherd

coryshepherd1@hotmail.com

To: LBH_InjuryManagement@health.qld.gov.au

LBH_InjuryManagement@health.qld.gov.au,

Lyndelle Forrest lyndelle.forrest@health.qld.gov.au

Cc: Heath Moran heath.moran@together.org.au,

James Zappia james.zappia@art.com.au

Sent: Tuesday 28 July at 17:39

Dear Michelle / Injury Management / HR,

I refer to your correspondence and Switchboard management correspondence between 2 and 15 July 2026, including:

- Chloe Taylor, 2 July 2026 — unable to "facilitate" return until a completed ECC; shifts processed as sick leave / sick leave no pay;
- my provision of the completed Employee Capability Checklist (ECC) of 3 July 2026 (Dr Day Hong Ma) while I was rostered and working;
- the same-day direction that I not attend "pending review";
- Injury Management correspondence of 7–13 July 2026 that restricted duties could not be facilitated without further medical clarification from Dr Ma;
- your email of 13 July 2026 to ART that Metro South Health was unable to facilitate my return under the ECC (MSH-INJ-5795 / CLM-317073);
- your email of 15 July 2026 (13:10) — pay sits with QSuper because of a "current" IP claim; alternative duties refused "in any capacity"; WCRA rehabilitation limited to accepted claims; return at employer discretion; further medical about psychosocial risk to my return; and
- leave coding as "income protection no pay" from 13 July 2026.

I remain ready, willing and able to work within the 3 July

2026 ECC. I do not abandon my employment. I am taking industrial advice through Together Union.

I am an HHS administrative employee. The industrial framework that applies is attached. Please respond by reference to each attachment.

ATTACHMENTS

ATT02 — Queensland Public Health Sector Certified Agreement (No. 12) 2025 (EB12) — current agreement

ATT03 — Hospital and Health Service General

Employees (Queensland Health) Award – State 2015

ATT05 — Managing the risk of psychosocial hazards at work Code of Practice 2022 (Qld)

ATT09 — QH-IMP-401-5 Workplace Rehabilitation Standard

ATT10 — QH-POL-210 Reasonable Adjustment (HR Policy G3)

ATT12 — Directive 12/24: Special Leave (applies under EB12 cl 9.12)

ATT13 — QH-POL-231 Special leave (HR Policy C7)

ATT14 — Administration Officer, Switchboard Services (AO3) Role Description

FACTS

The 3 July 2026 ECC certifies me fit for modified duties and reduced hours from 3 July 2026 as a continuation of the existing arrangement. The doctor viewed the AO3 Switchboard position description (ATT14) and recorded that usual switchboard duties remain suitable with stated modifications (including no complaint-handling; limits on peak weekday day/afternoon shifts; predictable rostering; minimum 10-hour break). I was performing work consistent with those limits when I provided the ECC. The same-day direction not to attend disrupted that work. The 28 August 2026 date on the ECC is a review of restrictions — not the start of capacity.

WHAT HAS BEEN WRONGLY APPLIED

1. "Current QSuper Income Protection claim" → ordinary pay sits with ART (your 15 July email)

Wrong application.

- An IP file is not ordinary wages for a permanent employee who is certified fit, ready and available, and directed by the employer not to attend.

- On the material available to me notified by employee (myself) May 2025 and February 2026 and now 3rd July 2026 certification of that partial capacity, ART put benefit payments are not applied from 1 June 2026. Treating the Fund as presently replacing ordinary pay is incorrect on that record. Partial capacity and partial payment does not dislodge this framing.
- IP administration is separate (Mr Zappia). It does not displace ATT02 EB12 cl 6.1 (employment security) or ATT02 EB12 cl 7.1.5(g)–(h) (injury management; management of ill or injured employees).
- Coding "IP no pay" while held out after an ECC is not a correct application of ATT02 or of ATT12/ATT13 (special leave).

2. WCRA rehabilitation only if accepted claim; return purely at "employer discretion" (your 15 July email)

Wrong application.

- Even if WCRA statutory rehab is limited to accepted claims, that does not exhaust your duties under ATT02 (EB12), ATT03 (HHS Award), ATT10 (QH-POL-210) and ATT09 (QH-IMP-401-5).
- ATT10 applies to all Queensland Health / HHS employees and includes temporary reasonable adjustment for rehabilitation/RTW and for capacity restriction — not only accepted workers' compensation claims.
- ATT02 EB12 cl 7.1.5 requires the parties to address injury management, management of ill or injured employees, fatigue, psychosocial issues and the Code of Practice 2022 (ATT05), and security for administrative staff in frontline positions.
- "Discretion" does not correctly authorise indefinite exclusion, refusal of interim duties "in any capacity," and non-payment without applying ATT09, ATT10 and ATT02 Parts 6 and 7.

3. Further medical framing me as the psychosocial risk before return "in any capacity" (your 13–15 July emails)

Wrong application of ATT05 and ATT02.

- ATT05 and ATT02 EB12 cl 7.2 require assessment of psychosocial hazards in the workplace and work system (including workplace risk assessments as requested through an HCF by employer or union).
- The 3 July ECC already identifies workplace stressors (complaint-handling; blame for others' failures;

rostering/fatigue; unresolved line-management matters). Those are workplace issues under ATT05 and ATT02 cl 7.1.5(j) and 7.2.

- Framing psychosocial risk as a personal barrier inside Injury Management / QSuper misapplies ATT02 cl 7.2 and cl 7.1.11 (feedback when WHS matters are raised).
- Workplace psychosocial issues I have raised over more than two years are not properly dealt with by indefinite IM hold-out. That process is not the correct forum under ATT02 cl 7.2.
- I will cooperate with reasonable medical requests that are specific and linked to identified job tasks under ATT14. Open-ended further medical is not a correct precondition to all duties where ATT14 and the ECC already define capacity I have been performing.

4. "Unable to facilitate" / no alternative duties (Chloe 2 July; your 13–15 July emails)

Wrong application of ATT09, ATT10 and ATT02.

- Chloe's 2 July email made facilitation conditional on a completed ECC. I provided that ECC on 3 July while working. Facilitation was then refused and I was directed off — the opposite of facilitating return on the medical basis required.
- The ECC states fit from 3 July as continuation of the existing arrangement and that usual switchboard duties remain suitable with modifications against ATT14. Refusing all duties "in any capacity" does not correctly apply ATT10, ATT09 or ATT02 cl 7.1.5(f)–(h).
- Correct application requires identifying any specific duty and restriction said to be incompatible, what modification was considered, and what was rejected — not a blanket refusal.

5. Leave coding as sick leave / sick leave no pay / "IP no pay" while certified fit and directed not to attend

Wrong application of ATT02, ATT12 and ATT13.

- From 3 July 2026 the relevant absence is employer-directed while I am certified fit and available. That is not ordinary sick leave for that period.
- "IP no pay" is not the correct industrial answer under ATT02 when the employer has directed me off after an ECC.
- ATT02 EB12 cl 9.12 provides that Directive 12/24: Special Leave (ATT12) applies to employees covered by the Agreement. ATT13 QH-POL-231 (C7) also applies.

- Special leave under ATT12 and ATT13 must be applied for employer-directed exclusion while I am ready, willing and able within the ECC. Where the only barrier is refusal to facilitate certified duties, special leave on full pay (or ordinary pay as if at work) must be applied. Please correct coding and identify the delegate and instrument for each day from 3 July.

6. Treating 28 August 2026 as if capacity only starts then

Wrong application of the ECC itself.

- 28 August 2026 is the next review of restrictions. 3 July 2026 is the date fit to commence the certified arrangement. Capacity is certified now.

WHAT YOU MUST APPLY

ATT02 EB12 cl 6.1 — employment security

ATT02 EB12 cl 7.1.5 — injury management; ill or injured employees; fatigue; psychosocial issues + ATT05; admin frontline security

ATT02 EB12 cl 7.2 — psychosocial risk assessments of workplaces

ATT02 EB12 cl 7.1.11 — feedback on WHS matters

ATT02 EB12 cl 9.12 + ATT12 + ATT13 — Special Leave

ATT03 — hours, breaks and rostering with the Agreement

ATT09 — workplace rehabilitation / RTW pathway

ATT10 — reasonable adjustment (including temporary adjustment and capacity restriction)

ATT05 — workplace psychosocial risk

ATT14 — task match to the AO3 role the doctor viewed

REQUESTS — PLEASE RESPOND IN WRITING WITHIN 5 BUSINESS DAYS

1. Facilitate my return under the 3 July 2026 ECC (or a written suitable duties plan implementing it: start date, hours, location, tasks, review date) by applying ATT09, ATT10 and ATT02 cl 7.1.5.

2. Complete a task-by-task match of ATT14 to the 3 July ECC. If you maintain you cannot facilitate, identify each restriction and duty and what was considered and rejected.

3. Be specific about any further medical still said to be required: exact questions; which ATT14 job task each relates to; what on the existing ECC is said to be insufficient.

4. Apply special leave under ATT02 cl 9.12, ATT12 and ATT13 for employer-directed days from 3 July while certified fit and available; correct "IP no pay" / sick no pay coding; identify delegate and instrument.
5. Confirm ordinary pay treatment for periods I was ready and available within the ECC but directed not to attend.
6. Confirm compliance with the direction not to attend is not abandonment or unauthorised absence (ATT02 cl 6.1).
7. Arrange a workplace-directed psychosocial risk assessment under ATT02 cl 7.2 and ATT05.
8. Identify the decision-maker for facilitation, suitable duties, special leave and pay, and the instrument for the 3 July direction not to attend.

I will continue to provide reasonable medical information directed to placement and safety. Please make further requests specific and linked to ATT14 tasks.

My QIRC appeal WC/2024/227 is separate. ART/QSuper is being addressed with Mr Zappia.

I look forward to your written response and a meeting date to facilitate return under the attached agreements and policies.

Yours sincerely,
Cory Lea Shepherd
[0417 400 227](tel:0417400227)
MSH-INJ-5795

**RE: Response to 2-15 July 2026
correspondence - incorrect application
of EB12, Award and QH policies - ECC 3
July 2026 (MSH-INJ-5795)**

From: LBH Injury Management

LBH_InjuryManagement@health.qld.gov.au

To: Cory Shepherd coryshepherd1@hotmail.com,
Lyndelle Forrest

Lyndelle.Forrest@health.qld.gov.au

Cc: Heath Moran heath.moran@together.org.au,
James Zappia james.zappia@art.com.au

Sent: Wednesday 29 July at 16:10

Good afternoon Cory,

Thank you for your email dated 28 July 2026. We acknowledge receipt of your correspondence and the attached documentation.

As previously advised, the Injury Management Team requires further information from your treating medical practitioner before we are able to determine whether Metro South Health can safely facilitate your return to duties.

The Employee Capabilities Checklist (ECC) provided has been reviewed; however, further clarification is required regarding your functional capacity and the medical restrictions identified to enable an informed assessment of whether suitable duties can be accommodated safely and operationally.

This request is consistent with Metro South Health's obligations under the *Work Health and Safety Act 2011 (Qld)* to ensure, so far as is reasonably practicable, the health and safety of workers and others in the workplace. While your treating medical practitioner's advice is an important consideration, Queensland Health has an independent obligation to assess the medical information provided alongside the inherent requirements of the role and determine whether duties can be safely facilitated.

To assist in progressing this matter, please confirm whether you would prefer to take our request for further medical information to your treating general practitioner yourself, or alternatively, complete and return the attached Employee Authority Form (EAF) authorising the Injury Management Team to write directly to your treating practitioner. Should you choose the latter option, you will be copied into our correspondence with your treating practitioner for your records.

Once the requested medical clarification has been received, we will review the information and advise you of the outcome regarding your capacity to return to duties.

Kindest Regards

Michelle Harrison

Injury

Management Consultant | LBHS HR Team 1

Human Resources

Logan and Beaudesert Hospital and Health Service

4/3-5 University Drive, Meadowbrook Qld 4131

[Metro South Health](#) |   

e. lbh_injurymanagement@health.qld.gov.au | [Chat on Teams](#)
p. [07 3542 2744](tel:0735422744)
a. Mon-Fri 8am - 4pm

Metro South Health recognises and pays respect to the traditional custodians of the land and waters – the Yugambeh, Quandamooka, Jaggera, Ugarapul, Turrbal, and Mununjali peoples – and to Elders, past, present and emerging.



Please be advised that we are currently transitioning into Solv, a new Injury Management Case System. Our documents and communications may change appearance slightly and you may even receive communications from us via the @[Solv.com.au](#) email pathway, as this is a new email pathway, please ensure you are regularly checking your Junk folder in your email accounts and save the @solv to your frequent emails so you do not

miss any communications. Please note these emails are legitimate and are from us here at lbh_injurymanagement@health.qld.gov.au. If you have any concerns, please reach out to us via the above contact details.

-----Original Message-----

From: Cory Shepherd <coryshepherd1@hotmail.com>

Sent: Tuesday, 28 July 2026 5:40 PM

To: LBH Injury Management

<LBH_injurymanagement@health.qld.gov.au>; Lyndelle

Forrest <Lyndelle.Forrest@health.qld.gov.au>

Cc: Heath Moran <heath.moran@together.org.au>; James

Zappia <james.zappia@art.com.au>

Subject: Response to 2-15 July 2026 correspondence -
incorrect application of EB12, Award and QH policies - ECC
3 July 2026 (MSH-INJ-5795)

This email originated from outside Queensland Health. DO
NOT click on any links or open attachments unless you
recognise the sender and know the content is safe.

Dear Michelle / Injury Management / HR,

I refer to your correspondence and Switchboard
management correspondence between 2 and 15 July
2026, including:

- Chloe Taylor, 2 July 2026 — unable to "facilitate" return until a completed ECC; shifts processed as sick leave / sick leave no pay;
- my provision of the completed Employee Capability Checklist (ECC) of 3 July 2026 (Dr Day Hong Ma) while I was rostered and working;
- the same-day direction that I not attend "pending review";
- Injury Management correspondence of 7–13 July 2026 that restricted duties could not be facilitated without further medical clarification from Dr Ma;
- your email of 13 July 2026 to ART that Metro South Health was unable to facilitate my return under the ECC (MSH-INJ-5795 / CLM-317073);
- your email of 15 July 2026 (13:10) — pay sits with QSuper because of a "current" IP claim; alternative duties refused "in any capacity"; WCRA rehabilitation limited to accepted claims; return at employer discretion; further medical about psychosocial risk to my return; and
- leave coding as "income protection no pay" from 13 July 2026.

I remain ready, willing and able to work within the 3 July 2026 ECC. I do not abandon my employment. I am taking industrial advice through Together Union.

I am an HHS administrative employee. The industrial framework that applies is attached. Please respond by reference to each attachment.

ATTACHMENTS

ATT02 — Queensland Public Health Sector Certified Agreement (No. 12) 2025 (EB12) — current agreement
ATT03 — Hospital and Health Service General Employees (Queensland Health) Award – State 2015
ATT05 — Managing the risk of psychosocial hazards at work Code of Practice 2022 (Qld)
ATT09 — QH-IMP-401-5 Workplace Rehabilitation Standard
ATT10 — QH-POL-210 Reasonable Adjustment (HR Policy G3)
ATT12 — Directive 12/24: Special Leave (applies under EB12 cl 9.12)
ATT13 — QH-POL-231 Special leave (HR Policy C7)
ATT14 — Administration Officer, Switchboard Services (AO3) Role Description

FACTS

The 3 July 2026 ECC certifies me fit for modified duties and reduced hours from 3 July 2026 as a continuation of the existing arrangement. The doctor viewed the AO3 Switchboard position description (ATT14) and recorded that usual switchboard duties remain suitable with stated modifications (including no complaint-handling; limits on peak weekday day/afternoon shifts; predictable rostering; minimum 10-hour break). I was performing work consistent with those limits when I provided the ECC. The same-day direction not to attend disrupted that work. The 28 August 2026 date on the ECC is a review of restrictions — not the start of capacity.

WHAT HAS BEEN WRONGLY APPLIED

1. "Current QSuper Income Protection claim" → ordinary pay sits with ART (your 15 July email)

Wrong application.

- An IP file is not ordinary wages for a permanent employee who is certified fit, ready and available, and directed by the employer not to attend.

- On the material available to me notified by employee (myself) May 2025 and February 2026 and now 3rd July 2026 certification of that partial capacity, ART put benefit payments are not applied from 1 June 2026. Treating the Fund as presently replacing ordinary pay is incorrect on that record. Partial capacity and partial payment does not dislodge this framing.
- IP administration is separate (Mr Zappia). It does not displace ATT02 EB12 cl 6.1 (employment security) or ATT02 EB12 cl 7.1.5(g)–(h) (injury management; management of ill or injured employees).
- Coding "IP no pay" while held out after an ECC is not a correct application of ATT02 or of ATT12/ATT13 (special leave).

2. WCRA rehabilitation only if accepted claim; return purely at "employer discretion" (your 15 July email)

Wrong application.

- Even if WCRA statutory rehab is limited to accepted claims, that does not exhaust your duties under ATT02 (EB12), ATT03 (HHS Award), ATT10 (QH-POL-210) and ATT09 (QH-IMP-401-5).
- ATT10 applies to all Queensland Health / HHS employees and includes temporary reasonable adjustment for rehabilitation/RTW and for capacity restriction — not only accepted workers' compensation claims.
- ATT02 EB12 cl 7.1.5 requires the parties to address injury management, management of ill or injured employees, fatigue, psychosocial issues and the Code of Practice 2022 (ATT05), and security for administrative staff in frontline positions.
- "Discretion" does not correctly authorise indefinite exclusion, refusal of interim duties "in any capacity," and non-payment without applying ATT09, ATT10 and ATT02 Parts 6 and 7.

3. Further medical framing me as the psychosocial risk before return "in any capacity" (your 13–15 July emails)

Wrong application of ATT05 and ATT02.

- ATT05 and ATT02 EB12 cl 7.2 require assessment of psychosocial hazards in the workplace and work system (including workplace risk assessments as requested through an HCF by employer or union).
- The 3 July ECC already identifies workplace stressors (complaint-handling; blame for others' failures; rostering/fatigue; unresolved line-management matters).

Those are workplace issues under ATT05 and ATT02 cl

7.1.5(j) and 7.2.

- Framing psychosocial risk as a personal barrier inside Injury Management / QSuper misapplies ATT02 cl 7.2 and cl 7.1.11 (feedback when WHS matters are raised).
- Workplace psychosocial issues I have raised over more than two years are not properly dealt with by indefinite IM hold-out. That process is not the correct forum under ATT02 cl 7.2.
- I will cooperate with reasonable medical requests that are specific and linked to identified job tasks under ATT14. Open-ended further medical is not a correct precondition to all duties where ATT14 and the ECC already define capacity I have been performing.

4. "Unable to facilitate" / no alternative duties (Chloe 2 July; your 13–15 July emails)

Wrong application of ATT09, ATT10 and ATT02.

- Chloe's 2 July email made facilitation conditional on a completed ECC. I provided that ECC on 3 July while working. Facilitation was then refused and I was directed off — the opposite of facilitating return on the medical basis required.
- The ECC states fit from 3 July as continuation of the existing arrangement and that usual switchboard duties remain suitable with modifications against ATT14. Refusing all duties "in any capacity" does not correctly apply ATT10, ATT09 or ATT02 cl 7.1.5(f)–(h).
- Correct application requires identifying any specific duty and restriction said to be incompatible, what modification was considered, and what was rejected — not a blanket refusal.

5. Leave coding as sick leave / sick leave no pay / "IP no pay" while certified fit and directed not to attend

Wrong application of ATT02, ATT12 and ATT13.

- From 3 July 2026 the relevant absence is employer-directed while I am certified fit and available. That is not ordinary sick leave for that period.
- "IP no pay" is not the correct industrial answer under ATT02 when the employer has directed me off after an ECC.
- ATT02 EB12 cl 9.12 provides that Directive 12/24: Special Leave (ATT12) applies to employees covered by the Agreement. ATT13 QH-POL-231 (C7) also applies.

- Special leave under ATT12 and ATT13 must be applied for employer-directed exclusion while I am ready, willing and able within the ECC. Where the only barrier is refusal to facilitate certified duties, special leave on full pay (or ordinary pay as if at work) must be applied. Please correct coding and identify the delegate and instrument for each day from 3 July.

6. Treating 28 August 2026 as if capacity only starts then

Wrong application of the ECC itself.

- 28 August 2026 is the next review of restrictions. 3 July 2026 is the date fit to commence the certified arrangement. Capacity is certified now.

WHAT YOU MUST APPLY

ATT02 EB12 cl 6.1 — employment security

ATT02 EB12 cl 7.1.5 — injury management; ill or injured employees; fatigue; psychosocial issues + ATT05; admin frontline security

ATT02 EB12 cl 7.2 — psychosocial risk assessments of workplaces

ATT02 EB12 cl 7.1.11 — feedback on WHS matters

ATT02 EB12 cl 9.12 + ATT12 + ATT13 — Special Leave

ATT03 — hours, breaks and rostering with the Agreement

ATT09 — workplace rehabilitation / RTW pathway

ATT10 — reasonable adjustment (including temporary adjustment and capacity restriction)

ATT05 — workplace psychosocial risk

ATT14 — task match to the AO3 role the doctor viewed

REQUESTS — PLEASE RESPOND IN WRITING WITHIN 5 BUSINESS DAYS

1. Facilitate my return under the 3 July 2026 ECC (or a written suitable duties plan implementing it: start date, hours, location, tasks, review date) by applying ATT09, ATT10 and ATT02 cl 7.1.5.

2. Complete a task-by-task match of ATT14 to the 3 July ECC. If you maintain you cannot facilitate, identify each restriction and duty and what was considered and rejected.

3. Be specific about any further medical still said to be required: exact questions; which ATT14 job task each relates to; what on the existing ECC is said to be insufficient.

- 4. Apply special leave under ATT02 cl 9.12, ATT12 and ATT13 for employer-directed days from 3 July while certified fit and available; correct "IP no pay" / sick no pay coding; identify delegate and instrument.
- 5. Confirm ordinary pay treatment for periods I was ready and available within the ECC but directed not to attend.
- 6. Confirm compliance with the direction not to attend is not abandonment or unauthorised absence (ATT02 cl 6.1).
- 7. Arrange a workplace-directed psychosocial risk assessment under ATT02 cl 7.2 and ATT05.
- 8. Identify the decision-maker for facilitation, suitable duties, special leave and pay, and the instrument for the 3 July direction not to attend.

I will continue to provide reasonable medical information directed to placement and safety. Please make further requests specific and linked to ATT14 tasks.

My QIRC appeal WC/2024/227 is separate. ART/QSuper is being addressed with Mr Zappia.

I look forward to your written response and a meeting date to facilitate return under the attached agreements and policies.

Yours sincerely,
Cory Lea Shepherd
[0417 400 227](tel:0417400227)
MSH-INJ-5795

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scanning and blocking of emails and attachments
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Health for the purposes of operating, protecting,
maintaining and ensuring appropriate use of its
computer network.

**Re: Response to 2-15 July 2026
correspondence - incorrect application
of EB12, Award and QH policies - ECC 3
July 2026 (MSH-INJ-5795)**

From: Cory Shepherd

coryshepherd1@hotmail.com

To: LBH Injury Management

LBH_InjuryManagement@health.qld.gov.au,

Lyndelle Forrest

Lyndelle.Forrest@health.qld.gov.au

Cc: Heath Moran heath.moran@together.org.au,

James Zappia james.zappia@art.com.au

Sent: Thursday 30 July at 11:25

Good morning Michelle,

Thank you for your email of 29 July 2026.

This email is sent on Thursday 30 July 2026. My appointment with Dr Ma is Friday 31 July 2026 at 2:00 pm. The Health Service's questions and completed forms (or the same information by email) are required before that time.

THE SEQUENCE

I was working my rostered shifts. Following my request to temporarily reduce my ordinary hours from 0.6 to 0.5 FTE for two weeks, I was told I could not work until I completed an Employee Capability Checklist. I obtained and provided that checklist on 3 July 2026. I was then told I still could not work, pending a review. The requirement has since changed again — to further medical clarification, for which no timeframe has been given.

I was not absent and I was not seeking to recommence. I had been performing the role on reduced hours within my now certified restrictions for approximately twelve

months. I mention this because the questions to Dr Ma should be directed to that situation.

I have elected to take the request to Dr Ma myself, and I have booked an appointment for 2:00 pm on Friday 31 July 2026.

WHAT I NEED BEFORE 2:00 PM FRIDAY 31 JULY 2026

1. The Health Service's questions for Dr Ma, or completed Attachment 1.
2. Completed Attachment 2, or the same information by email.
3. Confirmation of who will be responding before the appointment.
4. Reimbursement of the \$150.00 I have already paid, and the name and address to which the practice should direct the invoice for Friday's consultation.

If the forms are not convenient, the same information in an email is equally welcome — before 2:00 pm Friday.

If the questions are not received in time, I will attend the appointment regardless and ask Dr Ma to address my capacity and restrictions against the duties in the role description (attached). I will forward his response to you. I would prefer to put the Health Service's own questions to him, which is why I ask now.

ABOUT THE ATTACHMENTS

To make this as straightforward as possible, I have attached the following:

Attachment 1 — Duty / Restriction Mapping — sets out the twelve key responsibilities listed in the AO3 role description (also attached). If the Health Service indicates which duty or duties it says I cannot perform, and which restriction is said to prevent it, that identifies precisely what Dr Ma needs to address.

Attachment 2 — Psychosocial Hazard Identification — follows the structure of the Managing the risk of psychosocial hazards at work Code of Practice 2022:

hazard, source in the work, risk, assessment, controls, consultation. The form also provides for the Health Service to confirm the position if no hazard is identified.

Attachment 3 — Invoice 574370 and payment receipt for the \$150.00 case conference of 3 July 2026 (already paid).

Attachment 4 — AO3 Switchboard Services role description (source of the duties in Attachment 1).

Completing and returning Attachment 1 and/or Attachment 2 (or the same information by email) would give me what I need to put to Dr Ma for this step.

1. MY ELECTION

1.1 I will take the request to Dr Ma myself rather than add a further step.

1.2 I do not propose to complete the Employee Authority Form. It is broader than the present request — it extends to disclosures to and from WorkCover Queensland and income protection insurers, and it is open-ended in duration. If a written authority is needed for Dr Ma to respond to a specific request, I am happy to provide one confined to that purpose.

2. COST OF THE CONSULTATIONS

2.1 The consultation of 3 July 2026 was invoiced to me personally — My Doctors Clinic Surfers, invoice 574370, dated 3 July 2026, \$150.00 (item WCO002, case conference). I paid that invoice in full on 10 July 2026 (receipt N471065713633, reference 574370 C Shepherd). The invoice and payment receipt are attached as one document (Attachment 3).

2.2 A further consultation is booked for 2:00 pm on Friday 31 July 2026 and is expected to attract the same fee.

2.3 Both consultations are required by the Health Service for its own assessment; neither is material I

have volunteered. Consistently with the approach taken elsewhere in the public sector, where the entity that requires medical information meets the reasonable costs of obtaining it, I ask:

(a) that the \$150.00 I have paid be reimbursed to me; and

(b) that the Health Service provide the name and address to which the practice should direct Friday's invoice, so that the cost of the Health Service's own request is not again met from my funds.

2.4 If the Health Service does not intend to meet these costs, could you please confirm that in writing, together with the basis for that position.

3. THE QUESTIONS I NEED

3.1 Your email states that "further clarification is required regarding your functional capacity and the medical restrictions identified", but does not identify what is unclear. Dr Ma cannot respond to a request in those terms.

3.2 Could you please provide, in writing, before 2:00 pm Friday:

(a) the specific questions the Health Service asks Dr Ma to answer, set out as questions;

(b) for each question, the duty or task to which it relates — Attachment 1 is provided for this purpose;

(c) what in the 3 July Employee Capability Checklist is said to be insufficient — which entry, and why it does not enable the assessment; and

(d) who will consider Dr Ma's response, and the timeframe within which the decision about my duties will then be made.

4. THE ORIGIN OF THE REQUIREMENT

4.1 Could you please confirm the date on which, and the basis on which, the Health Service required me to complete an Employee Capability Checklist. My understanding is that the requirement was raised in response to my request to temporarily reduce my hours. I am not aware of any incident, complaint or performance concern preceding it.

4.2 Could you also please confirm the outcome of that request to reduce my hours, and if no decision has been made, when one will be made and by whom. EB12 clause 10.3 provides that an employee may ask for a change in the way they work, including their ordinary hours of work; that the request may be refused only on reasonable grounds; and that the employer must give written notice of its decision, with reasons, within 21 days.

5. THE RESTRICTIONS AND THE DUTIES (Attachment 1)

5.1 Since your email refers to the inherent requirements of the role, I have compared the restrictions certified on 3 July with the responsibilities set out in the AO3 role description, which Dr Ma reviewed before completing the checklist. Attachment 1 lists those twelve responsibilities as they appear in that document.

5.2 First, complaint-handling. The ECC of 3 July 2026 records: no complaint-handling duties; complaints received at the Switchboard to be logged and redirected, not actioned or resolved by me. That restriction has already been addressed in correspondence. In her email of 7 July 2026, Ms Forrest stated that the established process is for patient concerns to be redirected to Consumer Liaison Services (CLS); that there is no expectation for Switchboard operators to manage or deal with those matters; and that where a consumer raises concerns about a Switchboard employee, those concerns are referred to the Manager, Switchboard or, in her absence, CLS. My reply of 9 July 2026 noted that this confirmation, together with existing rostering practice, appears to accommodate that ECC recommendation. I treat complaint-handling as closed on the Health

Service's own account of the role. It does not need to be reopened with Dr Ma, and it is not a basis for refusing all duties in Attachment 1.

5.3 Second, the remaining restrictions — the limits on peak weekday day and afternoon shifts, predictable rostering, and a minimum ten-hour break between shifts — go to when I work, not to which duties I perform. They remove none of the responsibilities listed in Attachment 1.

5.4 I note the role description records the position as continuous shift work. I performed that role, on reduced hours and within my certified restrictions, for approximately twelve months, and the Health Service's correspondence of 7 July 2026 records that my restrictions were already being accommodated. If the rostering restrictions are now said to be incompatible with the role, when they were not during that period, could you please identify what has changed.

5.5 Third, reduced ordinary hours are expressly contemplated by the industrial instruments. Clause 8.2 of the Hospital and Health Service General Employees (Queensland Health) Award – State 2015 provides for part-time employment on a regular number of ordinary hours, pro rata to full-time conditions, with a minimum of eight ordinary hours per fortnight. A reduction in hours does not of itself remove any duty from the role. Could you please identify why the reduced hours certified on 3 July are treated differently — that is, what it is about those hours, as distinct from the reasons for them, that prevents the duties of the position from being performed.

5.6 QH-POL-210 applies to all Queensland Health and Hospital and Health Service employees and directs adjustments — including temporary adjustment for rehabilitation and return to work, and for capacity restriction — to enabling an employee to safely perform the genuine occupational requirements of the role. It reflects the Anti-Discrimination Act 1991 (Qld), which contemplates reasonable terms and special conditions where job capacity is restricted by impairment (s 34)

and permits departure only where unjustifiable hardship is established (ss 5, 35). QH-IMP-401-5 makes managers responsible for identifying suitable duties, guided by medical information, and its application is not limited to accepted workers' compensation claims. Each operates at the level of identified duties, which is what Attachment 1 is directed to.

6. THE CHARACTERISATION OF RISK (Attachment 2)

6.1 Your correspondence of 13, 15 and 29 July 2026 refers to whether my return can be "safely" facilitated, whether the matters identified by Dr Ma "present any psychosocial hazards or risks", and to the health and safety of workers "and others in the workplace". I do not suggest the Health Service should not satisfy itself as to safety.

6.2 However, in the twenty-seven days since 3 July, no incident, complaint or hazard has been identified to me. None was raised during the twelve months I performed the role on reduced hours. No finding of any kind has been made in relation to my performance of this role.

6.3 EB12 clause 7.2 and the Code of Practice proceed by identifying a hazard, assessing the risk, and considering controls, in consultation with workers and health and safety representatives — and they identify psychosocial hazards by reference to features of work and its management. Attachment 2 follows that structure.

6.4 Could you please confirm whether it is asserted that my performing my duties within the certified restrictions would present a risk to me or to any other person and, if so, whether the hazard is said to arise from the work and work system or from me, together with the material relied upon. Attachment 2 provides for both. If no such assertion is intended, could you please confirm that in writing.

6.5 For clarity, my cooperation with the medical request does not suspend the other matters: the restoration of

my roster, the treatment of my pay and leave, and the workplace psychosocial assessment remain live in parallel.

7. CONFLICT OF INTEREST

7.1 I raise one further matter, without alleging wrongdoing.

7.2 The requirement that I complete an Employee Capability Checklist was raised by my line manager, Ms Taylor. Ms Taylor is the subject of a complaint which the Ethical Standards Unit determined to be a public interest disclosure, and her conduct is a matter in issue in proceedings before the Queensland Industrial Relations Commission in which she may give evidence.

7.3 The Code of Conduct for the Queensland Public Service provides at clause 1.2 that having a conflict of interest is not unusual and is not wrongdoing in itself, but that failing to disclose and manage it appropriately is likely to be wrongdoing, and that employees will disclose any personal interest that could be seen as influencing the performance of their duties.

7.4 Could you please confirm what involvement Ms Taylor has had in the decisions concerning my attendance, duties and pay since 2 July 2026; whether any conflict of interest has been disclosed and managed, and how; and that decisions concerning my attendance, duties and pay will be made by a person without involvement in those proceedings.

8. SCOPE AND MEETING

8.1 I will provide Dr Ma with the questions and the role description. My consent extends to information concerning my capacity, my restrictions, and their operational application to the duties of my position, and I ask that the questions be limited to what is necessary for that purpose.

8.2 If it would assist, I am willing to meet at short notice — including before Friday — to work through

Attachment 1 duty by duty. I would ask that any meeting occur alongside, rather than in place of, a written response.

9. OTHER MATTERS

9.1 The matters raised in my emails of 28 July 2026 remain outstanding within the period identified in that correspondence. I do not repeat them here, and this email does not replace them.

10. WHAT I ASK

10.1 So that this can be resolved simply, I ask:

(a) that my roster be provided to me and that I be restored to my rostered shifts within the certified restrictions;

(b) that the rostered shifts from 3 July 2026 be processed by ad hoc payment, and the leave debited for that period be re-credited;

(c) that the \$150.00 I have paid for the 3 July consultation be reimbursed; and

(d) that the invoice for Friday's consultation be directed to the Health Service at an address you provide.

10.2 I have facilitated every step asked of me — I obtained the checklist the Health Service required and paid for it myself, I have elected to take its request to Dr Ma myself, I have booked the appointment, and I have prepared the attached forms so that the Health Service's questions can be framed quickly. I am conscious that time and cost are accumulating for both of us, and I would prefer to resolve this now, collaboratively, rather than through further process.

11. IN SUMMARY

11.1 The operative decision in this matter is the direction that I not attend work. Everything since follows from it — the treatment of my rostered shifts, the

refusal of duties in any capacity, and the requirement for further medical information.

11.2 A direction to an employee not to attend work is an exercise of authority. I have asked on more than one occasion for the source of that authority to be identified, and it has not been. Nor have I been told the process under which I am being managed, or the person who made the decision.

11.3 I raise this not as a complaint, but because without it I am unable to understand or respond to the basis on which I am being kept from my substantive position.

11.4 I ask that any further decision concerning my attendance, duties or pay identify the power relied upon, the person making it, and the reasons for it.

I remain certified fit, ready and available to work within the 3 July 2026 Employee Capability Checklist.

Kind regards,

Cory Lea Shepherd

Administration Officer (AO3), Switchboard Services

0417 400 227

MSH-INJ-5795

From: LBH Injury Management

<LBH_InjuryManagement@health.qld.gov.au>

Sent: Wednesday, July 29, 2026 4:10:13 pm

To: Cory Shepherd <coryshepherd1@hotmail.com>;

Lyndelle Forrest <Lyndelle.Forrest@health.qld.gov.au>

Cc: Heath Moran <heath.moran@together.org.au>;

James Zappia <james.zappia@art.com.au>

Subject: RE: Response to 2-15 July 2026

correspondence - incorrect application of EB12, Award and QH policies - ECC 3 July 2026 (MSH-INJ-5795)

Good afternoon Cory,

Thank you for your email dated 28 July 2026. We acknowledge receipt of your correspondence and the attached documentation.

As previously advised, the Injury Management Team requires further information from your treating medical practitioner before we are able to determine whether Metro South Health can safely facilitate your return to duties.

The Employee Capabilities Checklist (ECC) provided has been reviewed; however, further clarification is required regarding your functional capacity and the medical restrictions identified to enable an informed assessment of whether suitable duties can be accommodated safely and operationally.

This request is consistent with Metro South Health's obligations under the *Work Health and Safety Act 2011 (Qld)* to ensure, so far as is reasonably practicable, the health and safety of workers and others in the workplace. While your treating medical practitioner's advice is an important consideration, Queensland Health has an independent obligation to assess the medical information provided alongside the inherent requirements of the role and determine whether duties can be safely facilitated.

To assist in progressing this matter, please confirm whether you would prefer to take our request for further medical information to your treating general practitioner yourself, or alternatively, complete and return the attached Employee Authority Form (EAF) authorising the Injury Management Team to write directly to your treating practitioner. Should you choose the latter option, you will be copied into our correspondence with your treating practitioner for your records.

Once the requested medical clarification has been received, we will review the information and advise you of the outcome regarding your capacity to return to duties.

Kindest Regards

Michelle Harrison

Injury

Management Consultant | LBHS HR Team 1

Human Resources

Logan and Beaudesert Hospital and Health Service

lbh_injurymanagement@health.qld.gov.au | [Chat on Teams](#)
e. qld.gov.au | [ams](#)
p. 07 3542 2744
a. Mon-Fri 8am - 4pm

Metro South Health recognises and pays respect to the traditional custodians of the land and waters – the Yugambeh, Quandamooka, Jaggera, Ugarapul, Turrbal, and Mununjali peoples – and to Elders, past, present and emerging.



Please be advised that we are currently transitioning into Solv, a new Injury Management Case System. Our documents and communications may change appearance slightly and you may even receive communications from us via the @Solv.com.au email pathway, as this is a new email pathway, please ensure you are regularly checking your Junk folder in your email accounts and save the @solv to your frequent emails so you do not miss any communications. Please note these emails are legitimate and are from us here at lbh_injurymanagement@health.qld.gov.au. If you have any concerns, please reach out to us via the above contact details.

-----Original Message-----

From: Cory Shepherd <coryshepherd1@hotmail.com>

Sent: Tuesday, 28 July 2026 5:40 PM

To: LBH Injury Management

<LBH_injurymanagement@health.qld.gov.au>; Lyndelle

Forrest <Lyndelle.Forrest@health.qld.gov.au>

Cc: Heath Moran <heath.moran@together.org.au>; James

Zappia <james.zappia@art.com.au>

Subject: Response to 2-15 July 2026 correspondence -

incorrect application of EB12, Award and QH policies - ECC

3 July 2026 (MSH-INJ-5795)

This email originated from outside Queensland Health. DO NOT click on any links or open attachments unless you recognise the sender and know the content is safe.

Dear Michelle / Injury Management / HR,

I refer to your correspondence and Switchboard management correspondence between 2 and 15 July 2026, including:

- Chloe Taylor, 2 July 2026 — unable to "facilitate" return until a completed ECC; shifts processed as sick leave / sick leave no pay;
- my provision of the completed Employee Capability Checklist (ECC) of 3 July 2026 (Dr Day Hong Ma) while I was rostered and working;
- the same-day direction that I not attend "pending review";
- Injury Management correspondence of 7–13 July 2026 that restricted duties could not be facilitated without further medical clarification from Dr Ma;
- your email of 13 July 2026 to ART that Metro South Health was unable to facilitate my return under the ECC (MSH-INJ-5795 / CLM-317073);
- your email of 15 July 2026 (13:10) — pay sits with QSuper because of a "current" IP claim; alternative duties refused "in any capacity"; WCRA rehabilitation limited to accepted claims; return at employer discretion; further medical about psychosocial risk to my return; and
- leave coding as "income protection no pay" from 13 July 2026.

I remain ready, willing and able to work within the 3 July 2026 ECC. I do not abandon my employment. I am taking industrial advice through Together Union.

I am an HHS administrative employee. The industrial framework that applies is attached. Please respond by reference to each attachment.

ATTACHMENTS

ATT02 — Queensland Public Health Sector Certified Agreement (No. 12) 2025 (EB12) — current agreement

ATT03 — Hospital and Health Service General Employees (Queensland Health) Award – State 2015

ATT05 — Managing the risk of psychosocial hazards at work Code of Practice 2022 (Qld)

ATT09 — QH-IMP-401-5 Workplace Rehabilitation Standard

ATT10 — QH-POL-210 Reasonable Adjustment (HR Policy G3)

ATT12 — Directive 12/24: Special Leave (applies under EB12 cl 9.12)

ATT13 — QH-POL-231 Special leave (HR Policy C7)

ATT14 — Administration Officer, Switchboard Services

(AO3) Role Description

FACTS

The 3 July 2026 ECC certifies me fit for modified duties and reduced hours from 3 July 2026 as a continuation of the existing arrangement. The doctor viewed the AO3 Switchboard position description (ATT14) and recorded that usual switchboard duties remain suitable with stated modifications (including no complaint-handling; limits on peak weekday day/afternoon shifts; predictable rostering; minimum 10-hour break). I was performing work consistent with those limits when I provided the ECC. The same-day direction not to attend disrupted that work. The 28 August 2026 date on the ECC is a review of restrictions — not the start of capacity.

WHAT HAS BEEN WRONGLY APPLIED

1. "Current QSuper Income Protection claim" → ordinary pay sits with ART (your 15 July email)

Wrong application.

- An IP file is not ordinary wages for a permanent employee who is certified fit, ready and available, and directed by the employer not to attend.
- On the material available to me notified by employee (myself) May 2025 and February 2026 and now 3rd July 2026 certification of that partial capacity, ART put benefit payments are not applied from 1 June 2026. Treating the Fund as presently replacing ordinary pay is incorrect on that record. Partial capacity and partial payment does not dislodge this framing.
- IP administration is separate (Mr Zappia). It does not displace ATT02 EB12 cl 6.1 (employment security) or ATT02 EB12 cl 7.1.5(g)–(h) (injury management; management of ill or injured employees).
- Coding "IP no pay" while held out after an ECC is not a correct application of ATT02 or of ATT12/ATT13 (special leave).

2. WCRA rehabilitation only if accepted claim; return purely at "employer discretion" (your 15 July email)

Wrong application.

- Even if WCRA statutory rehab is limited to accepted claims, that does not exhaust your duties under ATT02

(EB12), ATT03 (HHS Award), ATT10 (QH-POL-210) and ATT09 (QH-IMP-401-5).

- ATT10 applies to all Queensland Health / HHS employees and includes temporary reasonable adjustment for rehabilitation/RTW and for capacity restriction — not only accepted workers' compensation claims.
- ATT02 EB12 cl 7.1.5 requires the parties to address injury management, management of ill or injured employees, fatigue, psychosocial issues and the Code of Practice 2022 (ATT05), and security for administrative staff in frontline positions.
- "Discretion" does not correctly authorise indefinite exclusion, refusal of interim duties "in any capacity," and non-payment without applying ATT09, ATT10 and ATT02 Parts 6 and 7.

3. Further medical framing me as the psychosocial risk before return "in any capacity" (your 13–15 July emails)

Wrong application of ATT05 and ATT02.

- ATT05 and ATT02 EB12 cl 7.2 require assessment of psychosocial hazards in the workplace and work system (including workplace risk assessments as requested through an HCF by employer or union).
- The 3 July ECC already identifies workplace stressors (complaint-handling; blame for others' failures; rostering/fatigue; unresolved line-management matters). Those are workplace issues under ATT05 and ATT02 cl 7.1.5(j) and 7.2.
- Framing psychosocial risk as a personal barrier inside Injury Management / QSuper misapplies ATT02 cl 7.2 and cl 7.1.11 (feedback when WHS matters are raised).
- Workplace psychosocial issues I have raised over more than two years are not properly dealt with by indefinite IM hold-out. That process is not the correct forum under ATT02 cl 7.2.
- I will cooperate with reasonable medical requests that are specific and linked to identified job tasks under ATT14. Open-ended further medical is not a correct precondition to all duties where ATT14 and the ECC already define capacity I have been performing.

4. "Unable to facilitate" / no alternative duties (Chloe 2 July; your 13–15 July emails)

Wrong application of ATT09, ATT10 and ATT02.

- Chloe's 2 July email made facilitation conditional on a completed ECC. I provided that ECC on 3 July while

working. Facilitation was then refused and I was directed off — the opposite of facilitating return on the medical basis required.

- The ECC states fit from 3 July as continuation of the existing arrangement and that usual switchboard duties remain suitable with modifications against ATT14. Refusing all duties "in any capacity" does not correctly apply ATT10, ATT09 or ATT02 cl 7.1.5(f)–(h).
- Correct application requires identifying any specific duty and restriction said to be incompatible, what modification was considered, and what was rejected — not a blanket refusal.

5. Leave coding as sick leave / sick leave no pay / "IP no pay" while certified fit and directed not to attend

Wrong application of ATT02, ATT12 and ATT13.

- From 3 July 2026 the relevant absence is employer-directed while I am certified fit and available. That is not ordinary sick leave for that period.
- "IP no pay" is not the correct industrial answer under ATT02 when the employer has directed me off after an ECC.
- ATT02 EB12 cl 9.12 provides that Directive 12/24: Special Leave (ATT12) applies to employees covered by the Agreement. ATT13 QH-POL-231 (C7) also applies.
- Special leave under ATT12 and ATT13 must be applied for employer-directed exclusion while I am ready, willing and able within the ECC. Where the only barrier is refusal to facilitate certified duties, special leave on full pay (or ordinary pay as if at work) must be applied. Please correct coding and identify the delegate and instrument for each day from 3 July.

6. Treating 28 August 2026 as if capacity only starts then

Wrong application of the ECC itself.

- 28 August 2026 is the next review of restrictions. 3 July 2026 is the date fit to commence the certified arrangement. Capacity is certified now.

WHAT YOU MUST APPLY

ATT02 EB12 cl 6.1 — employment security

ATT02 EB12 cl 7.1.5 — injury management; ill or injured employees; fatigue; psychosocial issues + ATT05; admin frontline security

ATT02 EB12 cl 7.2 — psychosocial risk assessments of workplaces

ATT02 EB12 cl 7.1.11 — feedback on WHS matters

ATT02 EB12 cl 9.12 + ATT12 + ATT13 — Special Leave

ATT03 — hours, breaks and rostering with the Agreement

ATT09 — workplace rehabilitation / RTW pathway

ATT10 — reasonable adjustment (including temporary adjustment and capacity restriction)

ATT05 — workplace psychosocial risk

ATT14 — task match to the AO3 role the doctor viewed

REQUESTS — PLEASE RESPOND IN WRITING WITHIN 5 BUSINESS DAYS

1. Facilitate my return under the 3 July 2026 ECC (or a written suitable duties plan implementing it: start date, hours, location, tasks, review date) by applying ATT09, ATT10 and ATT02 cl 7.1.5.
2. Complete a task-by-task match of ATT14 to the 3 July ECC. If you maintain you cannot facilitate, identify each restriction and duty and what was considered and rejected.
3. Be specific about any further medical still said to be required: exact questions; which ATT14 job task each relates to; what on the existing ECC is said to be insufficient.
4. Apply special leave under ATT02 cl 9.12, ATT12 and ATT13 for employer-directed days from 3 July while certified fit and available; correct "IP no pay" / sick no pay coding; identify delegate and instrument.
5. Confirm ordinary pay treatment for periods I was ready and available within the ECC but directed not to attend.
6. Confirm compliance with the direction not to attend is not abandonment or unauthorised absence (ATT02 cl 6.1).
7. Arrange a workplace-directed psychosocial risk assessment under ATT02 cl 7.2 and ATT05.
8. Identify the decision-maker for facilitation, suitable duties, special leave and pay, and the instrument for the 3 July direction not to attend.

I will continue to provide reasonable medical information directed to placement and safety. Please make further requests specific and linked to ATT14 tasks.

My QIRC appeal WC/2024/227 is separate. ART/QSuper is being addressed with Mr Zappia.

I look forward to your written response and a meeting date to facilitate return under the attached agreements and policies.

Yours sincerely,
Cory Lea Shepherd
0417 400 227
MSH-INJ-5795

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**RE: Response to 2-15 July 2026
correspondence - incorrect application
of EB12, Award and QH policies - ECC 3
July 2026 (MSH-INJ-5795)**

From: LBH Injury Management

LBH_InjuryManagement@health.qld.gov.au

To: Cory Shepherd coryshepherd1@hotmail.com

Cc: Notes - IPEC (notes@solv.com.au)

notes@solv.com.au

Sent: Thursday 30 July at 14:33

Good afternoon Cory,

Thank you for your email.

Please note that we are currently progressing the Request for Medical Information (RFMI) correspondence for delegate approval and will endeavour to expedite this process as quickly as possible. However, we are unable to guarantee when the approved documentation will be available for release.

The usual process is for the approved RFMI documentation to be sent directly to the employee, who can then arrange an appointment with their treating medical practitioner to have the information completed and returned.

If you have any further questions in the meantime, please do not hesitate to contact us.

Kindest Regards

Michelle Harrison

Injury

Management Consultant | LBHS HR Team 1

Human Resources

Logan and Beaudesert Hospital and Health Service

4/3-5 University Drive, Meadowbrook Qld 4131

[Metro South Health](#) |   

- lbh_injurymangement@health.qld.gov.au | [Chat on Teams](#)
e. lbh_injurymangement@health.qld.gov.au
p. 07 3542 2744
a. Mon-Fri 8am - 4pm

Metro South Health recognises and pays respect to the traditional custodians of the land and waters – the Yugambeh, Quandamooka, Jaggera, Ugarapul, Turrbal, and Mununjali peoples – and to Elders, past, present and emerging.



Please be advised that we are currently transitioning into Solv, a new Injury Management Case System. Our documents and communications may change appearance slightly and you may even receive communications from us via the @Solv.com.au email pathway, as this is a new email pathway, please ensure you are regularly checking your Junk folder in your email accounts and save the @solv to your frequent emails so you do not miss any communications. Please note these emails are legitimate and are from us here at lbh_injurymangement@health.qld.gov.au. If you have any concerns, please reach out to us via the above contact details.

From: Cory Shepherd <coryshepherd1@hotmail.com>
Sent: Thursday, 30 July 2026 11:26 AM
To: LBH Injury Management
<LBH_injurymangement@health.qld.gov.au>; Lyndelle Forrest <Lyndelle.Forrest@health.qld.gov.au>
Cc: Heath Moran <heath.moran@together.org.au>; James Zappia <james.zappia@art.com.au>
Subject: Re: Response to 2-15 July 2026 correspondence - incorrect application of EB12, Award and QH policies - ECC 3 July 2026 (MSH-INJ-5795)

This email originated from outside Queensland Health. DO NOT click on any links or open attachments unless you recognise the sender and know the content is safe.

Good morning Michelle,

Thank you for your email of 29 July 2026.

This email is sent on Thursday 30 July 2026. My appointment with Dr Ma is Friday 31 July 2026 at 2:00 pm. The Health Service's questions and completed forms (or the same information by email) are required before that time.

THE SEQUENCE

I was working my rostered shifts. Following my request to temporarily reduce my ordinary hours from 0.6 to 0.5 FTE for two weeks, I was told I could not work until I completed an Employee Capability Checklist. I obtained and provided that checklist on 3 July 2026. I was then told I still could not work, pending a review. The requirement has since changed again — to further medical clarification, for which no timeframe has been given.

I was not absent and I was not seeking to recommence. I had been performing the role on reduced hours within my now certified restrictions for approximately twelve months. I mention this because the questions to Dr Ma should be directed to that situation.

I have elected to take the request to Dr Ma myself, and I have booked an appointment for 2:00 pm on Friday 31 July 2026.

WHAT I NEED BEFORE 2:00 PM FRIDAY 31 JULY 2026

1. The Health Service's questions for Dr Ma, or completed Attachment 1.
2. Completed Attachment 2, or the same information by email.
3. Confirmation of who will be responding before the appointment.
4. Reimbursement of the \$150.00 I have already paid, and the name and address to which the practice should direct the invoice for Friday's consultation.

If the forms are not convenient, the same information in an email is equally welcome — before 2:00 pm Friday.

If the questions are not received in time, I will attend the appointment regardless and ask Dr Ma to address my capacity and restrictions against the duties in the role description (attached). I will forward his response to you. I would prefer to put the Health Service's own questions to him, which is why I ask now.

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To make this as straightforward as possible, I have attached the following:

Attachment 1 — Duty / Restriction Mapping — sets out the twelve key responsibilities listed in the AO3 role description (also attached). If the Health Service indicates which duty or duties it says I cannot perform, and which restriction is said to prevent it, that identifies precisely what Dr Ma needs to address.

Attachment 2 — Psychosocial Hazard Identification — follows the structure of the Managing the risk of psychosocial hazards at work Code of Practice 2022: hazard, source in the work, risk, assessment, controls, consultation. The form also provides for the Health Service to confirm the position if no hazard is identified.

Attachment 3 — Invoice 574370 and payment receipt for the \$150.00 case conference of 3 July 2026 (already paid).

Attachment 4 — AO3 Switchboard Services role description (source of the duties in Attachment 1).

Completing and returning Attachment 1 and/or Attachment 2 (or the same information by email) would give me what I need to put to Dr Ma for this step.

1. MY ELECTION

1.1 I will take the request to Dr Ma myself rather than add a further step.

1.2 I do not propose to complete the Employee Authority Form. It is broader than the present request — it extends to disclosures to and from WorkCover Queensland and income protection insurers, and it is open-ended in duration. If a written authority is needed for Dr Ma to respond to a specific request, I am happy to provide one confined to that purpose.

2. COST OF THE CONSULTATIONS

2.1 The consultation of 3 July 2026 was invoiced to me personally — My Doctors Clinic Surfers, invoice 574370, dated 3 July 2026, \$150.00 (item WCO002, case conference). I paid that invoice in full on 10 July 2026 (receipt N471065713633, reference 574370 C Shepherd). The invoice and payment receipt are attached as one document (Attachment 3).

2.2 A further consultation is booked for 2:00 pm on Friday 31 July 2026 and is expected to attract the same fee.

2.3 Both consultations are required by the Health Service for its own assessment; neither is material I have volunteered. Consistently with the approach taken elsewhere in the public sector, where the entity that requires medical information meets the reasonable costs of obtaining it, I ask:

(a) that the \$150.00 I have paid be reimbursed to me; and

(b) that the Health Service provide the name and address to which the practice should direct Friday's invoice, so that the cost of the Health Service's own request is not again met from my funds.

2.4 If the Health Service does not intend to meet these costs, could you please confirm that in writing, together with the basis for that position.

3. THE QUESTIONS I NEED

3.1 Your email states that "further clarification is required regarding your functional capacity and the medical restrictions identified", but does not identify what is unclear. Dr Ma cannot respond to a request in those terms.

3.2 Could you please provide, in writing, before 2:00 pm Friday:

(a) the specific questions the Health Service asks Dr Ma to answer, set out as questions;

(b) for each question, the duty or task to which it relates — Attachment 1 is provided for this purpose;

(c) what in the 3 July Employee Capability Checklist is said to be insufficient — which entry, and why it does not enable the assessment; and

(d) who will consider Dr Ma's response, and the timeframe within which the decision about my duties will then be made.

4. THE ORIGIN OF THE REQUIREMENT

4.1 Could you please confirm the date on which, and the basis on which, the Health Service required me to complete an Employee Capability Checklist. My understanding is that the requirement was raised in response to my request to temporarily reduce my hours. I am not aware of any incident, complaint or performance concern preceding it.

4.2 Could you also please confirm the outcome of that request to reduce my hours, and if no decision has been made, when one will be made and by whom. EB12 clause 10.3 provides that an employee may ask for a change in the way they work, including their ordinary hours of work; that the request may be refused only on reasonable grounds; and that the employer must give written notice of its decision, with reasons, within 21 days.

5. THE RESTRICTIONS AND THE DUTIES (Attachment 1)

5.1 Since your email refers to the inherent requirements of the role, I have compared the restrictions certified on 3 July with the responsibilities set out in the AO3 role description, which Dr Ma reviewed before completing the checklist. Attachment 1 lists those twelve responsibilities as they appear in that document.

5.2 First, complaint-handling. The ECC of 3 July 2026 records: no complaint-handling duties; complaints received at the Switchboard to be logged and redirected, not actioned or resolved by me. That restriction has already been addressed in correspondence. In her email of 7 July 2026, Ms Forrest stated that the established process is for patient concerns to be redirected to Consumer Liaison Services (CLS); that there is no expectation for Switchboard operators to manage or deal with those matters; and that where a consumer raises concerns about a Switchboard employee, those concerns are referred to the Manager, Switchboard or, in her absence, CLS. My reply of 9 July 2026 noted that this confirmation, together with existing rostering practice, appears to accommodate that ECC recommendation. I treat complaint-handling as closed on the Health Service's own account of the role. It does not need to be reopened with Dr Ma, and it is not a basis for refusing all duties in Attachment 1.

5.3 Second, the remaining restrictions — the limits on peak weekday day and afternoon shifts, predictable rostering, and a minimum ten-hour break between shifts — go to when I work, not to which duties I perform. They remove none of the responsibilities listed in Attachment 1.

5.4 I note the role description records the position as continuous shift work. I performed that role, on reduced hours and within my certified restrictions, for approximately twelve months, and the Health Service's correspondence of 7 July 2026 records that my restrictions were already being accommodated. If the

rostering restrictions are now said to be incompatible with the role, when they were not during that period, could you please identify what has changed.

5.5 Third, reduced ordinary hours are expressly contemplated by the industrial instruments. Clause 8.2 of the Hospital and Health Service General Employees (Queensland Health) Award – State 2015 provides for part-time employment on a regular number of ordinary hours, pro rata to full-time conditions, with a minimum of eight ordinary hours per fortnight. A reduction in hours does not of itself remove any duty from the role. Could you please identify why the reduced hours certified on 3 July are treated differently — that is, what it is about those hours, as distinct from the reasons for them, that prevents the duties of the position from being performed.

5.6 QH-POL-210 applies to all Queensland Health and Hospital and Health Service employees and directs adjustments — including temporary adjustment for rehabilitation and return to work, and for capacity restriction — to enabling an employee to safely perform the genuine occupational requirements of the role. It reflects the Anti-Discrimination Act 1991 (Qld), which contemplates reasonable terms and special conditions where job capacity is restricted by impairment (s 34) and permits departure only where unjustifiable hardship is established (ss 5, 35). QH-IMP-401-5 makes managers responsible for identifying suitable duties, guided by medical information, and its application is not limited to accepted workers' compensation claims. Each operates at the level of identified duties, which is what Attachment 1 is directed to.

6. THE CHARACTERISATION OF RISK (Attachment 2)

6.1 Your correspondence of 13, 15 and 29 July 2026 refers to whether my return can be "safely" facilitated, whether the matters identified by Dr Ma "present any psychosocial hazards or risks", and to the health and safety of workers "and others in the workplace". I do not suggest the Health Service should not satisfy itself as to safety.

6.2 However, in the twenty-seven days since 3 July, no incident, complaint or hazard has been identified to me. None was raised during the twelve months I performed the role on reduced hours. No finding of any kind has been made in relation to my performance of this role.

6.3 EB12 clause 7.2 and the Code of Practice proceed by identifying a hazard, assessing the risk, and considering controls, in consultation with workers and health and safety representatives — and they identify psychosocial hazards by reference to features of work and its management. Attachment 2 follows that structure.

6.4 Could you please confirm whether it is asserted that my performing my duties within the certified restrictions would present a risk to me or to any other person and, if so, whether the hazard is said to arise from the work and work system or from me, together with the material relied upon. Attachment 2 provides for both. If no such assertion is intended, could you please confirm that in writing.

6.5 For clarity, my cooperation with the medical request does not suspend the other matters: the restoration of my roster, the treatment of my pay and leave, and the workplace psychosocial assessment remain live in parallel.

7. CONFLICT OF INTEREST

7.1 I raise one further matter, without alleging wrongdoing.

7.2 The requirement that I complete an Employee Capability Checklist was raised by my line manager, Ms Taylor. Ms Taylor is the subject of a complaint which the Ethical Standards Unit determined to be a public interest disclosure, and her conduct is a matter in issue in proceedings before the Queensland Industrial Relations Commission in which she may give evidence.

7.3 The Code of Conduct for the Queensland Public Service provides at clause 1.2 that having a conflict of interest is not unusual and is not wrongdoing in itself, but that failing to disclose and manage it appropriately is likely to be wrongdoing, and that employees will disclose any personal interest that could be seen as influencing the performance of their duties.

7.4 Could you please confirm what involvement Ms Taylor has had in the decisions concerning my attendance, duties and pay since 2 July 2026; whether any conflict of interest has been disclosed and managed, and how; and that decisions concerning my attendance, duties and pay will be made by a person without involvement in those proceedings.

8. SCOPE AND MEETING

8.1 I will provide Dr Ma with the questions and the role description. My consent extends to information concerning my capacity, my restrictions, and their operational application to the duties of my position, and I ask that the questions be limited to what is necessary for that purpose.

8.2 If it would assist, I am willing to meet at short notice — including before Friday — to work through Attachment 1 duty by duty. I would ask that any meeting occur alongside, rather than in place of, a written response.

9. OTHER MATTERS

9.1 The matters raised in my emails of 28 July 2026 remain outstanding within the period identified in that correspondence. I do not repeat them here, and this email does not replace them.

10. WHAT I ASK

10.1 So that this can be resolved simply, I ask:

(a) that my roster be provided to me and that I be restored to my rostered shifts within the certified

restrictions;

(b) that the rostered shifts from 3 July 2026 be processed by ad hoc payment, and the leave debited for that period be re-credited;

(c) that the \$150.00 I have paid for the 3 July consultation be reimbursed; and

(d) that the invoice for Friday's consultation be directed to the Health Service at an address you provide.

10.2 I have facilitated every step asked of me — I obtained the checklist the Health Service required and paid for it myself, I have elected to take its request to Dr Ma myself, I have booked the appointment, and I have prepared the attached forms so that the Health Service's questions can be framed quickly. I am conscious that time and cost are accumulating for both of us, and I would prefer to resolve this now, collaboratively, rather than through further process.

11. IN SUMMARY

11.1 The operative decision in this matter is the direction that I not attend work. Everything since follows from it — the treatment of my rostered shifts, the refusal of duties in any capacity, and the requirement for further medical information.

11.2 A direction to an employee not to attend work is an exercise of authority. I have asked on more than one occasion for the source of that authority to be identified, and it has not been. Nor have I been told the process under which I am being managed, or the person who made the decision.

11.3 I raise this not as a complaint, but because without it I am unable to understand or respond to the basis on which I am being kept from my substantive position.

11.4 I ask that any further decision concerning my attendance, duties or pay identify the power relied upon, the person making it, and the reasons for it.

I remain certified fit, ready and available to work within the 3 July 2026 Employee Capability Checklist.

Kind regards,

Cory Lea Shepherd

Administration Officer (AO3), Switchboard Services

0417 400 227

MSH-INJ-5795

From: LBH Injury Management

<LBH_InjuryManagement@health.qld.gov.au>

Sent: Wednesday, July 29, 2026 4:10:13 pm

To: Cory Shepherd <coryshepherd1@hotmail.com>;

Lyndelle Forrest <Lyndelle.Forrest@health.qld.gov.au>

Cc: Heath Moran <heath.moran@together.org.au>;

James Zappia <james.zappia@art.com.au>

Subject: RE: Response to 2-15 July 2026

correspondence - incorrect application of EB12, Award
and QH policies - ECC 3 July 2026 (MSH-INJ-5795)

Good afternoon Cory,

Thank you for your email dated 28 July 2026. We
acknowledge receipt of your correspondence and the
attached documentation.

As previously advised, the Injury Management Team
requires further information from your treating medical
practitioner before we are able to determine whether
Metro South Health can safely facilitate your return to
duties.

The Employee Capabilities Checklist (ECC) provided has
been reviewed; however, further clarification is required
regarding your functional capacity and the medical
restrictions identified to enable an informed assessment of
whether suitable duties can be accommodated safely and
operationally.

This request is consistent with Metro South Health's obligations under the *Work Health and Safety Act 2011 (Qld)* to ensure, so far as is reasonably practicable, the health and safety of workers and others in the workplace. While your treating medical practitioner's advice is an important consideration, Queensland Health has an independent obligation to assess the medical information provided alongside the inherent requirements of the role and determine whether duties can be safely facilitated.

To assist in progressing this matter, please confirm whether you would prefer to take our request for further medical information to your treating general practitioner yourself, or alternatively, complete and return the attached Employee Authority Form (EAF) authorising the Injury Management Team to write directly to your treating practitioner. Should you choose the latter option, you will be copied into our correspondence with your treating practitioner for your records.

Once the requested medical clarification has been received, we will review the information and advise you of the outcome regarding your capacity to return to duties.

Kindest Regards

Michelle Harrison

Injury

Management Consultant | LBHS HR Team 1

Human Resources

Logan and Beaudesert Hospital and Health Service

4/3-5 University Drive, Meadowbrook Qld 4131

[Metro South Health](#) | 

lbh_injurymanagement@health.qld.gov.au | [Chat on Teams](#)

p. [07 3542 2744](tel:0735422744)

a. Mon-Fri 8am - 4pm

Metro South Health recognises and pays respect to the traditional custodians of the land and waters – the Yugambeh, Quandamooka, Jaggera, Ugarapul, Turrbal, and Mununjali peoples – and to Elders, past, present and emerging.

Please be advised that we are currently transitioning into Solv, a new Injury Management Case System. Our documents and communications may change appearance slightly and you may even receive communications from us via the @Solv.com.au email pathway, as this is a new email pathway, please ensure you are regularly checking your Junk folder in your email accounts and save the @solv to your frequent emails so you do not miss any communications. Please note these emails are legitimate and are from us here at lbh InjuryManagement@health.qld.gov.au. If you have any concerns, please reach out to us via the above contact details.

-----Original Message-----

From: Cory Shepherd <coryshepherd1@hotmail.com>

Sent: Tuesday, 28 July 2026 5:40 PM

To: LBH Injury Management

<LBH_InjuryManagement@health.qld.gov.au>; Lyndelle Forrest <Lyndelle.Forrest@health.qld.gov.au>

Cc: Heath Moran <heath.moran@together.org.au>; James Zappia <james.zappia@art.com.au>

Subject: Response to 2-15 July 2026 correspondence - incorrect application of EB12, Award and QH policies - ECC 3 July 2026 (MSH-INJ-5795)

This email originated from outside Queensland Health. DO NOT click on any links or open attachments unless you recognise the sender and know the content is safe.

Dear Michelle / Injury Management / HR,

I refer to your correspondence and Switchboard management correspondence between 2 and 15 July 2026, including:

- Chloe Taylor, 2 July 2026 — unable to "facilitate" return until a completed ECC; shifts processed as sick leave / sick leave no pay;
- my provision of the completed Employee Capability Checklist (ECC) of 3 July 2026 (Dr Day Hong Ma) while I was rostered and working;
- the same-day direction

that I not attend "pending review"; • Injury Management correspondence of 7–13 July 2026 that restricted duties could not be facilitated without further medical clarification from Dr Ma; • your email of 13 July 2026 to ART that Metro South Health was unable to facilitate my return under the ECC (MSH-INJ-5795 / CLM-317073); • your email of 15 July 2026 (13:10) — pay sits with QSuper because of a "current" IP claim; alternative duties refused "in any capacity"; WCRA rehabilitation limited to accepted claims; return at employer discretion; further medical about psychosocial risk to my return; and • leave coding as "income protection no pay" from 13 July 2026.

I remain ready, willing and able to work within the 3 July 2026 ECC. I do not abandon my employment. I am taking industrial advice through Together Union.

I am an HHS administrative employee. The industrial framework that applies is attached. Please respond by reference to each attachment.

ATTACHMENTS

ATT02 — Queensland Public Health Sector Certified Agreement (No. 12) 2025 (EB12) — current agreement
ATT03 — Hospital and Health Service General Employees (Queensland Health) Award – State 2015
ATT05 — Managing the risk of psychosocial hazards at work Code of Practice 2022 (Qld)
ATT09 — QH-IMP-401-5 Workplace Rehabilitation Standard
ATT10 — QH-POL-210 Reasonable Adjustment (HR Policy G3)
ATT12 — Directive 12/24: Special Leave (applies under EB12 cl 9.12)
ATT13 — QH-POL-231 Special leave (HR Policy C7)
ATT14 — Administration Officer, Switchboard Services (AO3) Role Description

FACTS

The 3 July 2026 ECC certifies me fit for modified duties and reduced hours from 3 July 2026 as a continuation of the existing arrangement. The doctor viewed the AO3 Switchboard position description (ATT14) and recorded that usual switchboard duties remain suitable with stated

modifications (including no complaint-handling; limits on peak weekday day/afternoon shifts; predictable rostering; minimum 10-hour break). I was performing work consistent with those limits when I provided the ECC. The same-day direction not to attend disrupted that work. The 28 August 2026 date on the ECC is a review of restrictions — not the start of capacity.

WHAT HAS BEEN WRONGLY APPLIED

1. "Current QSuper Income Protection claim" → ordinary pay sits with ART (your 15 July email)

Wrong application.

- An IP file is not ordinary wages for a permanent employee who is certified fit, ready and available, and directed by the employer not to attend.
- On the material available to me notified by employee (myself) May 2025 and February 2026 and now 3rd July 2026 certification of that partial capacity, ART put benefit payments are not applied from 1 June 2026. Treating the Fund as presently replacing ordinary pay is incorrect on that record. Partial capacity and partial payment does not dislodge this framing.
- IP administration is separate (Mr Zappia). It does not displace ATT02 EB12 cl 6.1 (employment security) or ATT02 EB12 cl 7.1.5(g)–(h) (injury management; management of ill or injured employees).
- Coding "IP no pay" while held out after an ECC is not a correct application of ATT02 or of ATT12/ATT13 (special leave).

2. WCRA rehabilitation only if accepted claim; return purely at "employer discretion" (your 15 July email)

Wrong application.

- Even if WCRA statutory rehab is limited to accepted claims, that does not exhaust your duties under ATT02 (EB12), ATT03 (HHS Award), ATT10 (QH-POL-210) and ATT09 (QH-IMP-401-5).
- ATT10 applies to all Queensland Health / HHS employees and includes temporary reasonable adjustment for rehabilitation/RTW and for capacity restriction — not only accepted workers' compensation claims.

- ATT02 EB12 cl 7.1.5 requires the parties to address injury management, management of ill or injured employees, fatigue, psychosocial issues and the Code of Practice 2022 (ATT05), and security for administrative staff in frontline positions.

- "Discretion" does not correctly authorise indefinite exclusion, refusal of interim duties "in any capacity," and non-payment without applying ATT09, ATT10 and ATT02 Parts 6 and 7.

3. Further medical framing me as the psychosocial risk before return "in any capacity" (your 13–15 July emails)

Wrong application of ATT05 and ATT02.

- ATT05 and ATT02 EB12 cl 7.2 require assessment of psychosocial hazards in the workplace and work system (including workplace risk assessments as requested through an HCF by employer or union).

- The 3 July ECC already identifies workplace stressors (complaint-handling; blame for others' failures; rostering/fatigue; unresolved line-management matters).

Those are workplace issues under ATT05 and ATT02 cl 7.1.5(j) and 7.2.

- Framing psychosocial risk as a personal barrier inside Injury Management / QSuper misapplies ATT02 cl 7.2 and cl 7.1.11 (feedback when WHS matters are raised).

- Workplace psychosocial issues I have raised over more than two years are not properly dealt with by indefinite IM hold-out. That process is not the correct forum under ATT02 cl 7.2.

- I will cooperate with reasonable medical requests that are specific and linked to identified job tasks under ATT14. Open-ended further medical is not a correct precondition to all duties where ATT14 and the ECC already define capacity I have been performing.

4. "Unable to facilitate" / no alternative duties (Chloe 2 July; your 13–15 July emails)

Wrong application of ATT09, ATT10 and ATT02.

- Chloe's 2 July email made facilitation conditional on a completed ECC. I provided that ECC on 3 July while working. Facilitation was then refused and I was directed

off — the opposite of facilitating return on the medical basis required.

- The ECC states fit from 3 July as continuation of the existing arrangement and that usual switchboard duties remain suitable with modifications against ATT14. Refusing all duties "in any capacity" does not correctly apply ATT10, ATT09 or ATT02 cl 7.1.5(f)–(h).
- Correct application requires identifying any specific duty and restriction said to be incompatible, what modification was considered, and what was rejected — not a blanket refusal.

5. Leave coding as sick leave / sick leave no pay / "IP no pay" while certified fit and directed not to attend

Wrong application of ATT02, ATT12 and ATT13.

- From 3 July 2026 the relevant absence is employer-directed while I am certified fit and available. That is not ordinary sick leave for that period.
- "IP no pay" is not the correct industrial answer under ATT02 when the employer has directed me off after an ECC.
- ATT02 EB12 cl 9.12 provides that Directive 12/24: Special Leave (ATT12) applies to employees covered by the Agreement. ATT13 QH-POL-231 (C7) also applies.
- Special leave under ATT12 and ATT13 must be applied for employer-directed exclusion while I am ready, willing and able within the ECC. Where the only barrier is refusal to facilitate certified duties, special leave on full pay (or ordinary pay as if at work) must be applied. Please correct coding and identify the delegate and instrument for each day from 3 July.

6. Treating 28 August 2026 as if capacity only starts then

Wrong application of the ECC itself.

- 28 August 2026 is the next review of restrictions. 3 July 2026 is the date fit to commence the certified arrangement. Capacity is certified now.

WHAT YOU MUST APPLY

ATT02 EB12 cl 6.1 — employment security

ATT02 EB12 cl 7.1.5 — injury management; ill or injured employees; fatigue; psychosocial issues + ATT05; admin frontline security

ATT02 EB12 cl 7.2 — psychosocial risk assessments of workplaces

ATT02 EB12 cl 7.1.11 — feedback on WHS matters

ATT02 EB12 cl 9.12 + ATT12 + ATT13 — Special Leave

ATT03 — hours, breaks and rostering with the Agreement

ATT09 — workplace rehabilitation / RTW pathway

ATT10 — reasonable adjustment (including temporary adjustment and capacity restriction)

ATT05 — workplace psychosocial risk

ATT14 — task match to the AO3 role the doctor viewed

REQUESTS — PLEASE RESPOND IN WRITING WITHIN 5 BUSINESS DAYS

1. Facilitate my return under the 3 July 2026 ECC (or a written suitable duties plan implementing it: start date, hours, location, tasks, review date) by applying ATT09, ATT10 and ATT02 cl 7.1.5.
2. Complete a task-by-task match of ATT14 to the 3 July ECC. If you maintain you cannot facilitate, identify each restriction and duty and what was considered and rejected.
3. Be specific about any further medical still said to be required: exact questions; which ATT14 job task each relates to; what on the existing ECC is said to be insufficient.
4. Apply special leave under ATT02 cl 9.12, ATT12 and ATT13 for employer-directed days from 3 July while certified fit and available; correct "IP no pay" / sick no pay coding; identify delegate and instrument.
5. Confirm ordinary pay treatment for periods I was ready and available within the ECC but directed not to attend.
6. Confirm compliance with the direction not to attend is not abandonment or unauthorised absence (ATT02 cl 6.1).
7. Arrange a workplace-directed psychosocial risk assessment under ATT02 cl 7.2 and ATT05.
8. Identify the decision-maker for facilitation, suitable duties, special leave and pay, and the instrument for the 3 July direction not to attend.

I will continue to provide reasonable medical information directed to placement and safety. Please make further requests specific and linked to ATT14 tasks.

My QIRC appeal WC/2024/227 is separate. ART/QSuper is being addressed with Mr Zappia.

I look forward to your written response and a meeting date to facilitate return under the attached agreements and policies.

Yours sincerely,
Cory Lea Shepherd
0417 400 227
MSH-INJ-5795

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maintaining and ensuring appropriate use of its
computer network.

ACTION REQUIRED | Request for Medical Information: Cory Shepherd (MSH-INJ-5795) [CLM-317073]

From: LBH Injury Management

LBH_InjuryManagement@health.qld.gov.au

To: Cory Shepherd coryshepherd1@hotmail.com

Cc: notes@solv.com.au notes@solv.com.au, LBHS Human Resources Team 1

LBH.HRTeam1@health.qld.gov.au

Sent: Friday 31 July at 11:43

Good morning Cory,

Thank you for agreeing to coordinate with your doctor to obtain the additional information that will be so important in helping us properly assess your current medical situation and workplace requirements. I have attached the following documents to assist with this:

1. A letter to you explaining the process
2. A letter to your GP, including some specific questions
3. Attachment 1 to the letter to your GP
4. Attachment 2 to the letter to your GP

Please let me if you have any questions.

Kindest Regards

Michelle Harrison

Injury

Management Consultant | LBHS HR Team 1

Human Resources

Logan and Beaudesert Hospital and Health Service

4/3-5 University Drive, Meadowbrook Qld 4131

[Metro South Health](#) |   

lbh_injurymanagement@health.qld.gov.au | [Chat on Teams](#)

p. [07 3542 2744](tel:0735422744)

a. Mon-Fri 8am - 4pm

Metro South Health recognises and pays respect to the traditional custodians of the land and waters – the Yugambeh, Quandamooka, Jaggera, Ugarapul, Turrbal, and Mununjali peoples – and to Elders, past, present and emerging.



Please be advised that we are currently transitioning into Solv, a new Injury Management Case System. Our documents and communications may change appearance slightly and you may even receive communications from us via the @Solv.com.au email pathway, as this is a new email pathway, please ensure you are regularly checking your Junk folder in your email accounts and save the @solv to your frequent emails so you do not miss any communications. Please note these emails are legitimate and are from us here at lbh.InjuryManagement@health.qld.gov.au. If you have any concerns, please reach out to us via the above contact details.

Disclaimer: This email and any attachments may contain legally privileged or confidential information and may be protected by copyright. You must not use or disclose them other than for the purposes for which they were supplied. The privilege or confidentiality attached to this message and attachments is not waived by reason of mistaken delivery to you. If you are not the intended recipient, you must not use, disclose, retain, forward or reproduce this message or any attachments. If you receive this message in error, please notify the sender by return email or telephone and destroy and delete all copies.

Unless stated otherwise, this email represents only the views of the sender and not the views of the Queensland Government.

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Enquiries to: Michelle Harrison
Injury Management Consultant
Human Resources
Telephone: 07 3542 2744



Metro South Health

Logan and Beaudesert Health Service

Mr Cory Shepherd
12304/3113 Surfers Paradise Blvd
SURFERS PARADISE QLD 4217

Dear Mr Shepherd

I refer to your employment as an Administration Officer (**AO**), Switchboard Operations, Corporate Services, Logan Hospital (**LH**), Metro South Health (**MSH**) on a Permanent Full-time basis 76 hours per fortnight and your recently completed Employee Capabilities Checklist (**ECC**) (**Attachment 1**) completed by Dr Day Hong Ma on the **3 July 2026** which was provided to us to facilitate a Return to Work Plan.

I am seeking further clarification from Dr Day Hong Ma, to understand the nature of some of the restrictions stated in the ECC and how this relates to your current medical condition/s and the effect they have on your ability to perform the inherent requirements of your role Administration Officer (**AO**), Switchboard Operations, Corporate Services (**CS**) Logan Hospital.

I require further information from your general practitioner to confirm:

- The impact of your medical condition and/or disorder on your ability to perform your role;
- Any risks to your health and safety in undertaking the role;
- Any other matters that are considered relevant to your medical condition and/or disorder.

You may choose to:

1. Obtain this information from your medical practitioner on the attached '*Request for Medical Information*'; or
2. Provide Metro South Health permission to obtain this information directly from your medical practitioner. An authority form is attached should you prefer this option. Please let me know if you prefer this option.

We are requesting this information in accordance with the provisions of the Human Resources (**HR**) Policy G3: Reasonable Adjustment, and sections 17 and 19 of the *Work Health and Safety Act 2011*. This information will primarily be used for the purposes of better understanding your condition and to support the implementation for appropriate controls within the working environment or any other reasonable adjustments.

I ask that you please return the applicable document within **seven (7) calendar days** of receipt of this letter. This will enable me to give proper consideration to your requested restrictions to duties and determine what accommodations we can provide. If you do not return the document within this timeframe or seek a reasonable extension, I will make a decision regarding your ability to perform your role based on the available information.

Better together

MSH will meet your reasonable costs of preparing this report and we appreciate your assistance with this request. Please send invoices to **Michelle Harrison**, Injury Management Consultant, Human Resources, Logan and Beaudesert Health Services via email at lbh.InjuryManagement@health.qld.gov.au.

Employee support

I acknowledge this may be a difficult time for you. I would like to take this opportunity to also advise you of the Employee Assistance Program. This is a confidential counselling service which is free of charge to all employees of MSH for up to six sessions per calendar year. Access to this service is by self-referral.

Should you wish to access Employee Assistance, please contact TELUS Health on 1800 604 640, or for Aboriginal and Torres Strait Islander support, please contact TELUS Health on 1800 816 152.

More information about the support TELUS Health can provide can be found out their website <https://app.one.telushealth.com/> or by downloading the free TELUS Health One app. You can log on to both using the following credentials:

Username: mshhs
Password: mshhs

Questions

If you have any questions in relation this correspondence or the process, please contact **Michelle Harrison** Injury Management Consultant, Human Resources, Logan and Beaudesert Health Service via email at lbh.InjuryManagement@health.qld.gov.au or Ph: (07) 3542 2744.

Yours sincerely,



Scott Hughes
Director, Corporate Services
Logan Beaudesert Health Service
Metro South Health
31/07/2026

Enquiries to: Michelle Harrison
Injury Management Consultant
Human Resources
Telephone: 07 3542 2744



Metro South Health

Logan and Beaudesert Health Service

Dr Day Hong Ma
My Doctors Clinic
16/3221 Surfers Paradise Boulevard
SURFERS PARADISE QLD 4217

Dear Dr Day Hong Ma,

Name of Employee: Mr Cory Shepherd
Date of Birth: 11 January 1991

I refer to your patient, Mr Cory Shepherd and his employment as an Administration Officer (**AO**), Switchboard Operations, Corporate Services (**CS**), Logan Hospital (**LH**), Metro South Health (**the Health Service**) on a Permanent Full-time bases (76 hours per fortnight).

Background

Logan and Beaudesert Health Service (**LBHS**) is in receipt of an Employee Capabilities Checklist (**ECC**) (**Attachment 1**) completed by you on the 3 July 2026.

In accordance with Queensland Health Human Resource (**HR**) Policy G3: *Reasonable Adjustment*, and sections 17 and 19 of the *Work Health and Safety Act 2011*, the Health Service is seeking further clarification of the medical information you have provided regarding Mr Shepherd. The purposes of this request is to better understand Mr Shepherd's condition and current capacity for work, including any restrictions or limitations that may need to be considered.

This information will assist the Health Service to identify and implement any appropriate workplace controls or reasonable adjustments to support a safe and sustainable return to his substantive position.

Questions

I appreciate your assistance with this request. Should you require any additional information or have a query about this request please contact Michelle Harrison, Injury Management Consultant, Human Resources, Logan and Beaudesert Health Service via email at lbh_injurymanagement@health.qld.gov.au or phone: (07) 3542 2744.

Yours sincerely

Scott Hughes
Director, Corporate Services
Logan Beaudesert Health Service
Metro South Health
31/07/2026

Attachment 1 – Employee Capabilities Checklist
Attachment 2 – Role Description

Better together

Information requested from Dr Day Hong Ma:

1. The attached Employee Capabilities Checklist states that Mr Shepherd has a diagnosis of Major Depressive Disorder (**MDD**) with anxious distress, with symptoms exacerbated on exposure to identified workplace stressors, including complaint handling, being held accountable and blamed for the failures of others, and unpredictable rostering.

Could you please clarify the below question in relation to this statement:

- a. When was Mr Shepherd first diagnosed with MDD?

- b. What is the clinical basis for identifying these specific workplace stressors as contributing to the exacerbation of Mr Shepherd's condition.

- c. Whether these workplace stressors are based solely on Mr Shepherd's self-report, your own clinical assessment, or other medical information/reports available to you.

- d. Whether exposure to these workplace stressors presents a foreseeable risk to Mr Shepherd's health or safety if he were to return to work, and if so, what specific workplace controls or adjustments you consider medically necessary to manage that risk.

2. You have described Mr Shepherd's condition as a "*work-related psychological injury*", with existing stressors that could impact on a return to substantive role, specifically:
- "(2) *being held accountable and blamed for failures of others*".
 - "(4) *unresolved workplace matters involving line management*".

As the Health Service is not aware of any concerns being raised for appropriate management, clarification on these statements is being sought to better understand the nature of his injury/illness, including:

- If Mr Shepherd is able to follow a reasonable and lawful direction is issued by his supervisor.

- Participate in discussions in relation to his workplace performance and/or conduct.

Please provide any further context and/or considerations relating to Mr Shepherd's *work-related psychological injury*?

3. Having regard to your reference to unresolved workplace matters involving line management, please confirm whether Mr Shepherd is medically fit to return to their substantive role under the existing reporting arrangements, including working with and reporting to their current line manager.

4. If Mr Shepherd is not able to return to his substantive role under the existing reporting arrangements, please specify any restrictions or adjustments you recommend, the clinical basis for those recommendations in functional terms, and the anticipated duration and review date.

5. The ECC recommends that Mr Shepherd not undertake complaint-handling duties, can you please clarify what specific activities are encompassed within “complaint handling” does this restriction relate to receiving complaints, documenting complaints, redirecting complaints, resolving complaints, or all complaint-related interactions.

For consideration in responding to this question, established process for client complaints received by Switchboard employees is for immediate escalation and management by the Health Service Client Liaison Officer and/or Manager, Switchboard Services.

6. In your medical opinion is Mr Shepherd able to fulfill the full inherent requirements of his Role Administration Officer – Switchboard Services (**Attachment 2**) *without* restrictions or modifications to duties?

7. Your report states Mr Shepherds “working memory is affected under stress”. Could you please clarify what this means in functional terms in the workplace, including:
- a. how this may affect their capacity to perform their substantive duties under normal workplace demands;
 - b. the types of circumstances likely to cause this difficulty;

8. Are there specific tasks, situations, or environments (e.g., high-pressure settings, shift work, patient acuity) that may exacerbate Mr Shepherd’s condition or symptoms?

9. If we are not able to accommodate the restrictions you have recommended, is Mr Shepherd able to safely return to the workplace?

Please include your practice details/stamp here

Signature

Date



Administration Officer, Switchboard Services (RD)

Job Ad Reference:			
Status:	Administration Officer, Switchboard Services		
Unit/Department:	Switchboard, Corporate Services		
Location:	Logan Hospital, Metro South Hospital and Health Service		
Contact:	Chloe Taylor (07) 2891 9190	Salary range:	
Classification:	A03	Closing date:	
Online applications:			
Email applications:	Chloe.taylor3@health.qld.gov.au		

****Applications from third parties will not be accepted****

Purpose of the role

To provide a highly effective and efficient communication service to the public and staff of the Logan Hospital within Metro South Hospital and Health Service.

The occupant of this position is required to work continuous shift work over the full 24-hour period, 7 days a week

Your key responsibilities

- This position reports to the Switchboard Manager, Corporate Services, Logan Hospital.
- Attend courteously to all telephone enquiries and clients' requests, displaying tact, diplomacy and interpersonal skills of a high standard when dealing with the public and members of staff. Maintain call queues to minimum at all times.
- Collate information and maintain Omnivista database and SharePoint to ensure information held within Switchboard Services is accurate and appropriate.
- Provide a service to support hospital staff in the allocation, coordination, and fault repair of all pager units for the hospital. This includes allocating pagers to hospital staff and submitting paging units for repair, maintaining database and registers with accurate and current information.
- Participate in the Emergency Response process by receiving emergency response notifications and distributing them to the appropriate response groups, dependent on the category of emergency, as per emergency code procedures, strictly adhering to protocols and timeframes.
- Ability to communicate at all levels and effectively establish and foster networks throughout the hospital to maintain accurate and current records for effective switchboard operations.
- Maintain discretion and exercise judgement where necessary to resolve problems within the scope of your role; in situations where precedence have not been set and procedures not defined.
- Interact in a sensitive, timely, effective, and appropriate manner with members of the general public and service providers, maintaining strict confidentiality at all times.
- The ability to multitask and operate under pressure, particularly where high volume call traffic is concerned.
- Contribute to a cohesive team environment by sharing information, supporting and assisting colleagues in a proactive manner.
- Ability to work effectively as an individual with limited supervision to meet deadlines and establish work priorities, and work as a member of a multi-disciplinary team in a changing environment.
- Ensure patients and other staff are treated fairly and with mutual respect and understanding, regardless of cultural, religious, ethnic and linguistic backgrounds, gender, identity, ages or abilities.

- Follow defined service quality standards, occupational health and safety policies and procedures relating to the work being undertaken to ensure high quality, safe services and workplaces.

Mandatory qualifications, professional registration and other requirements

- The position is a continuous shift working role. You must be able to work a roster which covers multiple shifts over a 24/7 period.
- Intermediate / advanced computer skills.
- Recent experience in the operation of a switchboard is desirable but not essential.
- **Vaccine Preventable Disease (VPD):** It is a condition of employment for this role for the employee to be, and remain, vaccinated against the following vaccine preventable diseases during their employment ([Health Employment Directive No. 01/16](#)):
 - measles, mumps, rubella (MMR)
 - varicella (chicken pox)
 - pertussis (whooping cough)
 - hepatitis B
- It is a condition of employment for this role for the employee to be, and remain, vaccinated against COVID-19 ([Health Employment Directive No. 12/21](#) and [Queensland Health Human Resources Policy B70](#)).

Are you the right person for the job?

How we do things is as important as what we do, therefore within the context of the responsibilities described under 'Your key responsibilities,' the ideal applicant will be someone who can demonstrate the following:

Communicate Effectively	▪ Strong communication and negotiation skills to understand customers' needs. Demonstrate respect, courtesy and fairness when interacting with patients, clients and colleagues.
Team Focus	▪ Value diversity of thought and include everyone within the team. Actively support a culture of diversity, trust, support and inclusion.
Technical Skills	▪ Intermediate / advanced computer skills.
Change & Quality Focus	▪ Be open to new ideas and approaches and be adaptable in the face of change. Support and participate in change initiatives. Follow procedures and processes understanding implications within a health care setting.
Work Ethics	▪ Be honest, ethical and professional. Work within boundaries of role.

How to apply

Please provide the following information to the panel to assess your suitability:

- A short written response (maximum 1-2 pages, dot points acceptable) on how your experience, abilities, knowledge and personal qualities would enable you to achieve the key responsibilities and meet the skill requirements of the role.
- Your current CV or resume, including two referees. You must seek approval prior to nominating a person as a referee.
- Late applications cannot be submitted online. For a late application to be considered, please arrange approval and submission via the contact person.
- Applications will remain current for 12 months or for the duration of the vacancy. Future vacancies of a temporary, full time and part time nature may also be filled through this recruitment process.
- Only those persons eligible to work in Australia may be employed by Queensland Health. Prospective employees are required to provide proof of identity and documentary evidence of their [right to work in Australia](#).

We are Metro South Health

We are the major public healthcare provider for Brisbane's south side, Logan, Redlands and the Scenic Rim. We operate five major hospitals and a range of community, specialty and state-wide healthcare services. We are renowned for teaching and research excellence.

We are truly dedicated people who care about our community. We come to work to make a difference. We save lives, changes lives and make the world a better place. We overcome the many challenges that come our way by going above and beyond and achieve the best possible outcomes by working together. We are bold, innovative, collaborative, inspiring.

We are part of and proud of the community we serve. We are *better together*.

Our values

Our values of Integrity, Compassion, Accountability, Respect, Engagement and Excellence, shape our culture. Our values guide our day-to-day decision making and are fundamental to what we care about as a health service, how we behave, how we interact with each other and provide care to the many patients who come through our doors every day.



INTEGRITY

We are authentic, truthful, and transparent, and strive for equity for all.



COMPASSION

We care for one another and ourselves with empathy, kindness and support.



ACCOUNTABILITY

We are accountable for our decisions, actions and behaviour.



RESPECT

We foster an environment of safety, civility, and inclusion.



ENGAGEMENT

We are one team working together to achieve our best outcomes.



EXCELLENCE

We empower each other and inspire innovation to deliver excellence.

Diversity and inclusion

We are the most culturally diverse area of Queensland with 28.5% of the community born overseas and 16% from non-English speaking countries. Our employees proudly reflect the community in which they work. At Metro South Health 20.5% of staff identify as coming from non-English speaking backgrounds.

Having a workforce that reflects and understands the needs and expectations of our community is important to delivering safe, kinder, and more inclusive care.

We recognise our strength comes from the diversity of our people and so we encourage people of all genders, ethnicities, ages, abilities, languages, sexual orientation and family responsibilities to apply.

We are committed to creating a diverse and inclusive workplace for our people and our community.

Flexible working arrangements

Metro South Health supports and encourages work-life balance for the mutual benefit of Metro South Health and its employees. Work-life balance is about a person's ability to manage their paid work commitments with their career goals, personal, community and cultural responsibilities. Through flexible working arrangements such as leave, flex time accrual or a reduction in working hours, there is an opportunity to match the individual's requirements with those of the workplace – delivering quality health services.

Additional information

For further information about this opportunity and the benefits and conditions of working at Metro South Health please refer to the **Information for Applicants** document attached with this Smart Jobs advertised vacancy.

So you have finished your Role Description, what's next? Check out the Recruitment Process Flow for next steps, [here](#)



Worker Authorisation

Collection notice:

Metro South Health is collecting your information in order to support rehabilitation and return to work processes. Personal information collected is handled in accordance with the Information Privacy Act 2009. With your consent, the health agency's Rehabilitation and Return to Work Coordinator or representative will contact your treating medical practitioners and health care providers, as well as authorised WorkCover Queensland representatives and authorised QSuper or other income protection insurer representatives where relevant, in order to obtain and communicate information about your current injury/illness, to assist the planning and management of your workplace rehabilitation and return to work.

All personal information will be securely stored and only accessible by appropriately authorised officers of Metro South Health involved in your workplace rehabilitation, such as your workplace supervisor, Rehabilitation and Return to Work Coordinator, Human Resources and other authorised representatives. Your personal information may be disclosed to WorkCover Queensland, QSuper or other income protection insurers for the purposes of your workplace rehabilitation program. Your personal information will not be disclosed to other third parties without consent, unless the disclosure is authorised or required by or under law. If you do not provide all the personal information requested in the form, the health agency may not be able to fully support your workplace rehabilitation for your current injury/illness, due to the requirement for this information to address any work health and safety concerns.

For any questions regarding this collection notice, please contact your local health and safety or rehabilitation contact. For information about how Metro South Health protects your personal information, how to access or correct your own personal information, or how to make a complaint about a breach of the privacy principles and learn how we deal with such a complaint, please refer to:

Metro South Health's [privacy policy](#).

Authority

Please tick the boxes below as appropriate to indicate whether you consent to the listed activity in relation to information (which may include sensitive or confidential health information) which is relevant to your current illness/injury ("your information") for the purpose of your workplace rehabilitation.

Activity	Yes	No	N/A
Do you consent to the health agency's appointed Rehabilitation and Return to Work Coordinator / employer representative disclosing and obtaining your information from WorkCover Queensland?	☐	☐	☐
* WorkCover Queensland collects your personal information in accordance with the <i>Workers' Compensation and Rehabilitation Act 2003</i> in order to assess your entitlement to compensation and manage your rehabilitation and return to work. Some of this information may be given to your employer for the purpose of payments, treatment, rehabilitation and return to work. By agreeing to the terms of WorkCover's privacy notice and worker's statement, you have authorised disclosure to WorkCover any information about your medical history relevant to this claim.			
Do you consent to the health agency's appointed Rehabilitation and Return to Work Coordinator / employer representative disclosing and obtaining your information from QSuper or other income protection insurer?	☐	☐	☐
Do you consent to the Rehabilitation and Return to Work Coordinator disclosing your information, including by email, to the treating medical and health care practitioners, external rehabilitation providers and any other insurers involved in your workplace rehabilitation?	☐	☐	☐
Do you consent to the medical and health care practitioners and any other insurers listed in the schedule below disclosing your information to the health agency's appointed Rehabilitation and Return to Work Coordinator/s?	☐	☐	☐

Medical/Health Practitioner	Address	Telephone

This authority shall remain current until either of the following actions result:	• I advise the health agency in writing of my decision to revoke this authority; or • my rehabilitation and return to work program has finalised.
--	--

Signature

Name (Please print):

Date

(A photocopy of this consent form maybe accepted with the same authority as the original)



**ACTION REQUIRED | Request for
Reimbursement: Cory Shepherd (MSH-
INJ-5795) [CLM-317073]**

From: LBH Injury Management

LBH_InjuryManagement@health.qld.gov.au

To: Cory Shepherd coryshepherd1@hotmail.com

Cc: notes@solv.com.au notes@solv.com.au

Sent: Friday 31 July at 15:15

Good afternoon Cory,

In relation to your request for reimbursement, we are more than happy to assist with this process.

Could you please ask the Medical Centre to email us directly at

LBH_InjuryManagement@health.qld.gov.au with a copy of the invoice and remittance advice confirming that payment has already been made?

This will allow us to commence the reimbursement process.

I note that you have already provided a copy of the invoice; however, we also require confirmation of payment directly from the Medical Centre via their practice email address before we can proceed.

If you have any questions, please let me know.

Kindest Regards

Michelle Harrison

Injury

Management Consultant | LBHS HR Team 1

Human Resources

Logan and Beaudesert Hospital and Health Service

4/3-5 University Drive, Meadowbrook Qld 4131

[Metro South Health](#) |   

lbh_injurymanagement@health.qld.gov.au | [Chat on Teams](#)

e. [qld.gov.au](mailto:lbh_injurymanagement@health.qld.gov.au)

p. [07 3542 2744](tel:0735422744)

a. Mon-Fri 8am - 4pm

Metro South Health recognises and pays respect to the traditional custodians of the land and waters – the Yugambeh, Quandamooka, Jaggera, Ugarapul, Turrbal, and Mununjali peoples – and to Elders, past, present and emerging.



Please be advised that we are currently transitioning into Solv, a new Injury Management Case System. Our documents and communications may change appearance slightly and you may even receive communications from us via the @Solv.com.au email pathway, as this is a new email pathway, please ensure you are regularly checking your Junk folder in your email accounts and save the @solv to your frequent emails so you do not miss any communications. Please note these emails are legitimate and are from us here at lbh.InjuryManagement@health.qld.gov.au. If you have any concerns, please reach out to us via the above contact details.

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Response — Request for Medical Information, MSH-INJ-5795 [CLM-317073]

From: Cory Shepherd

coryshepherd1@hotmail.com

To: LBH Injury Management

lbh_injurymanagement@health.qld.gov.au

Cc: LBH_HR lbh_hr@health.qld.gov.au, Heath

Moran heath.moran@together.org.au

Sent: Monday 3 August at 11:38

Dear Human Resources team,

I refer to the email from Injury Management of Friday 31 July 2026 at 11:43am and the two letters of the same date signed by Mr Scott Hughes, Director, Corporate Services. I have directed this response to Human Resources, and copied Injury Management so that it is received within the timeframe the letter set.

Nothing in this response is a personal complaint about any individual, and I ask for no finding about anyone. My concern is with how the process is being run, and with the support and adjustment that ordinarily apply when an employee is certified fit with restrictions. I have copied my union, and I have written separately to the Chief Executive today about the conflict of interest and the handling of my personal information.

I am engaging with this process and will obtain the information sought. My response is attached, with a proposed allocation of the nine questions and my proposal for return to work. In summary:

- Questions 1(d), 4, 5 and 8 I will obtain from my general practitioner; Questions 7, 1(a), 1(d) and 8 from my treating psychiatrist. Please confirm which questions the Health Service presses, and the invoices will be

directed to you under the arrangement in your letter.

- Questions 2, 3, 6 and 9 are not medical questions: they ask my doctor to answer matters which G03 and the Work Health and Safety Act place with the Health Service. I ask that the Health Service answer those itself; the medical part of each can then be put to my practitioners.
- Question 1(a) is answered openly in the attachment: the diagnosis was made on 24 October 2024, with the clinical context to come from my psychiatrist.
- Question 1(c) I consent to, and the answer is in Dr Ma's own file: his understanding of my condition derives from my psychiatrist's correspondence, not my self-report.
- Question 1(b) alone I ask be withdrawn. G03 clause 3 permits medical information "where necessary" to identify or assess adjustments; a causal formulation does neither, and it goes to a matter in issue in WC/2024/227.

I adopt option 1 and will obtain the information myself. I do not provide an authority under option 2.

Three matters are dealt with in the attachment and I flag them here.

First, timing. I met the 2:00pm deadline of 3 July; the checklist certified me fit with restrictions; within the hour I was told I could not attend, and I have been held out for five weeks without an instrument being identified or a date given. Your letter arrived two hours before a consultation with Dr Ma of which the Health Service had notice; I cancelled it rather than waste a cost, and will rebook once the question set is settled. I ask for the extension your letter contemplates, and for a

date by which the Health Service will tell me whether I am returning to work. I also ask what decision is meant by "a decision regarding your ability to perform your role", and under what instrument it would be made — this is the second occasion an adverse position has been foreshadowed without one.

Second, my exclusion. The Health Service's own letter confirms complaints are escalated to the Client Liaison Officer and/or Manager, and the Role Description contains no complaint-handling duty. There is no identified restriction preventing my performance of the role. I ask to be returned to my roster, and that the leave debited since 3 July 2026 be reversed.

Third, delegation and conflict. I previously asked who is making decisions about me and whether any conflict has been identified and managed; I have not been answered. Part 6 of the attachment asks again and proposes the ordinary separation of functions.

I also attach a short notice setting out the arrangements for the medical appointments — which I will rebook within three business days of the Health Service confirming which questions it presses — and the position on the cost of the reports.

I remain willing to return to work on terms which are safe for me and workable for the Health Service, and would be glad to meet with a support person present. My proposal is attached as a separate document, the Proposal for Return to Work, and I ask that it be received together with my separate request of today under clause 10.3.2 of the certified agreement.

Yours sincerely,

Cory Lea Shepherd

AO3, Switchboard Services, Logan Hospital

0417 400 227 · coryshepherd1@hotmail.com